Answer Key & Rationales · Form 1

Quick key. **P** = pretest (unscored; do not count). Bank 2026.10.23-0423.

#	Key	Area	Sub-area	P
1	**A**	III	III.B	
2	**B**	I	I.A	
3	**D**	I	I.B	
4	**A**	II	II.A	
5	**D**	III	III.B	
6	**B**	II	II.A	**P**
7	**B**	I	I.A	
8	**A**	III	III.A	
9	**B**	III	III.D	
10	**B**	I	I.B	
11	**B**	III	III.B	
12	**A**	III	III.B	
13	**B**	I	I.C	
14	**A**	II	II.A	
15	**B**	II	II.A	
16	**C**	II	II.C	
17	**B**	I	I.C	
18	**B**	III	III.B	
19	**D**	II	II.A	
20	**A**	I	I.C	
21	**C**	II	II.B	
22	**C**	I	I.B	
23	**B**	II	II.B	
24	**C**	II	II.B	
25	**A**	II	II.A	
26	**C**	II	II.C	
27	**A**	I	I.A	
28	**B**	II	II.C	
29	**C**	I	I.A	
30	**C**	III	III.B	
31	**B**	I	I.C	**P**
32	**B**	II	II.B	
33	**A**	III	III.A	
34	**A**	I	I.C	**P**
35	**C**	III	III.D	
36	**C**	II	II.B	
37	**C**	I	I.B	
38	**D**	III	III.C	
39	**B**	I	I.B	
40	**C**	I	I.B	
41	**A**	III	III.C	
42	**D**	III	III.A	
43	**A**	I	I.A	
44	**C**	II	II.A	**P**
45	**B**	III	III.B	
46	**C**	II	II.A	
47	**C**	III	III.B	
48	**C**	I	I.A	
49	**A**	I	I.C	
50	**C**	III	III.B	**P**
51	**D**	I	I.B	
52	**A**	II	II.B	
53	**B**	I	I.B	
54	**C**	III	III.B	
55	**A**	I	I.B	
56	**A**	III	III.B	
57	**C**	II	II.A	
58	**A**	II	II.A	
59	**B**	II	II.A	
60	**C**	III	III.B	**P**
61	**B**	I	I.C	
62	**C**	III	III.C	
63	**C**	II	II.A	
64	**B**	I	I.A	
65	**A**	I	I.B	
66	**C**	II	II.A	
67	**A**	I	I.A	
68	**A**	II	II.B	
69	**C**	II	II.B	
70	**C**	III	III.B	**P**
71	**A**	III	III.B	
72	**B**	III	III.C	
73	**C**	I	I.B	**P**
74	**B**	III	III.A	
75	**A**	II	II.A	
76	**C**	III	III.B	
77	**B**	I	I.A	
78	**A**	I	I.C	
79	**A**	I	I.C	
80	**B**	III	III.B	
81	**B**	III	III.B	
82	**B**	III	III.A	
83	**A**	II	II.A	
84	**A**	I	I.A	
85	**C**	II	II.C	
86	**A**	II	II.C	**P**
87	**C**	III	III.B	
88	**B**	III	III.B	
89	**A**	II	II.B	
90	**A**	I	I.C	
91	**D**	II	II.B	
92	**C**	II	II.B	
93	**C**	II	II.A	
94	**C**	II	II.A	
95	**A**	I	I.C	
96	**C**	I	I.B	
97	**B**	I	I.B	**P**
98	**D**	II	II.A	
99	**C**	I	I.B	
100	**B**	I	I.B	
101	**B**	II	II.B	
102	**C**	III	III.D	
103	**B**	II	II.C	**P**
104	**B**	III	III.C	
105	**D**	I	I.B	
106	**A**	III	III.B	
107	**B**	II	II.A	
108	**D**	I	I.A	
109	**B**	III	III.B	
110	**C**	III	III.A	
111	**D**	I	I.B	
112	**C**	III	III.B	
113	**B**	II	II.C	
114	**A**	I	I.C	
115	**C**	I	I.C	
116	**A**	III	III.B	
117	**B**	II	II.A	
118	**C**	III	III.B	**P**
119	**B**	I	I.B	
120	**B**	III	III.D	
121	**C**	I	I.B	
122	**C**	I	I.A	

Scoring worksheet

Count correct answers on **scored items only**. Item numbers for each sub-area are listed; pretest items are left out.

Area / sub-area	Scored item numbers	Items	Correct
I. Values and Ethics		**40**	**___ / 40**
I.A Ethical Principles and Responsibilities	2, 7, 27, 29, 43, 48, 64, 67, 77, 84, 108, 122	12	____
I.B Ethical Service Delivery	3, 10, 22, 37, 39, 40, 51, 53, 55, 65, 96, 99, 100, 105, 111, 119, 121	17	____
I.C Diversity and Social Justice	13, 17, 20, 49, 61, 78, 79, 90, 95, 114, 115	11	____
II. Assessment and Planning		**35**	**___ / 35**
II.A Assessment Concepts	4, 14, 15, 19, 25, 46, 57, 58, 59, 63, 66, 75, 83, 93, 94, 98, 107, 117	18	____
II.B Assessment Methods and Techniques	21, 23, 24, 32, 36, 52, 68, 69, 89, 91, 92, 101	12	____
II.C Assessment Practices	16, 26, 28, 85, 113	5	____
III. Intervention and Practice		**35**	**___ / 35**
III.A Practice Concepts	8, 33, 42, 74, 82, 110	6	____
III.B Intervention Methods and Techniques	1, 5, 11, 12, 18, 30, 45, 47, 54, 56, 71, 76, 80, 81, 87, 88, 106, 109, 112, 116	20	____
III.C Practice Evaluation and Research	38, 41, 62, 72, 104	5	____
III.D Supervision and Administration	9, 35, 102, 120	4	____

Pretest items (skip when scoring): 6, 31, 34, 44, 50, 60, 70, 73, 86, 97, 103, 118

Total correct (scored items)	_____ / 110
Practice pass line	77 / 110 (70%)
Correct needed to reach the line	77 − your total = _____

The real exam sets its passing score separately for each form (it is reported as a scaled score). 77 of 110 is a deliberately cautious practice target that builds a margin; it is not a prediction of your real result.

Score conversion (raw out of 110 → percent)

The boxed row is the practice pass line: **77 / 110 = 70%**. Any score at or above it meets the line.

Raw	%	Raw	%	Raw	%	Raw	%	Raw	%	Raw	%
110	100%	91	83%	72	65%	53	48%	34	31%	15	14%
109	99%	90	82%	71	65%	52	47%	33	30%	14	13%
108	98%	89	81%	70	64%	51	46%	32	29%	13	12%
107	97%	88	80%	69	63%	50	45%	31	28%	12	11%
106	96%	87	79%	68	62%	49	45%	30	27%	11	10%
105	95%	86	78%	67	61%	48	44%	29	26%	10	9%
104	95%	85	77%	66	60%	47	43%	28	25%	9	8%
103	94%	84	76%	65	59%	46	42%	27	25%	8	7%
102	93%	83	75%	64	58%	45	41%	26	24%	7	6%
101	92%	82	75%	63	57%	44	40%	25	23%	6	5%
100	91%	81	74%	62	56%	43	39%	24	22%	5	5%
99	90%	80	73%	61	55%	42	38%	23	21%	4	4%
98	89%	79	72%	60	55%	41	37%	22	20%	3	3%
97	88%	78	71%	59	54%	40	36%	21	19%	2	2%
96	87%	**77**	**70%**	58	53%	39	35%	20	18%	1	1%
95	86%	76	69%	57	52%	38	35%	19	17%	0	0%
94	85%	75	68%	56	51%	37	34%	18	16%		
93	85%	74	67%	55	50%	36	33%	17	15%		
92	84%	73	66%	54	49%	35	32%	16	15%		

Per-area practice guide: aim for at least 70% in each area as well as overall (Area I 28 of 40; Areas II and III 25 of 35 each).

Rationales for every item

Section 1

1. INT-1115 (rev 1) · Best answer: A

A client often runs late, and the social worker has been holding firm on ending sessions on time. With 2 minutes left today, the client says their partner shoved them last night, and they are going home after the session. Another client is waiting. What is the **BEST** action?

A. Briefly extend the session to check the client's safety tonight
B. End on time and make this the first topic next session
C. Give a hotline number and end the session on time

Why each option is right or wrong

A **Correct.** Limits on session time are useful, but safety comes before structure. A disclosure of recent physical violence by a partner the client is returning to tonight calls for a brief check of immediate danger and a basic safety plan before the client leaves (Davies, Lyon & Monti-Catania, 1998). A short, purposeful extension is a boundary crossing that serves the client, not a violation (Gutheil & Gabbard, 1993). It beats C because a hotline number is a helpful resource, but it does not tell the worker whether the client is in danger tonight.

B **Incorrect** (trap: Ignores risk). Holding the time limit protects the structure of the work and is usually right with chronic lateness. Here, a new disclosure of violence, with the client returning home tonight, makes waiting a week unsafe.

C **Incorrect** (trap: Good but not best). Hotline information is a useful part of safety planning, but handing it over without asking about tonight's danger leaves the risk unknown. It works best as part of a brief safety check, not in place of one.

Takeaway: Hold session-time limits, but make a brief, purposeful exception when a client discloses current danger at the end of a session.

Sources: Gutheil, T. G., & Gabbard, G. O. (1993). The concept of boundaries in clinical practice: Theoretical and risk-management dimensions. American Journal of Psychiatry, 150(2), 188-196, boundary crossings versus violations; time as a boundary dimension; Davies, J., Lyon, E., & Monti-Catania, D. (1998). Safety Planning with Battered Women: Complex Lives/Difficult Choices. Sage, safety planning

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.4 Setting limits and boundaries with clients · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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2. ETH-1100 (rev 2) · Best answer: B

Social workers at a community mental health center have voted to strike in 10 days over unsafe caseloads, and the employer has refused further talks. A social worker plans to join the strike. The worker's caseload includes two clients with suicide attempts in the past 3 months and several clients who rely on weekly sessions. What should the social worker do **FIRST**?

A. Tell clients the strike date at their next sessions
B. Plan interim coverage and crisis access for each client
C. Withdraw from the strike because of the high-risk clients

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Clients should hear about the interruption promptly, but announcing a date with nothing in place leaves high-risk clients without a way to get care. Notice fits right after coverage and crisis access are arranged, so it can include them.

B **Correct.** Social workers may take part in organized labor action, but the Code asks them to weigh the possible impact on clients before acting (NASW Code 3.10(a), 3.10(b)). Clients who still need services must not be abandoned, and an anticipated interruption calls for prompt transfer or continuation planning (1.17(b), 1.17(e)). With two recent suicide attempts on the caseload, the worker first arranges who will see each client and how each can reach crisis help. B beats A because telling clients the date is useful only once there is a plan to share with them.

C **Incorrect** (trap: Good but not best). The Code permits organized action and notes that reasonable social workers disagree about striking (3.10). Recent attempts make planning essential, but they do not by themselves bar participation; deciding to withdraw before exploring coverage treats the conflict as unresolvable when it can be addressed.

Takeaway: Before joining a strike or other job action, arrange coverage and crisis access so clients who still need services are not abandoned.

Sources: NASW Code of Ethics (2021), 3.10(a); NASW Code of Ethics (2021), 3.10(b); NASW Code of Ethics (2021), 1.17(b); NASW Code of Ethics (2021), 1.17(e)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 1 of 3

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3. ETH-1502 (rev 1) · Best answer: D

A social worker who is going through a divorce reviews notes from 4 recent sessions with a client whose marriage is ending. In each session, the social worker spent much of the time describing their own separation. The client recently said, 'It helps to know you get it.' What should the social worker do **FIRST**?

A. Keep sharing, since the client finds it helpful
B. Transfer the client to a colleague without a similar history
C. Tell the client that future sessions will focus on them
D. Seek supervision about the pattern of self-disclosure

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). The client's comment shows the disclosures feel validating, but the sessions now center on the worker. Disclosure that meets the worker's needs blurs the role and crowds out the client's own grief, even when the client says it helps.

B **Incorrect** (trap: Hands off too soon). Transfer is a large step for a problem the worker can likely correct with supervision. It disrupts the client's care and hands off a boundary issue the worker is responsible for fixing. Transfer fits if supervision shows the worker cannot regain objectivity.

C **Incorrect** (trap: Right action, wrong time). Bringing the focus back to the client is a needed change and will come soon. Done first, without supervision, it treats the

symptom but leaves the worker's own unresolved reaction, which produced the pattern, unexamined.

D **Correct.** Self-disclosure is appropriate only when it serves the client, stays brief and returns the focus to the client (Hepworth et al.). Long, repeated accounts of the worker's own divorce suggest the worker's needs are driving the sessions, a sign that a personal difficulty is affecting practice. The Code directs social workers to seek consultation when personal problems interfere with their work (NASW Code 4.05(b)). It beats C because refocusing is the right change, but without supervision the worker may not understand or resist the pull to disclose.

Takeaway: When self-disclosure starts serving the worker's needs, seek supervision first, then refocus sessions on the client.

Sources: NASW Code of Ethics (2021), 4.05(b); Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Ch. 5: authenticity and self-disclosure
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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4. ASM-1183 (rev 2) · Best answer: A

A parent asks a youth program's social worker about a volunteer coach who has grown close to the parent's 12-year-old. The coach praises the child in front of the team, offers rides home after practice, and recently gave the child a phone, saying the gift should stay 'just between us.' Which behavior is the **MOST** concerning sign of grooming?

A. Asking that the gift be kept from others
B. Praising the child in front of the team
C. Offering the child rides home after practice

Why each option is right or wrong

A **Correct.** Grooming builds trust and special status with a child while creating secrecy and isolation that make later abuse less likely to be disclosed. Craven et al. (2006) name keeping the child silent to avoid disclosure as a core goal of grooming, and note that many grooming behaviors look like ordinary caring. An adult asking a child to hide a gift, especially a private phone, has no innocent coaching purpose. It beats C because rides home can be ordinary help and become concerning mainly when combined with secrecy.

B **Incorrect** (trap: Near-miss diagnosis). Public praise is part of normal coaching and happens in front of others. It can feed a sense of being special, but on its own it is the least specific of these behaviors.

C **Incorrect** (trap: Near-miss diagnosis). Rides home can create time alone with a child, and offenders do seek access this way. Many coaches give rides for innocent reasons, though, so this is less specific than an explicit request for secrecy.

Takeaway: Grooming often looks like ordinary kindness; an adult's request that a child keep contact or gifts secret is a strong warning sign.

Sources: Craven, S., Brown, S., & Gilchrist, E. (2006). Sexual grooming of children: Review of literature and theoretical considerations. Journal of Sexual Aggression, 12(3), 287-299, whole article
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.12 Traits of perpetrators of abuse, neglect, exploitation · **Qualifier** MOST · **Cognitive** recall · **Difficulty** 1 of 3

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5. INT-1185 (rev 1) · Best answer: D

During a video session from home, a 67-year-old client who lives alone suddenly begins slurring words. The social worker sees that one side of the client's mouth droops and that the client cannot lift their right arm to adjust the camera. When asked how they feel, the client says, 'I'm just tired. I'll lie down.' What should the social worker do **FIRST**?

A. Ask about the client's recent sleep and medication changes
B. Call the client's emergency contact to check on them
C. Suggest the client rest and reconnect in one hour
D. Request an ambulance to the client's home

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Gathering history is useful for most changes in a client's condition. Here the observed signs already warrant emergency care, and every minute spent on questions delays treatment that works best when given quickly.

B **Incorrect** (trap: Right action, wrong time). Reaching the emergency contact is a sensible follow-up once emergency services are on the way. Relying on a relative first wastes minutes that matter in stroke care, and the contact may be slow or unreachable.

C **Incorrect** (trap: Ignores risk). A client's wishes usually guide practice, but these signs point to a medical emergency that can also cloud judgment. Waiting an hour could cost the window for effective stroke treatment.

D **Correct.** Sudden facial droop, one-sided arm weakness and slurred speech are stroke warning signs, and the American Stroke Association advises calling 911 immediately, even if symptoms fade. The worker has observed these signs directly and the client has already answered, so more questions only delay care; the client's reassurance does not change the emergency. It beats B because an emergency contact may be slow or unreachable, and calling them is a useful step after emergency services are dispatched.

Takeaway: Observed signs of a medical emergency outweigh a client's reassurance; call emergency services first, then notify supports.

Sources: American Stroke Association (2026). Stroke Symptoms: B.E. F.A.S.T., Warning signs; call 911 immediately

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.3 Active listening and observation skills · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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6. ASM-1530 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

Parents aged 79 and 81 have cared at home for decades for their 50-year-old adult child, who has schizophrenia. At a community mental health visit, the parents say they are slowing down and worry about who will help their child after they die. The adult child is stable, takes medication and attends a day program. Which response is **BEST**?

A. Help the parents draft a plan to share with their adult child later
B. Start planning meetings with the adult child as a core participant
C. Teach the parents to recognize early warning signs of relapse

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). The parents' initiative is welcome, but drafting the plan without the adult child decides major questions about housing and support for a stable adult who can take part. Plans made this way may not fit the person's preferences and are harder to carry out.

B **Correct.** Future care planning should start while aging parents are still able to help, and the person with serious mental illness should be a core participant whose preferences shape the plan (Sivakumar et al., 2021). Including the adult child also honors self-determination (NASW Code 1.02). It beats A because a plan drafted by the parents alone may not match the adult child's wishes or abilities, and parents often underestimate what their child can learn. Housing, finances, legal arrangements and the roles of siblings or friends can then be worked out together.

C **Incorrect** (trap: Good but not best). Relapse education helps families, but after decades of caregiving these parents are raising a different need: who will provide support when they no longer can. Teaching warning signs does not answer their stated concern.

Takeaway: When aging parents care for an adult child with serious mental illness, begin future care planning early and include the adult child as a core participant.

Sources: Sivakumar, T., et al. (2021). Future care planning: Concerns of elderly parents caring for a person with serious mental illness. Psychiatric Services, Open Forum, doi:10.1176/appi.ps.201900267; NASW Code of Ethics (2021), 1.02

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.14 How caregiving affects families · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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7. ETH-1023 (rev 1) · Best answer: B

A client ordered by a court to attend a driving-under-the-influence program asks the social worker at intake, 'What happens if I don't want to do this?' What is the **MOST** appropriate response?

A. Tell the client the court order removes the right to refuse

B. Explain the right to decline and what is reported

C. Suggest the client ask their attorney before going further

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). A court order creates consequences for refusing, but the client can still choose to decline and accept them. Saying the right to refuse is gone misstates the client's position and undermines informed consent.

B **Correct.** Mandated clients keep the right to make informed choices. The social worker explains the nature and extent of services and the extent of the client's right to refuse (NASW Code 1.03(d)), including what the program will report to the court and the likely consequences of declining. B beats C because legal advice belongs to the attorney, but explaining the program, its reporting and the client's choices is the social worker's own consent duty.

C **Incorrect** (trap: Hands off too soon). The attorney can advise on legal consequences, and the worker might suggest that as well. The social worker must still explain the

services, reporting and right to refuse; sending the client away first leaves the consent duty unmet.

Takeaway: Mandated clients may refuse services; explain the services, what will be reported, and the consequences of declining.

Sources: NASW Code of Ethics (2021), 1.03(d)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.4 Informed consent: principles and how it is obtained · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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8. INT-1563 (rev 1) · Best answer: A

Which practice change does the federal Family First Prevention Services Act of 2018 encourage in child welfare agencies?

A. Evidence-based in-home help so children can stay safely at home
B. Faster filing to end parental rights for children in long-term care
C. Wider use of group homes for youth with complex needs

Why each option is right or wrong

A **Correct.** The Family First Prevention Services Act lets states use federal Title IV-E funds for evidence-based mental health, substance use and in-home parenting services that help children at imminent risk of foster care stay safely with their families. It also limits federal payments for most group care after the first 2 weeks. It beats B because faster termination timelines come from the Adoption and Safe Families Act of 1997, an earlier permanency law. Knowing which law drives which practice helps social workers explain service changes to families.

B **Incorrect** (trap: Wrong model or concept). Timelines for seeking termination of parental rights, such as after 15 of the most recent 22 months in care, come from the Adoption and Safe Families Act of 1997. Family First focuses on prevention and placement quality.

C **Incorrect** (trap: Misreads law or ethics). Family First moves the other way. It restricts federal funding for most congregate care after the first 2 weeks unless the setting meets standards such as those for a qualified residential treatment program.

Takeaway: Family First funds evidence-based prevention services to keep children safely with families and limits federal funding for most group care.

Sources: Family First Prevention Services Act of 2018 (Bipartisan Budget Act of 2018, P.L. 115-123, Div. E, Title VII), Title IV-E prevention services; limits on congregate care payments; Adoption and Safe Families Act of 1997, 42 U.S.C. 675(5)(E)
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.7 Effects of policy, regulation, legislation on practice and services · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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9. INT-1600 (rev 1) · Best answer: B

A child welfare supervisor meets with a caseworker each week. In one meeting, the supervisor reviews the caseworker's cases and moves two families to a coworker who has fewer. The supervisor also checks that court reports will be filed on time. Which function of supervision is the supervisor **MOST** likely carrying out?

A. Educational
B. Administrative
C. Supportive

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The educational function builds knowledge and skill, such as teaching how to write a strong court report. Checking whether reports will be filed on time is quality control of the work, not teaching.

B **Correct.** Kadushin's administrative function covers assigning work, balancing workloads, and monitoring policy compliance and quality (Kadushin & Harkness, 2014; NASW & ASWB supervision standards, 2013). Reassigning families to even out caseloads and checking court reports against agency deadlines are all administrative tasks. It beats C because the supervisor acts on assignments and deadlines without attending to the caseworker's stress or morale; the cases move to manage the unit's work, not as emotional support. It beats A because no knowledge or skill is being taught.

C **Incorrect** (trap: Wrong model or concept). The supportive function addresses morale, stress and burnout. Lighter caseloads can ease stress, but this supervisor is managing assignments and deadlines rather than responding to the caseworker's feelings, which points to administrative work.

Takeaway: Assigning work, balancing caseloads and monitoring compliance are administrative supervision; teaching skills is educational; tending morale and stress is supportive.

Sources: Kadushin & Harkness (2014). Supervision in Social Work (5th ed.). Columbia University Press, Ch. 2: Administrative Supervision; NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Overview of Supervision: Administrative

Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.2 Supervision and consultation models: individual, peer, group · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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10. ETH-1072 (rev 2) · Best answer: B

A client at a community mental health center learns that extra weekly sessions there have a 6-week waitlist. The client asks to see the social worker at the social worker's evening private practice and offers to pay a reduced cash fee. What is the **MOST** appropriate response?

A. Agree to see the client privately at the reduced fee
B. Decline and pursue added services through the center
C. See the client outside the center in the evenings for free

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Even at a reduced fee, privately charging a client for services available through the agency conflicts with 1.13(c), which bars seeking such fees, and sets the worker's income against the client's interests (1.06(a)).

B **Correct.** Social workers should not seek a private fee for services clients are entitled to through their employer or agency (NASW Code 1.13(c)), and taking the client's offer would have the same effect. The client is entitled to services at the center, so the right path is to pursue options there, such as an interim group, a cancellation list or an urgent-need review. B beats A because a reduced fee does not cure the problem: it still moves an agency client into the worker's paid practice, creating a conflict of interest (1.06(a)).

C **Incorrect** (trap: Boundary slip or collusion). Free private sessions remove the fee but create a dual relationship outside the agency's structure and oversight. Added help should come through the center.

Takeaway: Do not charge agency clients privately for services they can get through the agency; seek more help within the agency.

Sources: NASW Code of Ethics (2021), 1.13(c); NASW Code of Ethics (2021), 1.06(a)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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11. INT-1527 (rev 2) · Best answer: B

At a senior center, a client caring for a spouse with major neurocognitive disorder is using structured problem solving to tackle exhaustion. Together they defined the problem and listed options, and the client chose an adult day program 2 days a week. The client has now used it for 3 weeks. What should the social worker do **NEXT**?

A. Brainstorm more respite options in case the program falls short
B. Review with the client how well the program is meeting the goal
C. Begin structured problem solving on another stressor

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Returning to alternatives is correct when the chosen solution has not worked. Here no one has checked the results yet, so brainstorming now skips verification and may signal that the plan is expected to fail.

B **Correct.** Structured problem solving ends with implementation and verification: once the chosen solution has been tried, client and worker check the results against the original goal (D'Zurilla & Nezu, 2007). Three weeks of day program use is enough to judge whether the caregiver's exhaustion is easing. It beats A because generating more options is the right move only if the review shows the solution is falling short; doing it now skips evaluation and may undercut a plan that is working.

C **Incorrect** (trap: Right action, wrong time). Taking on the next problem is part of structured problem solving, and other stressors may well need attention. Moving on before checking whether the day program has eased the exhaustion skips the verification step, so neither the client nor the worker learns whether the chosen solution worked.

Takeaway: In structured problem solving, after a solution is put in place, evaluate its results against the goal before generating new options.

Sources: D'Zurilla, T. J., & Nezu, A. M. (2007). Problem-Solving Therapy: A Positive Approach to Clinical Intervention (3rd ed.). Springer, rational problem-solving skills
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.16 Structured problem-solving approaches · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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12. INT-1127 (rev 2) · Best answer: A

Early in psychodynamic therapy, a client realized that they keep choosing partners who dismiss them, much as a parent did. Over the next 8 months, the client spots the same expectation with a boss, a close friend, and the social worker. Each time, the client feels less upset and tries a new response. Which process is **MOST** likely taking place?

A. Working through
B. Repetition compulsion
C. Corrective emotional experience
D. Abreaction

Why each option is right or wrong

A **Correct.** Working through is the gradual, repeated process of applying an insight across many situations until it changes feeling and behavior (Freud, 1914). The client gained the insight months ago and now recognizes the same expectation with a boss, a friend, and the social worker, with less distress and new responses each time. It beats B because repetition compulsion means reenacting the old pattern without awareness; this client is noticing the pattern and acting differently, which shows it loosening.

B **Incorrect** (trap: Wrong model or concept). Repetition compulsion is the unconscious pull to recreate early painful relationships. The history of choosing dismissive partners fits it, but the client is now recognizing the expectation and responding differently, which shows the pattern changing rather than repeating.

C **Incorrect** (trap: Wrong model or concept). A corrective emotional experience occurs when the therapist responds differently from the original figure, so the client relives the old conflict with a new ending (Alexander & French, 1946). The stem centers on the client's own repeated recognition across many relationships, not on a particular response from the social worker.

D **Incorrect** (trap: Wrong model or concept). Abreaction is the emotional release that comes from reliving a repressed memory, usually in one intense moment. The client here changes gradually over months, with feelings becoming milder, not through a sudden discharge of emotion.

Takeaway: Insight alone rarely changes a pattern; working through applies it repeatedly across situations until feelings and behavior shift.

Sources: Freud, S. (1914). Remembering, repeating and working-through. In Standard Edition of the Complete Psychological Works of Sigmund Freud, Vol. 12. Hogarth Press, working through; Alexander, F., & French, T. M. (1946). Psychoanalytic Therapy: Principles and Application. Ronald Press, corrective emotional experience
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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13. ETH-1581 (rev 2) · Best answer: B

In supervision, a supervisee calls a client who has missed four group sessions 'noncompliant' and proposes closing the case. The client works night shifts, relies on two buses, and asked last month whether the group could meet at another time. What is the **BEST** action by the supervising social worker?

A. Approve closing the case under the attendance policy
B. Guide the supervisee to plan access options with the client
C. Suggest the supervisee assess the client's readiness to change

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). Closing the case treats access barriers as noncompliance and ends service for a client who asked for a workable time. Closure under policy fits after reasonable efforts to remove barriers have failed.

B **Correct.** Anti-oppressive practice looks for structural barriers before labeling a client unmotivated, and the Code calls on social workers to expand choice and opportunity for oppressed and disadvantaged people (NASW Code 6.04(b)). The client works nights, depends on two buses and has already asked for another meeting time, so the absences point to access, not reluctance. Guiding the supervisee to work out schedule, format or transport options with the client addresses the real barrier and models a structural lens (Dominelli, 2002). It beats C because a readiness assessment still locates the problem in the client's motivation when the facts point to access.

C **Incorrect** (trap: Skips assessment). Assessing readiness is useful when ambivalence is the issue. Here the client asked for a different time, and the schedule and transport facts point to access, so this step misreads the problem.

Takeaway: Rule out structural barriers such as work hours and transport before labeling a client noncompliant or unmotivated.

Sources: NASW Code of Ethics (2021), 6.04(b); Dominelli, L. (2002). Anti-Oppressive Social Work Theory and Practice. Palgrave Macmillan, structural analysis of service barriers
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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14. ASM-1152 (rev 1) · Best answer: A

A school nurse refers a 9-year-old who comes to the health office with stomachaches on most test days and many Monday mornings. A pediatrician found no medical cause. The parent asks the school social worker to support a plan that sends the child home whenever the pain starts. What should the social worker do **FIRST**?

A. Assess the child's worries and avoidance with child and parent
B. Support the parent's go-home plan to ease distress
C. Teach the child slow breathing to use before each test

Why each option is right or wrong

A **Correct.** Assessment comes before intervention. Stomachaches and headaches are common in anxious children, and this pattern, pain tied to tests and the start of the school week with a normal medical workup, points toward anxiety. The AACAP practice parameter calls for gathering information from both the child and adults, because each sees different parts of the problem (Connolly & Bernstein, 2007). It beats C because breathing practice is a reasonable coping tool, but choosing it before learning what the child fears and avoids skips the assessment that should guide the plan.

B **Incorrect** (trap: Skips assessment). Sending the child home relieves distress in the moment but can reward avoidance and let school-related anxiety grow. Any plan for handling the pain at school should follow an assessment of what drives it.

C **Incorrect** (trap: Right action, wrong time). Relaxation skills are often part of treatment for childhood anxiety. Taught now, though, they rest on a guess about the problem; the social worker first needs to learn what the child worries about and avoids.

Takeaway: When a medical workup is negative, assess for anxiety behind a child's school-linked physical complaints before choosing accommodations or skills.

Sources: Connolly, S. D., & Bernstein, G. A. (2007). Practice parameter for the assessment and treatment of children and adolescents with anxiety disorders. Journal of the American Academy of Child and Adolescent Psychiatry, 46(2), 267-283, Clinical presentation; Recommendation 1
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.5 Signs of mental and emotional illness across the lifespan · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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15. ASM-1150 (rev 1) · Best answer: B

A 34-year-old group home resident with moderate intellectual developmental disorder communicates with a few words and gestures. Over the past 3 weeks, staff have seen the resident strike the side of their own jaw, wake often at night, and push meals away. The house manager asks the social worker to write a behavior plan. What should the social worker do **FIRST**?

A. Complete a functional assessment of the jaw striking
B. Arrange a medical and dental exam for possible pain
C. Refer the resident to psychiatry to evaluate depression
D. Draft a plan that rewards calm mealtimes and sleep

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A functional assessment is the usual basis for a behavior plan and would come next if no medical cause is found. Done first, it may label pain-driven behavior as attention or escape seeking and delay treatment.

B **Correct.** When a person with limited speech shows a sudden change in behavior, sleep or eating, physical causes such as pain are checked first. NICE guidance on behavior that challenges says initial assessment should take physical health problems into account (NICE NG11, 1.5.5). Striking the jaw, broken sleep and pushing food away over 3 weeks fit untreated dental or other pain. It beats A because a functional assessment is the right next step if the exam finds no medical cause; done first, it risks building a plan around pain that only treatment can relieve.

C **Incorrect** (trap: Right action, wrong time). Sleep and appetite changes can signal depression, which is often missed in people with intellectual disabilities. A psychiatric evaluation is reasonable once physical causes such as pain are ruled out, because pain can produce the same picture.

D **Incorrect** (trap: Skips assessment). A reward plan treats the behavior before anyone knows why it started. If pain is the cause, rewards will not help, and the resident's distress will continue unrecognized.

Takeaway: A sudden behavior change in a person with limited speech calls for a medical check for pain or illness before behavioral or psychiatric plans.

Sources: NICE (National Institute for Health and Care Excellence) (2015). Challenging behaviour and learning disabilities: prevention and interventions for people with learning disabilities whose behaviour challenges (NICE NG11), Recommendation 1.5.5

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.5 Signs of mental and emotional illness across the lifespan · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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16. ASM-1606 (rev 1) · Best answer: C

A social worker providing case management at an HIV services agency works with a client who also sees a primary care physician, a psychiatrist and an outpatient substance use program. The client arrives upset after receiving three different instructions about sleep medication timing and appointment days and says, 'Everyone tells me something different.' The client has signed releases for all providers. Which action is **MOST** helpful?

A. Help the client decide which provider's advice to follow

B. Write every instruction on one calendar for the client

C. Convene the providers to agree on one coordinated approach

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Asking the client to choose among clinical instructions places a provider problem on the person least equipped to judge it. The contradictions, which include medication timing, need to be settled by the providers themselves.

B **Incorrect** (trap: Misuses the technique). A single calendar helps with complex schedules and may be useful once the plan is unified. Recording contradictory instructions side by side does not resolve them and leaves the client unsure which to follow.

C **Correct.** Case management coordinates services so that providers work from one plan and care stays continuous (NASW Standards for Social Work Case Management, Standards 6 and 8). The conflicting instructions come from the providers, not the client, and the signed releases allow the social worker to bring them together. It beats B because a calendar organizes the advice but leaves the contradictions in place, so the client would still be following conflicting directions about medication and appointments.

Takeaway: When several providers give conflicting directions, the case manager coordinates them into one plan rather than leaving the client to sort it out.

Sources: NASW Standards for Social Work Case Management (2013), Standard 6: Service Planning, Implementation, and Monitoring; Standard 8: Interdisciplinary and Interorganizational Collaboration
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.2 Goal setting, treatment planning, triage, and service plans · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

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17. ETH-1595 (rev 3) · Best answer: B

A hospital social worker tracks discharges for a year and finds that older patients from a mostly Black zip code go to lower-rated nursing facilities more often than other patients with the same insurance. The social worker shares the data with the unit manager, who says to drop it. What is the **MOST** effective way for the social worker to address the pattern?

A. Steer each of the worker's own patients to higher-rated facilities
B. Present the findings to the quality improvement committee
C. File a complaint with the state civil rights office now

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Helping individual patients reach better facilities is worthwhile and can continue. As the sole response, it leaves the institutional pattern in place for patients served by other staff.

B **Correct.** When an agency practice disadvantages clients by race and a first internal step fails, the Code calls for working to align employer practices with ethical standards and for holding institutions accountable (NASW Code 3.09(d); 1.05(c); 6.04(d)). The social worker has solid data and the manager declined to act, so the effective step is the hospital committee charged with quality of care, which can change the pattern for every patient. It beats A because steering one's own patients helps a few people but leaves the discharge pattern in place for everyone else.

C **Incorrect** (trap: Right action, wrong time). An outside complaint may become appropriate if internal channels fail. The hospital's quality committee, charged with exactly this kind of pattern, has not yet seen the data, and going outside first skips a channel that can change practice directly.

Takeaway: If a manager dismisses evidence of racial disparity, take it to the next organizational channel rather than settling for individual workarounds.

Sources: NASW Code of Ethics (2021), 3.09(d); NASW Code of Ethics (2021), 1.05(c); NASW Code of Ethics (2021), 6.04(d)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 2 of 3

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18. INT-1052 (rev 1) · Best answer: B

A family in which three generations share a home seeks help for a 10-year-old who ignores household rules. By family tradition, the grandparent makes major household decisions. The parent works nights, and in session the parent and grandparent contradict each other's rules in front of the child. Using a structural approach, what is the **MOST** appropriate intervention?

A. Strengthen the parent's authority by limiting the grandparent's role
B. Help the parent and grandparent agree on rules as a team
C. Explore the child's feelings about the two sets of rules

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Reducing the grandparent's role imposes a nuclear-family model the family has not chosen. Structural work clarifies who leads, but it builds on the family's own arrangement, and here the grandparent's authority is part of it. This would fit only if the family itself wanted that shift.

B **Correct.** Structural family therapy aims for a clear, united executive subsystem, but who belongs in it depends on the family's own culture and arrangements (Minuchin, 1974). Here the grandparent's authority is a valued tradition, and the problem is that the two adults undercut each other. Building a parent-grandparent team restores a clear hierarchy without imposing an outside family form (McGoldrick et al., 2005). It beats A because textbook mapping often places parents alone at the top, but removing the grandparent's role would override the family's norms and likely cost the worker their trust.

C **Incorrect** (trap: Good but not best). The child's feelings matter, but the rule problem comes from the adults' split, not from the child. Exploring feelings alone leaves the contradiction in place; in structural terms, change comes when the executive subsystem works as a unit.

Takeaway: Structural work builds a united executive subsystem; define its members by the family's cultural arrangement, not by a nuclear-family template.

Sources: Minuchin, S. (1974). Families and Family Therapy. Harvard University Press, subsystems and hierarchy: the executive subsystem; McGoldrick, M., Giordano, J., & Garcia-Preto, N. (Eds.). (2005). Ethnicity and Family Therapy (3rd ed.). Guilford Press, overview: culture, ethnicity and family structure
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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19. ASM-1630 (rev 1) · Best answer: D

Helms described a series of statuses in White racial identity. Which status brings guilt, shame and confusion as a person first faces racism and White advantage?

A. Contact
B. Reintegration
C. Pseudo-independence
D. Disintegration

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Contact is the starting status, marked by little awareness of racism and a belief that race does not matter. A person in contact has not yet felt the guilt and confusion that come with recognizing racism.

B **Incorrect** (trap: Wrong model or concept). Reintegration often follows disintegration as a retreat into idealizing one's own group and denying racism. Anger, not guilt, is its typical feeling, so it does not match the description.

C **Incorrect** (trap: Wrong model or concept). Pseudo-independence involves an intellectual grasp of institutional racism with limited emotional engagement. The person has moved past raw guilt and confusion to a more distant, analytical stance.

D **Correct.** In Helms's model, disintegration begins when a person can no longer ignore racism and White advantage. The new awareness clashes with beliefs about fairness and brings guilt, shame and confusion, and the person may stay silent when racism occurs. It beats B because reintegration is the retreat that can follow: the person falls back on idealizing their own group, denies racism, and replaces guilt with anger (Helms, 1990).

Takeaway: Helms: contact (unaware), disintegration (guilt, confusion), reintegration (retreat, anger), pseudo-independence (intellectual), then immersion/emersion and autonomy.

Sources: Helms, J. E. (Ed.). (1990). Black and White Racial Identity: Theory, Research, and Practice. Greenwood Press, White racial identity development model

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.19 Racial, ethnic, cultural, spiritual development across the lifespan · **Qualifier** none · **Cognitive** recall · **Difficulty** 3 of 3

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20. ETH-1719 (rev 2) · Best answer: A

In a primary care clinic, a client with type 2 diabetes who uses insulin mentions a plan to fast during Ramadan next month. The physician, concerned about low blood sugar, asks the social worker to 'talk them out of it.' The social worker is meeting the client for the first time. What should the social worker do **FIRST**?

A. Ask the client to describe what fasting means to them

B. Arrange a pre-Ramadan medical visit to plan safer fasting

C. Help the client plan Ramadan meals before dawn and after sunset

D. Explain that the faith excuses people who are ill from fasting

Why each option is right or wrong

A **Correct.** Religion shapes health decisions (NASW Code 1.05(a)), and clients are the experts on their own culture (1.05(c)), so respecting a faith practice starts with learning what it means to the client. The social worker is meeting the client for the first time, the fast is a month away, and the physician wants persuasion. Asking about the fast first lets any safety plan be built with the practice rather than against it. It beats B because a medical visit to plan safer fasting is the right next step, but arranging it before hearing the client treats the practice as a problem to manage.

B **Incorrect** (trap: Right action, wrong time). Fasting while using insulin carries a real risk of low blood sugar, and guidelines call for risk stratification and medication adjustment before Ramadan (Hassanein et al., 2017). This visit should follow soon.

Arranged before the social worker learns the client's goals for the fast, it can feel like the persuasion the physician asked for.

C **Incorrect** (trap: Right action, wrong time). Meal timing matters when fasting with diabetes, and meal planning may follow. It belongs after the client's goals are known and the physician has reviewed insulin, since meal plans alone cannot make the current regimen safe.

D **Incorrect** (trap: Outside the social work role). Many traditions excuse the ill from fasting, but whether that applies is for the client and their religious advisors to decide. A social worker who rules on it oversteps and may damage trust.

Takeaway: When a religious practice raises a health concern, first learn what it means to the client, then plan safety with the practice rather than against it.

Sources: Hassanein, M., et al. (2017). Diabetes and Ramadan: Practical guidelines. Diabetes Research and Clinical Practice, 126, 303-316, risk stratification; medication adjustment; NASW Code of Ethics (2021), 1.05(a); NASW Code of Ethics (2021), 1.05(c)

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.3 Culture, race, gender, sexuality, intersectionality shaping identity and behavior · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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21. ASM-1589 (rev 3) · Best answer: C

At an opioid treatment program, a client on methadone for 4 months has had no opioid use and attends every counseling session. The client also uses cannabis daily and says, 'Weed isn't a problem. It's the only thing that helps me sleep.' Program staff want the treatment plan to require stopping cannabis now. What is the **BEST** way to proceed?

A. Make stopping cannabis a required goal in the plan

B. Explain to the client the health risks of using cannabis every day

C. Explore the client's own view of what cannabis gives and costs

D. Leave cannabis out of the plan while opioid goals come first

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Requiring abstinence from a substance the client does not yet see as a problem mismatches the client's stage and can provoke resistance or dropout. It also risks the progress the client has made with opioids.

B **Incorrect** (trap: Too directive). Giving risk information before learning how the client sees cannabis can feel like a lecture and push the client to defend a habit they value for sleep. Information offered with permission can help later, once the client's own view is known.

C **Correct.** Readiness is specific to each behavior: this client is in action for opioids but sees no problem with cannabis, a precontemplation stance toward it (SAMHSA TIP 35; Prochaska et al., 1992). Exploring the client's own view of what cannabis does for and against them, including the sleep problem it is meant to solve, is how motivational work raises ambivalence without provoking resistance, and it keeps goals matched to each behavior's stage. It beats A because requiring an unwanted goal can damage engagement that is clearly working.

D **Incorrect** (trap: Good but not best). Keeping opioid recovery first is sound, but leaving daily cannabis out entirely skips learning how the client sees it and what the sleep

problem needs. The plan can hold opioid goals steady while the cannabis conversation begins.

Takeaway: When a client in recovery sees no problem with a second substance, explore their own view of its benefits and costs before setting goals for it.

Sources: SAMHSA TIP 35 (2019), Ch. 4: From Precontemplation to Contemplation: Building Readiness; Prochaska, J. O., DiClemente, C. C., & Norcross, J. C. (1992). In search of how people change: Applications to addictive behaviors. American Psychologist, 47(9), 1102-1114, whole article

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.4 Assessing motivation, barriers, and readiness for change · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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22. ETH-1070 (rev 1) · Best answer: C

A client in a private practice signed a fee agreement at intake stating that sessions missed without 24 hours' notice cost $40. The client misses a session without calling. The office manager suggests billing the client's health plan for the missed hour instead. How should the social worker handle the missed session?

A. Bill the health plan for the missed session

B. Bill the plan at a shorter-session rate instead

C. Charge the client the agreed missed-session fee

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Health plans pay for services delivered, not appointments missed. Submitting a claim for a session that did not occur misrepresents the service and is insurance fraud.

B **Incorrect** (trap: Misreads law or ethics). Lowering the amount does not make the claim true; no service of any length took place. Any claim to the plan for this hour misrepresents services (3.05).

C **Correct.** Billing must accurately reflect the nature and extent of services actually provided (NASW Code 3.05), and billing a payer for a session that did not happen is fraud (4.04). Costs, including missed-session charges, are explained when services begin (1.03(a)), and this client agreed in writing to a $40 fee. C beats B because billing a shorter session still claims a service that never occurred; a smaller false claim is still false.

Takeaway: Bill payers only for services delivered; missed sessions may be charged to the client only under a fee agreed in advance.

Sources: NASW Code of Ethics (2021), 3.05; NASW Code of Ethics (2021), 4.04; NASW Code of Ethics (2021), 1.03(a)

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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23. ASM-1570 (rev 1) · Best answer: B

In a hospice family meeting, an adult child who has cared for a dying parent for 8 months says, 'I feel like a terrible person for wishing this were over.' The social worker replies, 'After months of caring day and night, wanting the strain to end makes sense.' Which interviewing skill is the social worker **MOST** likely using?

A. Reflection of feeling
B. Validation
C. Reassurance
D. Interpretation

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Reflection of feeling would name the emotion, for example, 'You feel guilty for wanting this to end.' The reply here does more than mirror guilt; it treats the reaction as understandable in context.

B **Correct.** Validation communicates that a person's feelings or reactions make sense given their circumstances, without judging them (Linehan, 1993). The social worker ties the caregiver's wish to months of exhausting care and says the wish makes sense, which counters the caregiver's harsh self-judgment. It beats A because reflection of feeling mirrors the emotion the client expressed, such as guilt; this response instead affirms that the reaction is legitimate given the situation.

C **Incorrect** (trap: Wrong model or concept). Reassurance offers comfort such as 'You're not a terrible person' or 'It will get easier.' This reply does not contradict the concern or promise relief; it explains why the feeling fits the situation.

D **Incorrect** (trap: Wrong model or concept). Interpretation offers a meaning the client has not recognized, often about underlying motives. The reply stays with the caregiver's stated experience rather than suggesting a hidden explanation.

Takeaway: Validation tells clients their reactions make sense in context; reflection mirrors the feeling; reassurance offers comfort that can dismiss it.

Sources: Linehan, M. M. (1993). Cognitive-Behavioral Treatment of Borderline Personality Disorder. Guilford Press, validation strategies

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.2 Interviewing skills: clarifying, validating, confronting, reflecting, interpreters · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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24. ASM-1130 (rev 1) · Best answer: C

A 31-year-old comes to a community agency at a parent's urging. The client has no close friends besides a sibling, dresses in mismatched layers and believes they can sense coworkers' moods through 'vibrations.' The client often thinks strangers' conversations are about them but agrees this may not be true. Unease around coworkers has not faded after 6 years at the same job. Which diagnosis is **MOST** likely?

A. Schizoid personality disorder
B. Schizophrenia
C. Schizotypal personality disorder
D. Avoidant personality disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Schizoid personality disorder also involves few close relationships, but without odd beliefs, magical thinking or eccentric behavior. The 'vibrations' and self-referential thinking point to schizotypal features.

B **Incorrect** (trap: Near-miss diagnosis). Schizophrenia requires fixed delusions, hallucinations or disorganization, usually with a decline in functioning. The client's self-referential beliefs are held loosely, and the client has kept the same job for 6 years.

C **Correct.** Schizotypal personality disorder combines discomfort with close relationships, odd beliefs or magical thinking, ideas of reference, eccentric appearance and social anxiety that does not ease with familiarity. The client shows each one: sensing 'vibrations,' self-referential thoughts held loosely, mismatched dress, and unease after 6 years with the same coworkers. It beats B because the client can doubt the self-referential thoughts, which marks them as ideas of reference rather than fixed delusions (DSM-5-TR).

D **Incorrect** (trap: Near-miss diagnosis). Avoidant personality disorder involves wanting relationships but fearing rejection, with feelings of inadequacy. It does not include magical thinking, ideas of reference or eccentric dress.

Takeaway: Odd beliefs, loosely held self-referential ideas, eccentricity and lasting social anxiety point to schizotypal personality disorder rather than psychosis.

Sources: DSM-5-TR (APA, 2022), Personality Disorders: Schizotypal Personality Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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25. ASM-1543 (rev 2) · Best answer: A

A 7-year-old in foster care has weekly visits with a birth parent, and reunification is the case goal. After each visit, the child cries, wets the bed and pushes the foster parents away for a day or two. The foster parents ask to reduce the visits. Visit notes from the past 2 months describe a warm, appropriate parent and no safety concerns. What should the social worker do **NEXT**?

A. Help the foster parents plan supportive routines around visits
B. Request fewer visits until the foster child's behavior settles
C. Ask the birth parent to avoid talk of reunification at visits

Why each option is right or wrong

A **Correct.** Distress before or after family time is a common response to separation and loss, not by itself a sign that contact is harmful, and federal guidance warns that cutting visits because of a child's negative reactions can deepen trauma (Children's Bureau, 2020). These visits are safe, and regular contact supports reunification. Coaching the foster parents to expect these reactions and build calming routines around visits helps the child manage grief and divided loyalties. It beats B because fewer visits would ease the foster home's strain while weakening the child's bond with the parent and the reunification plan.

B **Incorrect** (trap: Skips assessment). Fewer visits would ease the foster family's strain, but the child's reactions reflect grief and divided loyalties rather than unsafe contact.

Reducing family time on this basis can intensify the child's sense of loss and slow reunification.

C **Incorrect** (trap: Good but not best). Coaching the birth parent could help if visit content were upsetting the child, but the stem describes appropriate parent behavior. Limiting talk of reunification addresses an assumed cause instead of helping the child cope with repeated separations.

Takeaway: A child's distress around safe family visits usually reflects grief and loyalty conflict; support the child and caregivers rather than reducing contact.

Sources: Children's Bureau, U.S. Department of Health and Human Services (2020). Family Time and Visitation for Children and Youth in Out-of-Home Care, ACYF-CB-IM-20-02: children's reactions to family time
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.13 Effects of out-of-home placement (hospital, foster, residential, justice) · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 2 of 3

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26. ASM-1619 (rev 1) · Best answer: C

A school social worker has seen an 8-year-old for separation anxiety since the fall. The child now attends school every day, the teacher reports no morning distress for 6 weeks, and the parent handles drop-offs with the plan they practiced. The child says, 'I like coming here. Can we keep playing?' What is the **BEST** way to proceed?

A. Continue weekly sessions because the child enjoys them
B. End services at the next session to prevent dependence
C. Plan a gradual ending with fewer sessions over several weeks

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Enjoying sessions is not a treatment need. Continuing after goals are met serves the relationship rather than a clinical purpose and can suggest to the child and parent that the child cannot cope without help.

B **Incorrect** (trap: Good but not best). Ending is appropriate, but a sudden stop gives a young child little time to prepare and can feel like a loss. A short planned taper with a review of progress supports lasting gains better than an abrupt close.

C **Correct.** Readiness for termination shows when goals are met, gains hold over time and the client and family use their skills on their own (Hepworth et al.). Daily attendance, 6 weeks without morning distress and a parent who manages drop-offs meet that standard, and services should end when no longer needed (NASW Code 1.17(a)). Spacing out sessions lets the child practice coping with less support and makes ending a success rather than a loss. It beats B because stopping within a week gives a young child little time to prepare.

Takeaway: Once gains hold, taper toward a planned ending; a child's enjoyment of sessions is not a reason to continue, and abrupt stops feel like loss.

Sources: Hepworth, D. H., et al. Direct Social Work Practice: Theory and Skills (11th ed.). Cengage, termination: planned endings and readiness; NASW Code of Ethics (2021), 1.17(a)
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.6 Signs a client is ready for termination · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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27. ETH-0103 (rev 1) · Best answer: A

A hospital social worker notices that a colleague on the team has smelled of alcohol at 2 morning meetings and missed a patient appointment last week. The colleague is off today, and no patient has been harmed. What should the social worker do **FIRST**?

A. Talk privately with the colleague about the observed concerns
B. File a complaint about the colleague with the licensing board
C. Keep watching the colleague until the pattern is clearer

Why each option is right or wrong

A **Correct.** When a social worker has direct knowledge that a colleague's impairment is affecting practice, the Code's first step is to consult the colleague, when feasible, and help them take remedial action (NASW Code 2.08(a)). Here the social worker observed the signs personally, and no patient faces immediate danger, so a private conversation is feasible. It beats B because formal reporting comes later, if the colleague does not take adequate steps (NASW Code 2.08(b)). If a colleague arrived intoxicated to see patients, protecting patients through the supervisor would come first.

B **Incorrect** (trap: Right action, wrong time). A board complaint is an appropriate channel if the colleague does not address the problem after being approached. Going there first skips the direct conversation the Code calls for when no one faces immediate danger.

C **Incorrect** (trap: Ignores risk). The signs are already firsthand and affecting work, since an appointment was missed. Waiting for more evidence lets the problem grow and puts patients at risk. Observation without action does not meet the duty to colleagues and patients.

Takeaway: With direct knowledge of colleague impairment and no imminent danger, talk with the colleague first; use formal channels if that fails.

Sources: NASW Code of Ethics (2021), 2.08(a); NASW Code of Ethics (2021), 2.08(b)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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28. ASM-1609 (rev 1) · Best answer: B

A school social worker meets with the parents of a 14-year-old recently diagnosed with anorexia nervosa. The pediatrician has found the teen medically stable for outpatient care. The teen lives at home, and the parents are close to the teen and eager to help. The social worker has no training in eating disorder treatment. Which plan is **BEST**?

A. Begin weekly individual counseling with the teen at school
B. Refer the teen to a family-based treatment provider
C. Recommend a residential eating disorder treatment program

Why each option is right or wrong

A **Incorrect** (trap: Outside the social work role). Individual counseling by a social worker without eating disorder training is outside competence, and it leaves out the parents, who are central to adolescent recovery. School support can complement specialized treatment once it is in place.

B **Correct.** Modality is matched to the client's developmental stage and living situation. For adolescents with anorexia nervosa who live with involved parents, family-based treatment, in which parents temporarily take charge of restoring healthy eating, is a recommended approach (APA, 2023; Lock & Le Grange, 2013). The social worker lacks eating disorder training, so referral to a trained provider fits the competence standard (NASW Code 1.04(a)). It beats C because the teen is medically stable for outpatient care, and residential treatment would remove the teen from the parents who are central to recovery.

C **Incorrect** (trap: Good but not best). Residential care is for medical instability or for illness that has not responded to outpatient treatment. This teen is stable for outpatient care, so residential placement would be more restrictive than needed and would separate the teen from engaged parents.

Takeaway: For adolescents with anorexia nervosa and engaged parents, family-based treatment is a first-line modality; refer when it is outside your training.

Sources: American Psychiatric Association (2023). Practice Guideline for the Treatment of Patients With Eating Disorders, recommendations for adolescents with anorexia nervosa: family-based treatment; Lock, J., & Le Grange, D. (2013). Treatment Manual for Anorexia Nervosa: A Family-Based Approach (2nd ed.). Guilford Press, overview and phase 1: parents manage refeeding; NASW Code of Ethics (2021), 1.04(a)
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.3 Choosing intervention modality by ability, development, culture, life stage · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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29. ETH-1180 (rev 1) · Best answer: C

Months ago, a coworker privately told the social worker about a past psychiatric hospitalization. Now their shared supervisor, concerned about the coworker's late reports, asks the social worker, 'Is something going on with them that I should know about?' Nothing the social worker has seen points to risk for clients. What is the **BEST** response?

A. Share the history to help explain the late reports
B. Urge the coworker to tell the supervisor about the hospitalization
C. Advise the supervisor to raise the late reports with the coworker

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Wanting the coworker to get support is understandable, but the history was shared in confidence. Disclosing it without the coworker's permission breaches 2.02 and could expose the coworker to stigma at work.

B **Incorrect** (trap: Good but not best). Whether to tell a supervisor about a health history is the coworker's decision. Pressing the coworker to disclose it also assumes the hospitalization explains the late reports, which the social worker does not know.

C **Correct.** Social workers should respect confidential information shared by colleagues (2.02). The coworker disclosed the hospitalization privately, and nothing suggests client risk, so nothing overrides that confidence. The supervisor's legitimate concern is performance, which the supervisor can raise directly with the coworker, who then decides what to share. It beats A because a supportive motive does not make disclosing a colleague's health history appropriate. If impairment were harming clients, the impairment standard (2.08) would guide a different path.

Takeaway: Keep colleagues' confidences; redirect performance concerns to the colleague unless client safety is at risk.

Sources: NASW Code of Ethics (2021), 2.02; NASW Code of Ethics (2021), 2.08
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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30. INT-1016 (rev 1) · Best answer: C

Two months after moving into assisted living, a client with depression stays in their room most of the day. The client says, 'I'll join the activities once I feel like myself again.' The social worker is using cognitive behavioral therapy. Which intervention is **BEST** to start with?

A. Use thought records to track the client's negative thoughts
B. Explore the client's grief about the move to assisted living
C. Schedule one brief, enjoyable activity for each day

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Thought records are central to CBT and will likely help later. For a client this withdrawn, written cognitive work early on can feel overwhelming; activity scheduling builds momentum first.

B **Incorrect** (trap: Good but not best). Grief about the move may be part of the picture and deserves attention. Within CBT, though, waiting for feelings to change before acting is the pattern to break, and activity scheduling addresses it directly.

C **Correct.** In CBT for depression, behavioral activation often comes first for clients who are withdrawn and inactive. Scheduling small, rewarding activities lifts mood, and motivation tends to follow action rather than come before it (Beck, 2021). The client's plan to wait until feeling better keeps the cycle of withdrawal going. It beats A because thought records take energy and concentration that a markedly inactive client may lack; cognitive work is added as functioning improves.

Takeaway: For a withdrawn, depressed client waiting to feel motivated, CBT starts with behavioral activation because action comes before motivation.

Sources: Beck, J. S. (2021). Cognitive Behavior Therapy: Basics and Beyond (3rd ed.). Guilford Press, behavioral activation
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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31. ETH-1716 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

The Missing and Murdered Indigenous Women and Relatives (MMIW) movement chiefly calls attention to which problem?

A. Loss of Native languages caused by past boarding school policies
B. Disappearances and killings met by weak official response
C. Overrepresentation of Native children in foster care

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Boarding schools did suppress Native languages and are part of the historical trauma many Native communities carry. That history is distinct from the MMIW movement, which focuses on current violence and the official response to it.

B **Correct.** The MMIW movement, also called MMIWR or MMIP to include relatives and people, highlights the high rates at which Indigenous women and girls go missing or are killed, along with underreporting, racial misclassification and weak investigation (Urban Indian Health Institute, 2018). Social workers serving Native communities need this context to act quickly on missing-person concerns and to understand community grief and mistrust. It beats C because child welfare overrepresentation is a related injustice that the Indian Child Welfare Act addresses, not the focus of MMIW.

C **Incorrect** (trap: Wrong model or concept). Native children are overrepresented in foster care, a problem the Indian Child Welfare Act was passed to address (25 U.S.C. 1901(4)). The MMIW movement focuses instead on violence against Native women and relatives.

Takeaway: MMIW highlights disproportionate violence against Indigenous women and relatives and gaps in official response, context for urgent, culturally informed action.

Sources: Urban Indian Health Institute (2018). Missing and Murdered Indigenous Women & Girls: A snapshot of data from 71 urban cities in the United States, Findings: underreporting in law enforcement records; racial misclassification; Indian Child Welfare Act of 1978, 25 U.S.C. 1901(4)

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.7 Exploitation's cultural impact on marginalized communities (trafficking, MMIW) · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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32. ASM-1062 (rev 1) · Best answer: B

On a mental status exam, a client misperceives a real object, such as seeing a coat on a hook as an intruder. Which finding is this?

A. Visual hallucination
B. Illusion
C. Delusion
D. Derealization

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). A visual hallucination is a sensory perception without any external stimulus, such as seeing a person in an empty corner. Here a real coat is being misperceived.

B **Correct.** An illusion is a misperception of a real external stimulus: the coat is really there but is perceived as an intruder. It is recorded under perception and is common with poor lighting, fatigue, delirium or substance effects. It beats A because a hallucination is a perception with no external stimulus at all, so nothing real is being misread (Kaplan & Sadock).

C **Incorrect** (trap: Wrong model or concept). A delusion is a fixed false belief, a disturbance of thought content rather than perception. Misreading a coat in passing is a perceptual error, not a belief held against evidence.

D **Incorrect** (trap: Wrong model or concept). Derealization is a sense that the surroundings are unreal, dreamlike or distant. It changes how the whole environment feels, not the identity of one object.

Takeaway: Illusions misperceive a real stimulus; hallucinations occur without one; delusions are false beliefs, not perceptions.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Mental status examination: perception

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.10 Mental status exam: components and purpose · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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33. INT-0102 (rev 1) · Best answer: A

A patient with advanced lung cancer is continuing chemotherapy aimed at slowing the disease. A team now also visits to manage the patient's pain and breathlessness and to support the family's stress. Which type of care is the patient **MOST** likely receiving from this team?

A. Palliative care

B. Hospice care

C. Caregiver respite care

Why each option is right or wrong

A **Correct.** Palliative care relieves symptoms and stress and supports quality of life for people with serious illness at any stage, and it can be given alongside treatment aimed at the disease (National Consensus Project, 2018). The decisive detail is that the patient continues chemotherapy while the team manages symptoms and family stress. It beats B because hospice serves people with a prognosis of 6 months or less who choose comfort-focused care; under Medicare, electing hospice waives coverage for treatment of the terminal illness itself (42 CFR 418.3, 418.24).

B **Incorrect** (trap: Wrong model or concept). Hospice also manages symptoms and supports families, but it serves patients with a prognosis of about 6 months or less who have generally stopped treatment aimed at the disease. Ongoing chemotherapy points away from hospice.

C **Incorrect** (trap: Wrong model or concept). Respite care gives family caregivers short-term relief by providing temporary care for the patient. It does not describe a team that manages pain and breathlessness during active treatment.

Takeaway: Palliative care can accompany disease-directed treatment at any stage; hospice is comfort-focused care for a prognosis of about 6 months or less.

Sources: National Consensus Project for Quality Palliative Care (2018). Clinical Practice Guidelines for Quality Palliative Care (4th ed.). National Coalition for Hospice and Palliative Care, definition of palliative care; Medicare hospice regulations, 42 CFR Part 418, 42 CFR 418.3 (terminally ill); 418.24 (election and waiver)

Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.5 End-of-life practice: hospice, palliative care, continuity, dying stages · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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34. ETH-1602 (rev 1) · Best answer: A

PRETEST — unscored on this form; do not count it

A 79-year-old resident of an assisted living facility tells the social worker that the 'friend' who visits daily is the resident's same-sex partner of 35 years. Staff have left the partner out of care-planning meetings because the partner is not family. The resident says, 'When I was young, people lost their jobs over this.' What should the social worker do **FIRST**?

A. Ask the resident how to include the partner and what to share
B. Tell staff about the relationship so the partner is included
C. Help the resident name the partner as health care agent
D. Encourage the resident to tell staff for better support

Why each option is right or wrong

A **Correct.** Clients control disclosure of their sexual orientation, and many older lesbian and gay adults learned to hide relationships after decades of legal and social penalties. The resident has shared the relationship with the social worker but has not said what staff should know or what role the partner should have. Learning those wishes comes before any action (NASW Code 1.02; 1.05(d)). It beats C because naming a health care agent may be a sound next step, but choosing it now assumes the resident's goals and leaves the exclusion from care planning unaddressed.

B **Incorrect** (trap: Misreads law or ethics). Including the partner is the right goal, but telling staff about the relationship without the resident's consent takes a disclosure decision away from the resident. Advocacy with staff fits once the resident decides what may be shared.

C **Incorrect** (trap: Right action, wrong time). Naming the partner as health care agent can protect the couple's role and is often worth raising. Choosing it first presumes the resident's goals; the social worker should first learn how the resident wants the partner included.

D **Incorrect** (trap: Too directive). Coming out may bring support, but urging it imposes the social worker's view on a resident whose caution comes from real past penalties. The resident decides whether and when to disclose.

Takeaway: Before advocating for an LGBT client's partner, learn the client's wishes; the client controls whether and how their identity is shared.

Sources: NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.05(d)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.3 Culture, race, gender, sexuality, intersectionality shaping identity and behavior · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

35. INT-0118 (rev 2) · Best answer: C

A supervisee who usually welcomes feedback starts rejecting every suggestion the supervisor offers about one case, saying, 'That won't work either.' In that case, the client turns down each resource the supervisee proposes. The supervisee stays open to feedback on other cases. What is **MOST** likely happening?

A. Transference toward the supervisor
B. Countertransference toward the client
C. Parallel process
D. Burnout

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Transference toward a supervisor stems from the supervisee's earlier relationships with authority figures and would tend to color supervision broadly. Here the behavior appears on one case and copies that client's pattern.

B **Incorrect** (trap: Wrong model or concept). Countertransference is the supervisee's emotional reaction to the client, such as frustration. It may be present, but it does not explain why the supervisee is acting out the client's rejecting stance with the supervisor.

C **Correct.** In parallel process, a dynamic from the client relationship is unconsciously reenacted in the supervisory relationship (Searles, 1955). The client rejects every resource; the supervisee now rejects every suggestion, and only on this case. That specificity is the decisive clue. Recognizing the parallel lets the supervisor explore it with the supervisee, which can deepen understanding of the client's help-rejecting stance (Kadushin & Harkness, 2014). It beats A because transference toward the supervisor would draw on the supervisee's past relationships and show up across cases, not mirror one client's pattern.

D **Incorrect** (trap: Near-miss diagnosis). Burnout brings exhaustion, cynicism and reduced effectiveness that usually spread across a caseload. This supervisee stays open on other cases, which points to a case-specific dynamic.

Takeaway: When a supervisee treats the supervisor the way a specific client treats the supervisee, suspect parallel process and explore it.

Sources: Searles, H. F. (1955). The informational value of the supervisor's emotional experiences. Psychiatry, 18(2), 135-146, parallel process; Kadushin, A., & Harkness, D. (2014). Supervision in Social Work (5th ed.). Columbia University Press, parallel process in educational supervision

Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.3 Transference and countertransference in supervision · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

36. ASM-1032 (rev 1) · Best answer: C

A hospice patient, 81, with advanced pancreatic cancer has low mood, poor appetite, weight loss, fatigue and broken sleep. The hospice team asks the social worker whether the patient also has major depression. How can the social worker **BEST** judge whether a depressive disorder is present?

A. Count the appetite, sleep and energy changes as depressive symptoms
B. View the low mood as an expected reaction to dying
C. Focus on hopelessness, worthlessness and loss of pleasure

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). In advanced cancer these physical symptoms are often caused by the illness. DSM-5-TR excludes symptoms clearly due to another medical condition, so counting them would overstate depression.

B **Incorrect** (trap: Good but not best). Sadness and grief are common near death, but major depression is not a normal part of dying. Treating all low mood as expected can leave a treatable disorder, and its suffering, unaddressed.

C **Correct.** DSM-5-TR tells clinicians not to count symptoms clearly caused by another medical condition toward a major depressive episode. In advanced cancer, appetite loss, weight loss, fatigue and poor sleep are often caused by the illness, so they say little about depression. Hopelessness, worthlessness, excessive guilt and loss of pleasure in everything are more telling. It beats B because sadness near death is common, but pervasive hopelessness and worthlessness are not a normal part of dying and deserve treatment.

Takeaway: In serious medical illness, judge depression by psychological symptoms like hopelessness and worthlessness, not by symptoms the illness itself causes.

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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37. ETH-1158 (rev 1) · Best answer: C

A client living with bipolar I disorder tells the social worker about a public video they posted describing their first year back at work after a long hospitalization. The client says they would like the social worker to watch it and talk about it at the next session. What is the **MOST** appropriate response?

A. Explain that the Code bars viewing a client's online videos
B. Follow the client's channel to show support
C. View the video as invited and discuss it next session

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). The Code does not ban looking at client content; it requires consent or a compelling reason before an electronic search. A refusal here misreads the rule and may leave the client feeling that a hard-won milestone is unwelcome.

B **Incorrect** (trap: Boundary slip or collusion). Following the client's channel creates an ongoing online connection, and the Code advises against personal relationships with clients on social networking sites (1.06(h)). Support can be shown in session instead.

C **Correct.** The Code asks for client consent before an electronic search about a client (NASW Code 1.03(i)). Here the client has invited the social worker to view one specific public video and wants to discuss it, so consent is clear and the purpose is clinical. Watching it and processing it together can affirm the client's recovery. It beats A because refusing a clear, consented, treatment-related request is rigid and could feel like rejection. Viewing should stay limited to what the client shared, not a broader look at their accounts (Tech Standard 3.09).

Takeaway: With a client's clear invitation and a clinical purpose, viewing content they shared is ethical; keep it limited and process it in session.

Sources: NASW Code of Ethics (2021), 1.03(i); NASW Code of Ethics (2021), 1.06(h); NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 3.09
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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38. INT-1663 (rev 1) · Best answer: D

A client with depression has completed the PHQ-9 at the start of each session for 10 weeks. The social worker records the scores in the chart but has not discussed them with the client. The client now asks to stop, saying, 'I fill it out and never hear about it again.' What is the social worker's **BEST** response?

A. Stop the measure and rely on discussion in sessions
B. Explain why the scores matter and ask the client to continue
C. Switch to completing the measure once a month
D. Begin reviewing a graph of the scores with the client

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Clients may decline a measure, and that choice would be respected. Here the objection is that the results are never used, which the social worker can fix. Dropping the measure loses the clearest record of progress before the fix is tried.

B **Incorrect** (trap: Good but not best). Explaining the purpose is part of good monitoring, but the client's point is that nothing visible happens with the scores. More explanation without changing how they are used leaves the complaint unanswered.

C **Incorrect** (trap: Misuses the technique). Less frequent measurement lowers the burden but leaves the scores unused and makes changes harder to spot. The fix the client points to is using the results together, not filling out the form less often.

D **Correct.** Routine outcome monitoring helps when results are fed back and used; feedback reduces deterioration and improves outcomes, especially for clients who are off track (Lambert et al., 2018). The client's complaint names the problem: the scores go into the chart and nowhere else. Reviewing the trend together each session turns the measure into shared information for judging progress and adjusting the plan (Bloom, Fischer & Orme, 2009), and the client can still decline after seeing it used. It

beats A because stopping would discard 10 weeks of data when the objection is to unused scores, not to measurement itself.

Takeaway: Progress measures help when results are shared and used with the client; unused scores turn monitoring into a burden.

Sources: Lambert, M. J., Whipple, J. L., & Kleinstäuber, M. (2018). Collecting and delivering progress feedback: A meta-analysis of routine outcome monitoring. Psychotherapy, 55(4), 520-537, whole article; Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, visual analysis of single-system data: level and trend

Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.1 Evaluating progress and effectiveness of intervention and service plans · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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39. ETH-1040 (rev 1) · Best answer: B

In an outpatient session, a client says they will stab a former business partner tonight, gives the partner's home address, and shows a folding knife. The social worker urges voluntary hospitalization, but the client refuses and walks out. What should the social worker do **NEXT**?

A. Call the client tomorrow to reassess the threat

B. Notify law enforcement and warn the intended victim

C. Raise the case at the next team consultation

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Reassessment fits when a threat is vague or the client remains engaged. This client named a victim, a time and a weapon, then left after refusing help, so waiting a day leaves the partner unprotected tonight.

B **Correct.** A serious threat of violence against an identifiable person triggers a duty to take reasonable protective steps (Tarasoff, 1976). The client named the victim, set a time, showed a weapon and refused hospitalization, so the protective options left are notifying police and warning the target, which HIPAA permits for a serious and imminent threat (45 CFR 164.512(j)(1)(i)). B beats A because tonight's threat cannot wait for tomorrow's call. The worker then documents and follows the applicable state law.

C **Incorrect** (trap: Ignores risk). Consultation is valuable, but a scheduled team meeting comes too late for a threat set for tonight. Same-day consultation is fine only if it does not delay protective action.

Takeaway: When a client with a specific threat, victim and weapon leaves care, notify police and the intended victim without delay.

Sources: Tarasoff v. Regents of the University of California, 17 Cal. 3d 425 (1976); HIPAA Privacy Rule, 45 CFR 164.512(j)(1)(i)

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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40. ETH-1683 (rev 1) · Best answer: C

A client in private practice met every treatment goal for generalized anxiety 4 months ago, and symptom ratings have stayed low since. Recent sessions consist of friendly talk about the client's week. The client pays out of pocket and has explained, 'I just like having this hour.' The social worker's practice income has dropped this year. What should the social worker do **FIRST**?

A. Pick a date for the last session and inform the client
B. Continue weekly sessions, since the client values them
C. Raise ending with the client and review the gains made

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A final date belongs in the plan, but choosing it before discussing ending with the client skips the review of gains and the client's reactions. The date should come out of that conversation.

B **Incorrect** (trap: Boundary slip or collusion). The client's enjoyment is real, but the sessions no longer treat anything, and the worker's lost income makes continuing a self-interested choice. Ongoing sessions would fit if new treatment goals emerged.

C **Correct.** Services should end when they are no longer required or no longer serve the client's needs (NASW Code 1.17(a)). Goals were met months ago, and sessions have drifted into social time. The worker's falling income is a conflict of interest that makes continuing tempting, so the worker acts on the clinical picture (1.06(a)). Termination starts with a conversation that reviews gains and plans the ending together. It beats A because setting a date first turns a shared process into an announcement.

Takeaway: When goals are met and sessions no longer serve treatment needs, begin planned termination, especially when the worker benefits from continuing.

Sources: NASW Code of Ethics (2021), 1.17(a); NASW Code of Ethics (2021), 1.06(a)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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41. INT-0115 (rev 1) · Best answer: A

At the end of a 3-year grant, an agency analyzes whether participants in its supportive housing program stayed housed and made fewer emergency room visits. The board will use the results to decide whether to continue the program. Which type of evaluation is the agency conducting?

A. Summative evaluation
B. Formative evaluation
C. Needs assessment

Why each option is right or wrong

A **Correct.** Summative, or outcome, evaluation takes place at the end of a program or cycle and judges whether it achieved its intended outcomes, often to inform decisions about continuing or funding it (Royse, Thyer & Padgett, 2016). The timing (end of the grant), the focus on outcomes (housing stability and emergency visits) and the purpose (a continuation decision) all point to it. It beats B because formative evaluation happens while a program runs, to check delivery and improve it, rather than to judge final results.

B **Incorrect** (trap: Wrong model or concept). Formative, or process, evaluation is done during implementation to monitor fidelity and improve delivery. This agency is judging results at the end of the grant to make a continuation decision.

C **Incorrect** (trap: Wrong model or concept). A needs assessment comes before a program is designed, to identify problems and gaps in a population. It does not measure the results of a program that has already run.

Takeaway: Summative evaluation judges outcomes at the end to inform decisions; formative evaluation improves delivery midstream; needs assessment precedes design.

Sources: Royse, D., Thyer, B. A., & Padgett, D. K. (2016). Program Evaluation: An Introduction to an Evidence-Based Approach (6th ed.). Cengage, summative and outcome evaluation
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.4 Agency program evaluation: needs assessment, formative/summative, cost analyses · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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42. INT-1565 (rev 1) · Best answer: D

In a year-end report to a funder, which statement describes a program outcome rather than an output?

A. One hundred twenty caregivers attended 8 or more group sessions
B. The program held 48 support groups at 3 community sites
C. Two bilingual facilitators were hired and trained this year
D. Caregivers' average strain scores fell 25% by 6 months

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Attendance shows how many people received the service, which makes it an output. It shows reach but says nothing about whether caregivers' strain changed.

B **Incorrect** (trap: Wrong model or concept). The number of groups and sites describes the volume of activity delivered. It is an output or activity measure that belongs in the report, but it is not evidence of change.

C **Incorrect** (trap: Wrong model or concept). Hiring and training staff is an input or activity that builds program capacity. It shows the program is ready to deliver services, not that participants benefited.

D **Correct.** Outcomes are changes in participants' conditions, such as reduced stress or better functioning, while outputs count the services delivered (Kettner, Moroney & Martin, 2022). A 25% drop in caregiver strain measures change in the people served, so it belongs with outcomes. It beats A because attendance is the most outcome-like

distractor, yet it still counts participation rather than showing that anyone's situation improved.

Takeaway: Outputs count services delivered; outcomes measure change in participants. Funder reports need both, labeled correctly.

Sources: Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 10: Performance Measurement, Monitoring, and Program Evaluation

Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.8 Writing formal documents: grants, proposals, reports, evaluations · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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43. ETH-1132 (rev 1) · Best answer: A

Under 42 CFR Part 2, when state law lets a teen consent alone to substance use treatment, whose written consent is needed to share a progress update with the teen's parent?

A. The teen's consent alone
B. The parent's consent alone
C. Both teen and parent consent
D. The program director's approval

Why each option is right or wrong

A **Correct.** Under 42 CFR Part 2, when state law lets a minor obtain substance use treatment without a parent, only the minor may give written consent to disclose, including to a parent (42 CFR 2.14(a)). Here the teen may consent to treatment alone, so the update needs the teen's consent. A beats C because joint consent from teen and parent is the rule only where state law requires a parent's consent to treatment (2.14(b)), which is not the case here.

B **Incorrect** (trap: Misreads law or ethics). A parent's consent controls disclosure only when the parent had authority over the treatment decision. Here state law lets the teen consent alone, so the parent cannot authorize this disclosure.

C **Incorrect** (trap: Misreads law or ethics). Joint consent applies where state law requires a parent's consent to treatment (2.14(b)). This state lets the teen consent alone, so the teen alone controls disclosure.

D **Incorrect** (trap: Misreads law or ethics). A program director's judgment matters only in a narrow exception: a minor who lacks capacity and faces a substantial threat to life or physical well-being (2.14(c)). That exception is not the issue in a routine progress request.

Takeaway: Under Part 2, when state law lets a minor consent to SUD treatment alone, only the minor can authorize disclosure, even to a parent.

Sources: 42 CFR Part 2, 42 CFR 2.14(a); 42 CFR Part 2, 42 CFR 2.14(b); 42 CFR Part 2, 42 CFR 2.14(c)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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44. ASM-1640 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

Parents of a 23-year-old who lives at home ask for a family session. In 2 years, the young adult has changed jobs three times, switched college majors and moved in and out of a shared apartment. The young adult works part time, has close friends and says, 'I'm still figuring out what fits.' The parents worry something is wrong. Which response by the social worker is **BEST**?

A. Recommend vocational testing to settle on a career path
B. Suggest the parents set a firm date for the young adult to move out
C. Explain that such exploration is typical in the early twenties

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). Vocational testing can help a young adult who wants direction and may be useful later if the young adult asks for it. Offered now, it treats normal exploration as a problem and bypasses the parents' question about whether something is wrong.

B **Incorrect** (trap: Too directive). A move-out date is the parents' choice to make, but recommending it frames typical exploration as failure. It raises family conflict before the parents understand what is typical at this age.

C **Correct.** Arnett describes the late teens through the twenties as emerging adulthood, a distinct period marked by identity exploration, instability in work, love and housing, and feeling in-between. The young adult's job and major changes, close friendships and steady part-time work fit this typical pattern rather than a disorder. It beats A because career testing assumes the exploration is a problem to fix, while the parents first need accurate information about what is typical at this age (Arnett, 2000).

Takeaway: Exploration and instability in work, study and housing are typical in emerging adulthood; distinguish them from impairment before treating them as problems.

Sources: Arnett, J. J. (2000). Emerging adulthood: A theory of development from the late teens through the twenties. American Psychologist, 55(5), 469-480, whole article
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.16 Typical vs. atypical development across the lifespan · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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45. INT-1642 (rev 2) · Best answer: B

A client with schizophrenia lives in a supportive housing program. The landlord calls the social worker and says the client yelled at neighbors in the hallway on several nights this week. Staff have not seen this happen. How should the social worker record the call?

A. Record that the client has been aggressive toward neighbors
B. Record the landlord's report and name the landlord as its source
C. Leave out the landlord's secondhand report

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). This turns an unverified secondhand report into a staff conclusion and uses a label in place of the described behavior. Later readers could treat aggression as an established fact.

B **Correct.** Records should be accurate and include only information directly relevant to the delivery of services (NASW Code 3.04(a), (c)). A landlord's report bears directly on housing stability, so it belongs in the record, attributed to its source and described in behavioral terms, so readers can tell what staff observed from what others reported (Kagle & Kopels, 2008). It beats C because omitting a report that could threaten the client's housing removes information the team needs to plan, such as talking with the client and working to prevent an eviction.

C **Incorrect** (trap: Good but not best). Information does not need to be witnessed by staff to be relevant. Omitting a landlord's complaint leaves the team unaware of a threat to the client's housing that needs follow-up.

Takeaway: Record relevant third-party reports in behavioral terms and attribute them to their source, so observation and secondhand information stay distinct.

Sources: NASW Code of Ethics (2021), 3.04(a), (c); Kagle, J. D., & Kopels, S. (2008). Social Work Records (3rd ed.). Waveland Press, Content of records: sources of information

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.18 Case management: recording, documentation, case presentation · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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46. ASM-1540 (rev 2) · Best answer: C

A social worker leads a closed 12-week bereavement group for adults whose partners have died. In week 10, members are reviewing what they have learned and planning how to say goodbye. The intake coordinator asks the social worker to add a newly bereaved client who is struggling and on the waitlist. Which action is **BEST**?

A. Add the client now so they can gain support from the group

B. Ask the members to vote on whether to admit the client

C. Offer the client individual support until the next group starts

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Group support can help a grieving client, and open groups often add members. In a closed group two weeks from ending, a new member would join a group that is saying goodbye and would disrupt the members' work on separation.

B **Incorrect** (trap: Misuses the technique). Members' views matter, but a vote shifts a clinical and structural decision onto the group and can stir guilt or conflict during termination. The leader is responsible for decisions about a closed group's membership.

C **Correct.** Closed groups keep the same members from start to finish, which supports cohesion and lets members move through the group's stages together (Toseland & Rivas, 2017). In week 10, these members are doing ending work: reviewing gains and preparing to separate. A newly bereaved member would pull the group back to beginning tasks and would join a group about to end. It beats A because the new client's need is real, but individual support now, followed by a place in the next cycle, meets it without disrupting the current members' termination.

Takeaway: Closed groups keep stable membership; near termination, meet a new client's needs outside the group rather than disrupting the ending stage.

Sources: Toseland, R. W., & Rivas, R. F. (2017). An Introduction to Group Work Practice (8th ed.). Pearson, planning the group: open and closed membership
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.3 Relationship dynamics and development: families, couples, groups, polyamory · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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47. INT-1540 (rev 3) · Best answer: C

A 15-year-old has received wraparound services for 3 months to avoid residential placement. After a second school suspension, the school asks the team to recommend a residential program. The parent and youth want to keep working on the home-based plan. Which response by the wraparound facilitator is **MOST** consistent with wraparound principles?

A. Ask the school to hold its request until the plan's next review
B. Add a separate anger management program while the plan continues
C. Reconvene the team with the family to revise the plan

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Asking for time protects the home-based plan the family wants, so it is tempting. It waits for a scheduled review, though, instead of responding to the setback now; wraparound treats a suspension as a reason to bring the team, including the school, together promptly to revise strategies.

B **Incorrect** (trap: Good but not best). Anger management might become part of the plan, but adding it outside a team meeting bypasses family choice and joint planning. In wraparound, strategies are chosen together and fitted to this youth.

C **Correct.** Wraparound is unconditional: the team does not give up on or reject a family when things go wrong but treats setbacks as a reason to revise the plan, and family voice and choice carry the most weight (Bruns, Walker et al., 2008). The parent and youth want to continue, so the facilitator brings the team, including the family and school, back together to rework strategies after the suspension. It beats B because adding a service may help, but choosing it outside the team bypasses family voice and the joint, individualized planning wraparound requires.

Takeaway: In wraparound, setbacks lead the team and family to revise the plan rather than end home-based care or add services unilaterally.

Sources: Bruns, E. J., Walker, J. S., & NWI Advisory Group (2008). Ten principles of the wraparound process. The Resource Guide to Wraparound. National Wraparound Initiative, Principle 1: Family voice and choice; Principle 9: Unconditional
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.22 Discharge planning, aftercare, wraparound, follow-up · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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48. ETH-1104 (rev 1) · Best answer: C

A hospital social worker helped a patient begin a disability benefits appeal and said the filing deadline was 30 days away. A week later, rereading the denial letter, the social worker sees the deadline is actually 3 days away. The patient was discharged home yesterday and has a working phone. What should the social worker do **FIRST**?

A. Tell the supervisor and wait for direction on the patient
B. Correct the deadline date in the patient's chart
C. Call today with the correct date and help submit it

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Supervisors should learn of errors that affect clients, but waiting for direction risks missing a 3-day deadline. Informing the supervisor fits right after the patient has the correct date and a plan to file.

B **Incorrect** (trap: Good but not best). The correction belongs in the record, but a chart note does nothing to help the patient file on time. Documentation follows the call to the patient.

C **Correct.** The Integrity principle calls for honesty and responsibility, which includes telling clients about one's own errors that affect them, and client well-being is the primary responsibility (NASW Code 1.01; Integrity principle). The deadline is 3 days away, so the patient needs the correct date and help filing now. C beats A because waiting for supervisory direction could cost the appeal; the supervisor should be told promptly, but not before the patient knows. Concealing the error would also conflict with the bar on deception (4.04).

Takeaway: When a social worker's own error threatens a client's interests, tell the client promptly and help fix it before other steps.

Sources: NASW Code of Ethics (2021), Ethical Principles: Integrity; NASW Code of Ethics (2021), 1.01; NASW Code of Ethics (2021), 4.04
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.5 Core values: competence, social justice, integrity, dignity and worth · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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49. ETH-1633 (rev 3) · Best answer: A

After a budget cut eliminated a paid peer specialist position, a recovery agency asks 3 clients in long-term recovery to serve as volunteer peer mentors for 20 hours a week each. They would do the duties the peer specialist once did. The clients are eager to help. The social worker sits on the planning committee. What is the **BEST** recommendation?

A. Seek funds to pay the mentors a fair wage
B. Accept the clients' offer to serve as volunteers
C. Thank the mentors with gift cards each month

Why each option is right or wrong

A **Correct.** The Code's Social Justice principle directs change efforts at poverty and toward equal access to resources and opportunity (NASW Code, Ethical Principles: Social Justice), and the Code bars taking unfair advantage of any professional relationship (1.06(b)); fair pay for work is part of economic justice. The clients would fill a staff job at 20 hours a week without pay, and their status as clients makes it hard to refuse or bargain. Proposing funding for paid roles treats their lived expertise as valuable labor. It beats B because willingness does not make unpaid replacement of a paid job fair, and the clients' reliance on the agency weakens the freedom of their offer.

B **Incorrect** (trap: Boundary slip or collusion). Honoring clients' wish to give back has value, and brief, optional volunteering can support recovery. Here the role replaces a paid job at 20 hours a week, and clients may feel unable to say no to the agency that serves them.

C **Incorrect** (trap: Good but not best). Gift cards show appreciation but fall far short of the value of a staff role, so the agency still gains a job's worth of work for little cost. Token thanks fit occasional volunteering, not ongoing staff duties.

Takeaway: Economic justice means fair pay for work; agencies should not fill paid roles with unpaid clients whose reliance limits their freedom to refuse.

Sources: NASW Code of Ethics (2021), 1.06(b); NASW Code of Ethics (2021), Ethical Principles: Social Justice
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** BEST · **Cognitive** application · **Difficulty** 3 of 3

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50. INT-1190 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

A social worker leads an illness management group at a community mental health center for adults with schizophrenia. During a session on stress and symptoms, a member says, 'This is a waste of time. I'm not sick.' Earlier, the member said they want to keep their apartment and get a job. What is the **BEST** response?

A. Review the member's symptoms with the group to build insight
B. Note the member's view and continue with the planned material
C. Connect the skills to what the member wants for work and housing
D. Suggest individual therapy to work on accepting the diagnosis

Why each option is right or wrong

A **Incorrect** (trap: Too directive). Pressing symptom evidence in front of peers is likely to shame the member and harden their position. Lack of insight in schizophrenia often reflects the illness itself, so confrontation rarely produces agreement.

B **Incorrect** (trap: Good but not best). Acknowledging the member's view is a good start, and the group can move on. Without linking the content to something the member values, though, the session stays irrelevant to them and engagement fades.

C **Correct.** When a person does not see themselves as ill, arguing about the diagnosis usually deepens resistance. Amador's LEAP approach instead listens, empathizes, finds points of agreement and partners on the person's own goals, and illness management and recovery work ties information to personal recovery goals (Amador, 2000;

SAMHSA IMR, 2009). This member wants an apartment and a job, so framing stress skills as tools for those goals keeps them engaged without a fight over labels. It beats B because continuing as planned is respectful but gives the member no reason to use the content.

D **Incorrect** (trap: Hands off too soon). Individual work may help some members, but sending the member away to accept the diagnosis treats agreement with the label as the goal and hands off a problem the group can address now.

Takeaway: When a client rejects a diagnosis, deliver psychoeducation through the client's own goals instead of debating the label.

Sources: Amador, X. (2000). I Am Not Sick, I Don't Need Help! Vida Press, LEAP: listen, empathize, agree, partner; SAMHSA (2009) Illness Management and Recovery Evidence-Based Practices KIT, Recovery goals as the frame for psychoeducation

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.14 Delivering psychoeducation · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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51. ETH-1544 (rev 1) · Best answer: D

In a state where medical aid in dying is legal and the hospital permits it, a patient with terminal pancreatic cancer asks the palliative care social worker how to request it. The patient is calm and describes long-held views about dying on their own terms. What is the **MOST** appropriate response?

A. Refer the question to the hospital chaplain

B. Request an urgent suicide risk evaluation

C. Encourage the patient to focus on comfort care instead

D. Discuss the request and involve the physician

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). Spiritual care may help, and the patient can be offered it. Handing off the question entirely, though, avoids the social worker's own role in end-of-life decisions and leaves the patient without help with a legal process.

B **Incorrect** (trap: Skips assessment). Treating a lawful, consistent request from a terminally ill patient as a suicide emergency misreads the situation and can feel stigmatizing. Capacity and depression review is built into the legal process; an urgent evaluation fits signs of crisis, such as a hidden plan.

C **Incorrect** (trap: Overrides the client's choice). Steering the patient toward comfort care imposes the social worker's preference on a decision that belongs to the patient. Comfort care should be offered alongside the request, not as a substitute for discussing it.

D **Correct.** Where medical aid in dying is legal, social workers discuss it in line with the governing law, support the patient's informed choice and work with the team (NASW Standards for Palliative and End of Life Care, Standard 1; NASW Code 1.02). The patient is terminally ill, calm and consistent with long-held values, and the process runs through the physician. It beats B because urgent suicide evaluation fits signs of a mental health crisis, not a lawful, consistent request; eligibility review, including capacity, is part of the legal process.

Takeaway: Where aid in dying is legal, support the patient's informed choice and work through the physician-led legal process rather than steering or deflecting.

Sources: NASW Standards for Palliative and End of Life Care, Standard 1: Ethics and Values; NASW Code of Ethics (2021), 1.02

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.6 Law and ethics around death and dying · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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52. ASM-0109 (rev 1) · Best answer: A

In an integrated primary care clinic, a client who lost their job 3 months ago meets with the social worker about stress. The client says, 'My family would be better off without me,' and mentions giving a prized guitar to a friend last week. The client then changes the subject to money worries. What should the social worker do **FIRST**?

A. Ask the client directly about thoughts of suicide

B. Arrange an emergency psychiatric evaluation today

C. Reflect the client's feeling of being a burden

Why each option is right or wrong

A **Correct.** Indirect statements about being a burden, combined with giving away a valued possession, are warning signs that call for a direct question about suicidal thoughts. Asking does not plant the idea or raise risk (Dazzi et al., 2014), and a structured inquiry into ideation, plan, intent and means guides the response (Posner et al., 2011). It beats C because reflecting the feeling is supportive but lets the change of subject stand and leaves the risk unassessed. Reflection can lead into the direct question.

B **Incorrect** (trap: Skips assessment). An emergency evaluation is right when assessment shows a plan, intent or imminent danger. Nothing about plan or intent is known yet, so arranging it now escalates before the facts are gathered and may damage trust.

C **Incorrect** (trap: Good but not best). Reflecting the feeling of being a burden shows empathy and can open the door, but on its own it does not find out whether the client is thinking of suicide. With these warning signs, the risk question needs to be asked plainly.

Takeaway: Indirect suicidal statements or warning signs call for a direct question about suicidal thoughts before choosing any intervention.

Sources: Posner, K., et al. (2011). The Columbia-Suicide Severity Rating Scale: Initial validity and internal consistency findings from three multisite studies with adolescents and adults. American Journal of Psychiatry, 168(12), 1266-1277, whole article; Dazzi, T., Gribble, R., Wessely, S., & Fear, N. T. (2014). Does asking about suicide and related behaviours induce suicidal ideation? What is the evidence? Psychological Medicine, 44(16), 3361-3363, whole article

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.1 Assessing risk of harm to self or others · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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53. ETH-1679 (rev 1) · Best answer: B

A 45-year-old client with no dependents receives a notice that food benefits will stop after 3 months because the client is not working or in a work program 80 hours a month. The client has severe major depressive disorder, sleeps most of the day and has not held a job in a year. The client's psychiatrist confirms the client cannot work now. What should the social worker do **FIRST**?

A. Help the client search for part-time jobs that meet the hour rule
B. Help the client request a medical exemption from the time limit
C. Write to state legislators about the harm of the work rule
D. Refer the client to food pantries to replace the lost benefits

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Job searching runs against the psychiatrist's judgment that the client cannot work now, and a failed job could deepen the depression. Work-focused planning fits later, if recovery makes employment realistic.

B **Correct.** When a policy threatens a client's benefits, the first move is to use the protections the policy itself contains, then advocate for change (NASW Code 1.01; 6.04(a)). Federal SNAP rules exempt people certified as physically or mentally unfit for employment from the time limit for adults without dependents, and a physician's statement can supply that certification (7 CFR 273.24(c)(2)). The psychiatrist's confirmation makes this request strong. It beats D because a pantry only partly replaces benefits the client likely qualifies to keep.

C **Incorrect** (trap: Right action, wrong time). Advocating for policy change is part of the social work role and may follow, especially if other clients face the same rule. It will not keep this client's benefits from ending in 3 months.

D **Incorrect** (trap: Good but not best). Food pantries can bridge a gap and are worth mentioning. They do not replace a benefit the client likely qualifies to keep, and relying on them first gives up the exemption the rules provide.

Takeaway: When a policy threatens a client's benefits, first use the policy's own exemptions and appeals; then advocate for change.

Sources: NASW Code of Ethics (2021), 1.01; NASW Code of Ethics (2021), 6.04(a); SNAP time limit regulations, 7 CFR 273.24(c)(2)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.8 How local, state, federal policy affects service delivery · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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54. INT-1085 (rev 2) · Best answer: C

A behavioral health clinic serving a tribal nation offers a weekly PTSD skills group that few people attend. Community members tell the social worker that their grandparents were taken to boarding schools and that outside agencies have rarely kept their promises. Several elders lead traditional healing gatherings that are well attended. What is the **BEST** step to make the clinic more trauma-informed?

A. Ask the elders to promote the PTSD group at gatherings
B. Hire tribal community staff to lead the clinic's PTSD group
C. Invite the elders to help shape the clinic's services

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Endorsement by elders may raise attendance, but it uses their trust to promote a model they had no part in shaping. After historical trauma and broken promises, the community needs a voice in what is offered, not just publicity.

B **Incorrect** (trap: Good but not best). Hiring community members can build trust and is a real trauma-informed strategy. Placing them in the existing PTSD group, though, keeps a service the community had no part in choosing. After broken promises, and with traditional gatherings already well attended, the community needs a share in deciding what is offered.

C **Correct.** SAMHSA's trauma-informed principles include attention to cultural, historical and gender issues: recognizing historical trauma and drawing on the healing value of traditional cultural connections (SAMHSA, 2014). Collective losses such as boarding-school removals, and broken promises by outside agencies, explain low trust in a clinic-run group (Brave Heart, 2003). Partnering with respected elders who already lead healing gatherings builds trust and fits services to the community. It beats A because asking elders to promote the existing group borrows their trust without sharing decisions; collaboration means the community helps decide what is offered.

Takeaway: Trauma-informed care addresses historical trauma by partnering with community leaders and drawing on traditional healing practices.

Sources: SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), six key principles: cultural, historical, and gender issues; Brave Heart, M. Y. H. (2003). The historical trauma response among Natives and its relationship with substance abuse: A Lakota illustration. Journal of Psychoactive Drugs, 35(1), 7-13, whole article

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.1 Trauma-informed care across trauma types (complex, vicarious) · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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55. ETH-1501 (rev 1) · Best answer: A

At the last of 10 sessions at a community clinic, a client gives the social worker a small jar of homemade jam. The client explains that in their family, sharing food is how people show gratitude. What is the **MOST** appropriate response?

A. Accept the jam and express appreciation

B. Decline the jam, explaining that gifts blur roles

C. Suggest the client share the jam with clinic staff

Why each option is right or wrong

A **Correct.** Small, low-value gifts with cultural meaning can generally be accepted. The Code has no gift ban; gifts are weighed under the conflict-of-interest standard by value, timing and meaning (NASW Code 1.06(a)). A jar of jam at the final session is a modest, culturally rooted thank-you with no risk of exploitation, and accepting it graciously honors the client and the ending. It beats C because politely redirecting the gift still sidesteps it and can read as rejection at a sensitive moment (Reamer, 2020).

B **Incorrect** (trap: Misreads law or ethics). Refusing treats every gift as a boundary violation, which the Code does not require. Turning down a small, culturally

meaningful token at termination can shame the client and undo the goodwill of a good ending. Declining fits money or valuable gifts.

C **Incorrect** (trap: Good but not best). Redirecting the gift to staff avoids accepting it personally, but the client offered it to the social worker as a personal thank-you. The polite deflection can feel like rejection and misses the cultural meaning of the gesture.

Takeaway: Small, culturally meaningful gifts can usually be accepted graciously; refusing every gift can harm the relationship.

Sources: NASW Code of Ethics (2021), 1.06(a); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, gifts from clients
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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56. INT-1051 (rev 2) · Best answer: A

A 14-year-old with anorexia nervosa is discharged home after medical stabilization, with weekly weight checks by a pediatrician. The family begins family-based treatment with the social worker. In the second session, the parents say they want to let their teen choose and plan their own meals so that eating does not become a battle. Which response is **BEST**?

A. Coach the parents to plan and supervise every meal for now
B. Support the parents' plan to let the teen choose meals
C. Start individual sessions on the teen's need for control

Why each option is right or wrong

A **Correct.** In family-based treatment for adolescent anorexia nervosa, parents take charge of restoring weight in the first phase, because the illness, not the teen, is driving food choices. Control over eating returns to the adolescent in stages once weight is restored (Lock & Le Grange, 2013). The parents' wish to avoid conflict is understandable, so the social worker supports them while coaching them to stay in charge. It beats B because autonomy over meals is the right goal for a later phase; offered two sessions after discharge, it hands decisions back to the illness while weight is still low.

B **Incorrect** (trap: Right action, wrong time). Returning control of eating to the adolescent is a planned step in family-based treatment, but it comes after weight is largely restored. Two sessions after discharge, letting the teen plan meals risks renewed weight loss and medical danger.

C **Incorrect** (trap: Wrong model or concept). Individual insight-oriented work is an alternative model for some adolescents, but this family has begun family-based treatment, which keeps the parents central. Shifting the focus to the teen's need for control would sideline the parents when they are most needed.

Takeaway: In family-based treatment for adolescent anorexia, parents first take charge of meals and weight restoration; control returns to the teen as health recovers.

Sources: Lock, J., & Le Grange, D. (2013). Treatment Manual for Anorexia Nervosa: A Family-Based Approach (2nd ed.). Guilford Press, Phase 1: parents take charge of weight restoration
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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57. ASM-1527 (rev 2) · Best answer: C

An employee assistance client tells the social worker that their widowed 78-year-old parent will move into the client's home next month. The parent drives, manages their own medications and volunteers weekly. The client's two teenagers will have to share a bedroom and are openly resentful, and the client's partner worries about privacy. What should the social worker recommend **FIRST**?

A. An in-home assessment of the parent's daily functioning
B. Individual counseling for the teenagers about their resentment
C. A meeting of the three generations to plan roles and space

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). An in-home functional assessment fits when an older adult's ability to manage daily tasks is in doubt. This parent is independent, so the assessment would not address the conflict over space and privacy that the household is already showing.

B **Incorrect** (trap: Hands off too soon). The teenagers' resentment is real, but it is one part of a household-wide change. Sending them to individual counseling treats a family reorganization as a personal problem and leaves the parent and partner out of the solution.

C **Correct.** A multigenerational move reorganizes the whole household. Family life-cycle theory notes that later-life transitions require families to shift generational roles and make room for elders without overfunctioning for them (Carter & McGoldrick, 2005). Bringing all three generations together before the move lets the teenagers, partner and parent negotiate space, privacy and household roles. It beats A because this parent drives, manages medications and volunteers, so the issue is family reorganization, not care needs; assessing functioning first would delay the conversation the family needs.

Takeaway: When an older parent joins a household, help all generations negotiate roles, space and routines before the move, not one member alone.

Sources: Carter, B., & McGoldrick, M. (Eds.) (2005). The Expanded Family Life Cycle: Individual, Family, and Social Perspectives (3rd ed., classic ed.). Allyn & Bacon, Ch. 1, Table 1.1 (p. 2): families in later life, second-order changes

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.18 Aging's impact on individuals and family systems · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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58. ASM-1040 (rev 1) · Best answer: A

During a home visit 7 weeks after delivery, a parent tearfully says they keep getting sudden images of the baby falling down the stairs. The images horrify them, so they now avoid carrying the baby near the staircase. The parent also reports feeling worthless and sleeping poorly even when the baby sleeps. What should the social worker do **FIRST**?

A. Assess the thoughts, mood, safety and any intent to act
B. Refer the parent to a perinatal mental health specialist
C. Explain that such unwanted thoughts are common after birth

Why each option is right or wrong

A **Correct.** Unwanted, horrifying images of accidental harm to an infant are common after birth; when the parent is distressed by them and avoids acting on them, they usually signal anxiety or depression rather than danger (Fairbrother & Woody, 2008). Worthlessness and poor sleep suggest a depressive episode, which also calls for checking suicidal thinking (DSM-5-TR). Assessing the thoughts, mood, intent and safety comes first. It beats C because reassurance fits only after the social worker confirms the thoughts are unwanted and finds no intent, psychosis or suicidal thinking.

B **Incorrect** (trap: Right action, wrong time). A perinatal specialist referral is likely the right step soon. Making it before assessing intent, psychotic symptoms and suicidal thinking skips the information that decides how urgent the referral must be.

C **Incorrect** (trap: Skips assessment). Normalizing unwanted thoughts helps many new parents, but it should follow an assessment. Reassuring first assumes the thoughts are harmless before ruling out intent, psychosis or suicidal thinking.

Takeaway: When a new parent reports intrusive harm images, assess the thoughts, mood and intent before reassuring, referring or reporting.

Sources: Fairbrother, N., & Woody, S. R. (2008). New mothers' thoughts of harm related to the newborn. Archives of Women's Mental Health, 11(3), 221-229, whole article; DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.5 Signs of mental and emotional illness across the lifespan · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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59. ASM-1158 (rev 2) · Best answer: B

An emergency department asks the social worker to see a young adult patient on a fourth visit this year for days of repeated vomiting. Staff wonder about an eating disorder. Labs and imaging have been normal. The patient describes smoking cannabis several times a day for years, taking long hot showers that ease the nausea, and smoking more to settle the stomach. What should the social worker do **NEXT**?

A. Tell the patient that cannabis is causing the vomiting

B. Share the use and bathing history with the physician

C. Refer the patient for an eating disorder evaluation

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Cannabis education will matter once the physician confirms the cause, so this step belongs later. Stating the cause to the patient now, before the medical team has weighed the history, amounts to a diagnosis, which is outside the social work role.

B **Correct.** Social workers gather history but do not diagnose medical conditions. Years of daily cannabis use, repeated vomiting episodes and relief from hot showers form the typical pattern of cannabinoid hyperemesis syndrome, which stopping cannabis usually resolves (Sorensen et al., 2017). Because patients often use more cannabis to ease the nausea, the pattern is easy to miss. It beats A because the information is likely accurate, but the diagnosis belongs to the physician, who can confirm it and then explain it to the patient.

C **Incorrect** (trap: Skips assessment). Eating disorders can involve vomiting, but the history the patient gave points to a substance-related cause. Referring now acts on staff's first guess and sets aside information the medical team should weigh first.

Takeaway: Pass substance-use history that may explain physical symptoms to the medical team; diagnosis belongs to the physician.

Sources: Sorensen, C. J., DeSanto, K., Borgelt, L., Phillips, K. T., & Monte, A. A. (2017). Cannabinoid hyperemesis syndrome: Diagnosis, pathophysiology, and treatment, a systematic review. Journal of Medical Toxicology, 13(1), 71-87, Clinical features; treatment

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.8 Addiction and substance use: signs and impact on systems · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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60. INT-1178 (rev 2) · Best answer: C

PRETEST — unscored on this form; do not count it

During a first meeting at a community agency, a social worker leans forward in an open posture to show interest. Each time the social worker leans in, the client slides back in the chair, folds their arms, and gives shorter answers. How should the social worker respond?

A. Shift to less personal questions for now

B. Point out the folded arms and ask what they mean

C. Sit back and allow more personal space

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Lighter topics can ease a guarded client, but this client pulls back each time the worker leans in, not when a topic turns personal. Changing the questions misreads the cue and leaves the closeness that feels intrusive in place.

B **Incorrect** (trap: Too directive). Naming nonverbal behavior can be useful later in a relationship. In a first meeting it can feel like being watched and judged, and it asks the client to explain a reaction the worker can simply adjust to.

C **Correct.** Attending guidelines such as Egan's SOLER, which includes leaning toward the client, are meant to be used flexibly and adjusted to each client's comfort and culture (Egan, 2014). Here the client pulls back, closes up and shortens answers each time the worker leans in, a clear sign that the closeness feels intrusive. Matching the client's preferred distance shows respect without words. It beats B because spotlighting the client's body language in a first meeting can feel exposing and invites defensiveness before trust exists.

Takeaway: Attending behaviors serve the client's comfort; when a client withdraws from closeness, adjust posture and distance instead of persisting or interpreting.

Sources: Egan, G. (2014). The Skilled Helper (10th ed.). Brooks/Cole, Cengage Learning, Attending: SOLER as flexible guidelines

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.2 Verbal and nonverbal communication; congruence, body language · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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61. ETH-1638 (rev 1) · Best answer: B

A county child welfare agency and two tribal nations agree to a joint truth-and-reconciliation effort about decades of Native children placed in non-Native foster homes. Agency leaders want to open with a formal apology and a list of reforms drafted by agency staff. The social worker on the planning group is asked for input. Which opening step is **MOST** consistent with this kind of effort?

A. Release the agency's apology and reform list at the first session
B. Gather statements from affected families, with tribes co-leading
C. Have agency staff review old case files to find errors

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Acknowledgment and apology are part of reconciliation and may come early in some efforts. Issued before affected families have spoken, an apology and agency-written reforms define the harm from the agency's view and risk seeming hollow.

B **Correct.** Truth and reconciliation work starts by hearing and documenting the experiences of those harmed, under shared leadership with the affected nations; reconciliation and reform follow from that truth. The Maine Wabanaki-State Child Welfare Truth and Reconciliation Commission gathered statements from Wabanaki people and child welfare staff before making recommendations (Maine Wabanaki-State TRC, 2015). It beats A because an apology and reforms written before families speak let the agency define both the harm and the remedy. Canada's Commission likewise called for child welfare workers to be trained in the history it documented (TRC of Canada, 2015, Call 1(iii)).

C **Incorrect** (trap: Good but not best). File review can document patterns and support truth-telling. Done alone by agency staff, it keeps the agency in control of the account and leaves out the families and tribal nations who lived the history.

Takeaway: Truth and reconciliation begins with hearing those harmed, under shared leadership; apologies and reforms should follow from that truth.

Sources: Maine Wabanaki-State Child Welfare Truth and Reconciliation Commission (2015). Beyond the Mandate: Continuing the Conversation, statement gathering and recommendations; Truth and Reconciliation Commission of Canada (2015). Calls to Action, Call to Action 1(iii)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.8 Restorative practice, truth and reconciliation with historically oppressed groups · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 3 of 3

Section 2

62. INT-0114 (rev 1) · Best answer: C

A school social worker plans to use a token reward system to reduce a 7-year-old's classroom outbursts. The social worker wants data showing whether the system works. What should the social worker do **FIRST**?

A. Start the token system and count outbursts weekly
B. Ask the teacher to rate progress at the end of the term
C. Count the outbursts daily for a period before starting

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Tracking during the intervention is part of the design, but starting the token system first skips the baseline. Without pre-intervention data, the social worker cannot tell whether outbursts changed. Skipping a baseline is reserved for crises in which delay is unsafe.

B **Incorrect** (trap: Good but not best). A single end-of-term rating is subjective and gives no pattern over time or comparison with how things were before. Teacher input is useful as repeated, specific counts rather than one summary rating.

C **Correct.** Single-system evaluation begins with a baseline phase: repeated measurement of the target behavior before intervention, so later change can be compared with the starting pattern (Bloom, Fischer & Orme, 2009). Counting outbursts daily for a period, ideally until the pattern is stable, provides that comparison. It beats A because starting the intervention first leaves no record of how often outbursts happened before, so later counts have nothing to be measured against.

Takeaway: To evaluate an intervention with one client, measure the target behavior repeatedly before starting (baseline), then keep measuring.

Sources: Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, baseline phase; AB design
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.1 Evaluating progress and effectiveness of intervention and service plans · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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63. ASM-0103 (rev 2) · Best answer: C

A primary care provider refers a 74-year-old client for memory concerns that began about 2 months ago. The client says, 'My mind is gone,' and answers 'I don't know' to many screening questions. The client also reports poor sleep, weight loss, lost interest in gardening and feeling like a burden. The client still manages bills and medications, alertness is steady and medical tests are normal. What is the **MOST** likely explanation?

A. Major neurocognitive disorder
B. Delirium
C. Major depressive disorder
D. Mild neurocognitive disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Major neurocognitive disorder requires decline that interferes with independence, such as needing help with bills or medications. This client handles both. People with Alzheimer's disease also tend to minimize deficits rather than stress them.

B **Incorrect** (trap: Near-miss diagnosis). Delirium develops over hours to days and involves fluctuating attention and awareness, usually with a medical cause. Steady alertness, a 2-month course and normal medical tests make it unlikely. A sudden change would call for urgent medical evaluation.

C **Correct.** Depression in older adults often presents as memory complaints. Clues include a fairly recent, datable onset, emphasis on deficits, poor effort on testing ('I don't know') and other depressive symptoms (DSM-5-TR). This client also has loss of interest, insomnia, weight loss and feelings of worthlessness. It beats D because mild neurocognitive disorder involves modest measured decline not better explained by another disorder, and DSM-5-TR lists depression as a key differential. Treating the depression and then rechecking cognition is the standard path.

D **Incorrect** (trap: Near-miss diagnosis). Mild neurocognitive disorder fits modest decline with preserved independence, so it is tempting. But a full depressive syndrome with a datable onset better explains the complaints. Cognition should be rescreened once the depression is treated.

Takeaway: In older adults, recent memory complaints with depressive symptoms and poor effort on testing suggest depression; treat it and reassess cognition.

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder; DSM-5-TR (APA, 2022), Neurocognitive Disorders: Major and Mild Neurocognitive Disorders (differential diagnosis)
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.5 Signs of mental and emotional illness across the lifespan · **Qualifier** MOST LIKELY · **Cognitive** application · **Difficulty** 3 of 3

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64. ETH-1129 (rev 1) · Best answer: B

A client attends court-ordered outpatient therapy after an assault conviction. The client signed a release allowing the social worker to report attendance and participation to the probation officer. The officer calls and asks what the client has said in sessions about a new dating relationship. What is the **MOST** appropriate response?

A. Describe what the client said, since the court ordered treatment
B. Report attendance and participation, and decline to share more
C. Decline to report attendance or participation until a subpoena arrives

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). A court order to attend treatment is not consent to reveal what is said in sessions. The client authorized attendance and participation reports only, and this request goes well past that.

B **Correct.** A release defines what may be shared, and disclosure stays within that scope and the least amount needed (NASW Code 1.07(b), 1.07(c)). This client authorized reports of attendance and participation only; what the client said about a relationship falls outside it. The officer can seek more through the client's consent or the court. B

beats C because the client did authorize some disclosure, and refusing all of it could hurt the client's standing with probation for no reason.

C **Incorrect** (trap: Good but not best). The release is valid and covers attendance and participation, so the officer is entitled to that much. Holding it back until a subpoena arrives withholds what the client agreed to share and may count against the client.

Takeaway: Share what the release covers and no more; a court-ordered referral does not widen the client's consent.

Sources: NASW Code of Ethics (2021), 1.07(b); NASW Code of Ethics (2021), 1.07(c)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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65. ETH-1541 (rev 2) · Best answer: A

An 82-year-old patient on dialysis for 6 years tells the dialysis social worker they want to stop treatment and enter hospice. Over 3 weeks the patient has given the same reasons, and the nephrologist and a psychiatrist have confirmed decision-making capacity. The patient hopes the family will come to accept the decision. The patient's adult children ask the social worker to talk the patient out of it. What is the **BEST** response?

A. Offer the patient a family meeting to explain the choice
B. Ask the patient to continue until the family agrees
C. Request another psychiatric evaluation first
D. Remind the children the patient may legally decide

Why each option is right or wrong

A **Correct.** A competent adult may refuse life-sustaining treatment, including dialysis (Cruzan, 1990; NASW Code 1.02). Capacity is confirmed and the decision has held for weeks, so the social worker's task is support, not persuasion. The patient wants the family's acceptance, and a family meeting lets the patient explain the reasons and helps the children begin to grieve. It beats D because the legal point is accurate, but stating it alone shuts the children out; social workers elevate the patient's wishes while helping the family (NASW Standards for Palliative and End of Life Care, Standard 1).

B **Incorrect** (trap: Overrides the client's choice). Delaying until the family agrees lets the children's wishes override a competent patient's decision about their own treatment. The children's distress deserves support, but it does not change who decides.

C **Incorrect** (trap: Good but not best). A capacity and depression evaluation is essential before honoring a request to stop life-sustaining care. Here the nephrologist and a psychiatrist have already confirmed capacity, and the decision is stable, so repeating it delays the patient's choice.

D **Incorrect** (trap: Good but not best). The statement is legally accurate, which makes it tempting. Delivered alone, it dismisses grieving children, may strain the patient's remaining time with family, and misses the social worker's role in helping families through end-of-life decisions.

Takeaway: A capable patient may stop life-sustaining treatment; the social worker supports the choice and helps the family understand rather than persuading.

Sources: Cruzan v. Director, Missouri Department of Health, 497 U.S. 261 (1990); NASW Code of Ethics (2021), 1.02; NASW Standards for Palliative and End of Life Care, Standard 1: Ethics and Values
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.6 Law and ethics around death and dying · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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66. ASM-1628 (rev 1) · Best answer: C

In a family session about a parent's alcohol use, an 11-year-old cracks jokes and makes faces whenever the adults' voices rise. The family laughs, and the tension drops for a moment. Teachers call the child the class comedian, but the child tells the social worker privately, 'I'm always scared something bad will happen.' In Wegscheider-Cruse's model, which family role is this child **MOST** likely filling?

A. Hero
B. Scapegoat
C. Mascot
D. Lost child

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The hero overachieves and takes on responsibility, giving the family pride and a sense that things are fine. This child manages tension with jokes rather than with achievement or caretaking.

B **Incorrect** (trap: Wrong model or concept). The scapegoat draws attention away from the drinking by acting out and getting into trouble, so the family blames the child. This child's behavior earns laughter and relief, not blame.

C **Correct.** Wegscheider-Cruse described roles that children take on in families affected by addiction. The mascot uses humor and antics to relieve family tension and pull attention away from the problem, often while hiding fear. The child's joking when voices rise, the family's relief and the private fear all fit this role. It beats B because the scapegoat diverts attention through defiance and trouble that earns blame, not through humor that earns laughter (Wegscheider, 1981).

D **Incorrect** (trap: Wrong model or concept). The lost child withdraws, stays quiet and avoids notice to keep from adding stress. This child does the opposite, stepping into the center of attention whenever conflict rises.

Takeaway: In addiction-affected families, the mascot eases tension with humor, the hero achieves, the scapegoat acts out, and the lost child withdraws.

Sources: Wegscheider, S. (1981). Another Chance: Hope and Health for the Alcoholic Family. Science and Behavior Books, family roles
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.2 Family functioning and dynamics and their effects · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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67. ETH-1176 (rev 1) · Best answer: A

An ethics committee debates whether a hospital social worker may share a patient's history with a landlord to save the patient's housing. One member grants that this single disclosure might help. The member argues that keeping patient information private, case after case, yields the best results for patients overall, so the history should stay private. Which ethical theory is this member **MOST** likely using?

A. Rule utilitarianism
B. Act utilitarianism
C. Deontology

Why each option is right or wrong

A **Correct.** Utilitarian (teleological) theories judge actions by their consequences. Rule utilitarianism asks which general practice produces the best outcomes when followed consistently, even if breaking it once might help. The member concedes the single disclosure could help but defends privacy because the consistent practice yields the best results for patients overall, which is a rule-utilitarian argument. It beats C, deontology, because deontology treats a duty as binding regardless of outcomes; this member justifies privacy by its results (Reamer, 2018).

B **Incorrect** (trap: Wrong model or concept). Act utilitarianism weighs the consequences of this one act. On that reasoning, the member would favor disclosure if it helps this patient. The member instead sets the single case aside in favor of the general practice.

C **Incorrect** (trap: Wrong model or concept). Deontology treats certain duties, such as keeping confidences, as binding in themselves, whatever the outcome. This member's argument rests on the results of the practice, not on the duty's built-in rightness, so it is consequentialist.

Takeaway: Rule utilitarianism judges a practice by its overall consequences when followed consistently; deontology holds duties binding regardless of consequences.

Sources: Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, Ethical decision making: deontological and teleological (act and rule utilitarian) theories
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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68. ASM-1018 (rev 2) · Best answer: A

A probation officer refers a 27-year-old client with a letter listing bipolar disorder from a jail intake screening. The client describes days of feeling invincible with almost no sleep, but only during cocaine binges, followed by several days of crashing. The client has had negative drug tests for the past 6 weeks of court-ordered treatment. What is the social worker's **MOST** appropriate next step?

A. Assess the client's mood since substance use stopped
B. Accept the listed diagnosis and refer for mood stabilizers
C. Request the jail's full screening record before going further

Why each option is right or wrong

A **Correct.** A mood episode does not count toward bipolar disorder when it is attributable to a substance; DSM-5-TR points to an independent disorder when symptoms began before use or persist for a substantial period, such as about a month, after use stops. Highs only during cocaine binges, followed by crashes, fit stimulant intoxication and withdrawal. Six weeks of negative tests give a window to see how mood behaves without cocaine. It beats C because the jail screening is the source of the questionable label; the decisive information is the timing of symptoms relative to use.

B **Incorrect** (trap: Skips assessment). Accepting a screening label without assessment repeats a possible error. Mood symptoms tied only to cocaine use suggest a substance-induced picture, and medication decisions belong to a prescriber after a sound diagnosis.

C **Incorrect** (trap: Good but not best). Records can add context and may help later, with the client's written consent. They do not answer the key question, which is whether mood episodes occur apart from cocaine use.

Takeaway: Before accepting a mood diagnosis, check whether episodes occur only during substance use or withdrawal; substance-induced episodes do not count toward bipolar disorder.

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Substance/Medication-Induced Bipolar and Related Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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69. ASM-1090 (rev 2) · Best answer: C

On a night of subzero temperatures, an outreach social worker finds a client with schizophrenia sleeping outside in a light jacket. The client's fingers are white and numb. The client insists that shelters 'pump poison through the vents,' cannot describe any other way to stay warm, and refuses a blanket, a ride and the warming center. What should the social worker do **NEXT**?

A. Leave blankets nearby and return early tomorrow
B. Explore what would make a shelter feel safe
C. Call emergency services for medical and psychiatric care

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Respecting refusal and returning is right on most nights. Tonight, with numb, white fingers and no way to stay warm, waiting until morning risks severe frostbite or death.

B **Incorrect** (trap: Right action, wrong time). Engagement around what feels safe is usually the best path with a client who distrusts shelters. Here several options have already been offered and refused, a delusion blocks every route to warmth, and frostbite has begun.

C **Correct.** Nearly all states treat an inability to provide for basic needs because of mental illness as a form of danger to self, which can justify emergency evaluation even over objection (Kaplan & Sadock). Here a delusion blocks every route to warmth on a

dangerously cold night, and white, numb fingers suggest frostbite, a medical emergency. Less restrictive options were offered and refused. It beats B because continued engagement fits most nights; tonight delay risks serious injury or death.

Takeaway: When mental illness leaves a person unable to meet basic survival needs in immediate danger, emergency evaluation can override refusal.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Forensic psychiatry: involuntary hospitalization criteria
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.1 Assessing risk of harm to self or others · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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70. INT-1063 (rev 2) · Best answer: C

PRETEST — unscored on this form; do not count it

As an outpatient substance use group begins, a member arrives smelling of alcohol, slurring words, and says they drove to the session. The member asks to stay and talk about the relapse, and several members say they want to help. What should the social worker do **FIRST**?

A. Let the member stay and discuss the relapse with the group
B. Discharge the member from the program for drinking
C. Excuse the member from group and arrange a safe ride

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Discussing the relapse is valuable, and the group's wish to help is a strength. With the member intoxicated now, the talk will not be usable and may unsettle others. It fits the next session, when the member is sober.

B **Incorrect** (trap: Hands off too soon). A relapse is an expected part of recovery, not a reason to end treatment. Discharging the member removes support when it is most needed. Today's issue is taking part while intoxicated, not program eligibility.

C **Correct.** An intoxicated member cannot use group work, and processing with them present can unsettle members in early recovery. Guidance for substance use groups is to have the person leave that session and return next time, while making sure they do not drive (SAMHSA TIP 41). This member has said they drove, so a safe ride is part of the first step. It beats A because the relapse is important group material, but it is best worked on at the next session, when the member is sober and can use it.

Takeaway: A member who arrives intoxicated leaves that session safely, without driving, and the relapse is addressed once they are sober.

Sources: SAMHSA TIP 41 (2005), Ch. 6: Group Leadership, Concepts, and Techniques (intoxicated clients)
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.23 Group work: managing process and building cohesion · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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71. INT-1633 (rev 1) · Best answer: A

On a mild evening, a street outreach social worker approaches an adult who has slept in the same park for months. The person accepts a bottle of water but says, 'I don't need your programs,' and turns away. What should the social worker do **FIRST**?

A. Leave on friendly terms and say when the worker will return
B. Describe the shelter's open beds and services before leaving
C. Complete the housing intake assessment with the person

Why each option is right or wrong

A **Correct.** Engaging a person who is wary of services begins with respect for the refusal and reliable, low-pressure contact. SAMHSA TIP 55 (2013) describes short early contacts, accepting refusals without confrontation, and returning consistently with items the person may accept. Leaving on good terms and naming a return time builds the predictability that trust grows from. It beats B because pressing shelter information right after the person declined repeats the push behind the refusal; information can come once the person shows interest.

B **Incorrect** (trap: Too directive). Shelter information is useful once the person is interested. Delivered right after a clear refusal, it signals that the social worker did not hear the person, and it can make the next approach harder.

C **Incorrect** (trap: Right action, wrong time). A detailed housing assessment belongs after rapport is built and the person wants help with housing. Starting it after a refusal treats the person's answer as an obstacle rather than a decision.

Takeaway: With people wary of services, engagement starts with respecting refusals and returning reliably; formal assessment follows trust and expressed interest.

Sources: SAMHSA TIP 55 (2013) Behavioral Health Services for People Who Are Homeless, Part 1: outreach and engagement vignettes
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.11 Engaging and motivating voluntary and mandated clients · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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72. INT-0116 (rev 2) · Best answer: B

A risk scale is completed at intake. Two years later, clients who scored high at intake have returned to the hospital far more often than clients who scored low. Which measurement property does this show?

A. Concurrent validity
B. Predictive validity
C. Test-retest reliability
D. Content validity

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Concurrent validity is also criterion validity, but the criterion is measured at about the same time as the scale. Forecasting a later outcome is what distinguishes predictive validity.

B **Correct.** Predictive validity is a form of criterion validity in which scores on a measure are checked against a criterion that occurs later (Bloom, Fischer & Orme, 2009). Here the criterion, rehospitalization, happens during the 2 years after the scale is

completed, and high intake scores line up with it. It beats A because concurrent validity compares scores with a criterion measured at about the same time, such as current hospital status, rather than with a later outcome.

C **Incorrect** (trap: Wrong model or concept). Test-retest reliability checks whether the same people get similar scores when they complete a measure twice. The 2-year span here is a follow-up period for an outcome, not a second administration of the scale.

D **Incorrect** (trap: Wrong model or concept). Content validity is the extent to which a measure's items cover the full range of the concept, usually judged by experts. It says nothing about whether scores forecast outcomes.

Takeaway: Criterion validity is predictive when scores forecast a later outcome and concurrent when they match a criterion measured at the same time.

Sources: Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, criterion validity: predictive and concurrent
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** none · **Cognitive** recall · **Difficulty** 3 of 3

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73. ETH-1162 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

A social worker in private practice sees a 74-year-old client with Medicare for depression. The client's only income is a small pension. After a large dental bill, the client says they may have to stop sessions because of the copayment and asks whether it could be waived for a while. What is the **MOST** appropriate response?

A. Explain that Medicare rules bar waiving copays
B. Waive copays for every client living on a fixed income
C. Waive it after documenting the client's hardship

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Saying copays can never be waived overstates the rule. Federal guidance permits an occasional waiver based on an individual's good-faith hardship determination, and refusing outright may push this client out of needed treatment.

B **Incorrect** (trap: Misreads law or ethics). A standing waiver for a whole group of clients is a routine waiver, which the guidance treats as misstating the actual charge to Medicare. Hardship waivers must rest on each client's own circumstances.

C **Correct.** Federal fraud guidance treats routine copay waivers as improper but allows a provider to waive a copayment for an individual patient after a good-faith determination of financial hardship, applied occasionally rather than routinely (HHS OIG, 1994). This client has a specific change in finances and risks dropping needed care. A case-by-case waiver, recorded with its basis, keeps treatment going and honors the Code's call to consider ability to pay (NASW Code 1.13(a)). It beats B because a blanket policy for a class of clients is exactly the routine waiver the guidance warns against.

Takeaway: Copays may be waived for an individual client's documented financial hardship, but not routinely or for whole groups of clients.

Sources: HHS Office of Inspector General (1994). Special Fraud Alert: Routine Waiver of Copayments or Deductibles Under Medicare Part B, Federal Register 59(242), December 19, 1994; NASW Code of Ethics (2021), 1.13(a)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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74. INT-1509 (rev 1) · Best answer: B

In Solomon's empowerment framework, a clinic rule that turns away people who lack a fixed address is an example of which concept?

A. Indirect power block
B. Direct power block
C. Learned helplessness

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Indirect power blocks are internal: negative messages absorbed through family and community experience that weaken self-esteem and skills. The clinic rule is an external barrier, not an internalized one.

B **Correct.** Solomon (1976) described two sources of powerlessness for stigmatized groups. Direct power blocks are applied by social institutions, such as rules that deny services or resources. Indirect power blocks are negative valuations absorbed through development and family life, which limit skills and self-belief. A clinic rule that excludes people without a fixed address is imposed from outside by an institution, so it is a direct block. It beats A because nothing here has been internalized; the barrier sits in the policy.

C **Incorrect** (trap: Wrong model or concept). Learned helplessness describes passivity after repeated uncontrollable events. It is not part of Solomon's framework, and it describes a person's response rather than an institutional rule.

Takeaway: Solomon: direct power blocks are institutional barriers; indirect power blocks are internalized negative valuations that erode skills and self-belief.

Sources: Solomon, B. B. (1976). Black Empowerment: Social Work in Oppressed Communities. Columbia University Press, direct and indirect power blocks
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.1 Strengths-based and empowerment interventions · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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75. ASM-1629 (rev 1) · Best answer: A

A child welfare social worker visits a kinship home where a grandparent cares for a 15-month-old born 8 weeks preterm. The social worker has compared the child's skills with milestones for the corrected age of 13 months. The child is not yet walking alone and does not babble, wave or look up when called by name. The grandparent says a nurse told them preemies catch up. What should the social worker do **NEXT**?

A. Help the grandparent arrange an early intervention evaluation
B. Plan to recheck milestones at a corrected age of 18 months
C. Reassure the grandparent that preemies often walk later
D. Ask the grandparent to describe the child's daily routine

Why each option is right or wrong

A **Correct.** Corrected age is used to judge a premature child's milestones during the first 2 years, so not walking alone at a corrected 13 months is still within range. But babbling, waving and responding to one's name are expected by about 9 to 12 months even after correction. Missing them calls for a developmental evaluation now, and families can ask early intervention for one without waiting. It beats B because waiting 5 more months delays help for delays that prematurity does not explain (AAP, 2018; Centers for Disease Control and Prevention milestones).

B **Incorrect** (trap: Ignores risk). Rechecking later would be reasonable if the only concern were walking, which corrected age explains. The missing babbling, waving and response to name fall outside the range even for a corrected 13 months, so waiting postpones an evaluation the child needs now.

C **Incorrect** (trap: Good but not best). Late walking in a preterm child often does reflect prematurity, and correcting for age shows it is still typical. But this reassurance covers only the motor milestone and leaves the social and communication red flags unaddressed.

D **Incorrect** (trap: Right action, wrong time). Learning about routines belongs in a full assessment, but the social worker has already compared the child's skills with corrected-age milestones. Gathering more history now postpones the referral that the red flags already justify.

Takeaway: Correct for prematurity through age 2, but skills missing even for the corrected age still call for prompt developmental evaluation.

Sources: American Academy of Pediatrics, HealthyChildren.org (2018). Corrected Age for Preemies, use of corrected age during the first 2 years; Centers for Disease Control and Prevention. Learn the Signs. Act Early.: CDC's Developmental Milestones, 9-month and 12-month milestone checklists
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.16 Typical vs. atypical development across the lifespan · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

76. INT-1114 (rev 1) · Best answer: C

In a supportive housing program, a client with schizoaffective disorder invites the social worker to a public gallery showing of the client's paintings. Showing work in public was a recovery goal the two set a year ago. The event is an evening opening at a community center. What is the **MOST** appropriate response?

A. Decline, explaining that attending would create a dual relationship
B. Ask the client to bring photos of the paintings to a session
C. Attend briefly after agreeing on boundaries with the client
D. Attend and join the client's celebration dinner afterward

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). The Code warns against dual relationships that risk exploitation or harm, not against every contact outside the office (NASW Code 1.06(c)). A short, planned visit to a public event tied to a treatment goal is a boundary crossing, not a second relationship.

B **Incorrect** (trap: Good but not best). Seeing the paintings in session is warm and safe, and it may suit a worker who cannot attend. Here, though, the public showing itself was the goal, and redirecting to photos misses a chance to support it.

C **Correct.** Not every boundary crossing is harmful. A brief, planned crossing that serves a client's treatment goal can be appropriate when the worker weighs the risks, keeps the role clear and documents the reason (Reamer, 2020). Attending a public showing that marks a recovery goal the two set together supports the client's progress and community ties. Agreeing in advance on how long the worker will stay and how they will greet each other keeps the role clear. It beats B because viewing photos in session is safe but misses a milestone the client specifically invited the worker to witness.

D **Incorrect** (trap: Boundary slip or collusion). Joining a private celebration moves from a brief, purposeful appearance into a social role. That blurs the professional relationship and can confuse the client about what the relationship is.

Takeaway: A brief, planned boundary crossing that serves a client's goal, such as attending a public milestone, can be appropriate; avoid social roles.

Sources: Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, boundary crossings versus boundary violations; NASW Code of Ethics (2021), 1.06(c)
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.4 Setting limits and boundaries with clients · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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77. ETH-1080 (rev 4) · Best answer: B

During a home visit, a 79-year-old client who lives alone says the adult children want an in-home aide after two minor falls this year, neither causing injury. The client understands the risks, describes a plan to wear a medical alert pendant, and declines the aide. Under the Ethical Principles Screen, which principle takes priority in this situation?

A. Least harm
B. Self-determination
C. Quality of life
D. Protection of life

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Least harm matters, and the family's worry reflects it. In the screen, though, it ranks below self-determination, so it governs only if the client's choice were not informed and voluntary.

B **Correct.** The Ethical Principles Screen ranks protection of life first, then social justice, then self-determination, autonomy and freedom, ahead of least harm and quality of life (Dolgoff et al., 2012). Minor falls and an informed safety plan do not pose an immediate threat to life, so the client's autonomy ranks highest among the principles in play. This fits the Code, which allows limiting self-determination when a client's actions pose a serious, foreseeable and imminent risk (NASW Code 1.02). B beats A because least harm ranks below self-determination when life is not at stake.

C **Incorrect** (trap: Wrong model or concept). Quality of life ranks fifth in the screen, below both self-determination and least harm. It may shape how services are offered but does not outweigh an informed choice.

D **Incorrect** (trap: Wrong model or concept). Protection of life ranks first, but the screen ranks only the principles actually in conflict, and this one applies when a choice directly threatens life. Two minor falls without injury and a working alert plan do not show that level of danger.

Takeaway: In the Ethical Principles Screen, an informed choice outranks least harm and quality of life unless life itself is threatened.

Sources: Dolgoff, R., Harrington, D., & Loewenberg, F. M. (2012). Ethical Decisions for Social Work Practice (9th ed.). Brooks/Cole Cengage, Ethical Principles Screen; NASW Code of Ethics (2021), 1.02
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** none · **Cognitive** recall · **Difficulty** 3 of 3

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78. ETH-1718 (rev 3) · Best answer: A

A client works 38 hours a week at two jobs, supports two children, and falls behind on rent each winter. The social worker and client have reviewed the household budget together and found spending limited to basics. The client has never filed a tax return because the income seemed too low to matter. Which step is **MOST** helpful?

A. Help the client file to claim refundable tax credits
B. Help the client apply for emergency rental assistance
C. Review the client's spending for further savings

Why each option is right or wrong

A **Correct.** The Code's Social Justice principle aims change efforts at poverty and other structural injustice, not only personal habits (NASW Code, Ethical Principles: Social Justice). Low-wage working families who do not file can miss the refundable earned income tax credit, which pays out even when no tax is owed (26 U.S.C. 32). The budget review has shown that spending is limited to basics, so the gap is income. Helping the client file, for example through free tax preparation, can bring a sizable yearly payment. It beats B because emergency rental aid covers one shortfall, while refundable credits add income every year the client files, which addresses a gap that returns each winter.

B **Incorrect** (trap: Good but not best). Emergency rental aid can prevent an eviction and fits when the client faces one now. It covers a single shortfall, though, and the gap returns every winter because basics exceed income, so it leaves the recurring cause in place.

C **Incorrect** (trap: Right action, wrong time). A spending review was the right first step, and it is done. Repeating it adds little when basics alone exceed income, and it can signal blame for a structural problem.

Takeaway: When a budget review shows basics exceed income, pursue income supports such as unclaimed tax credits rather than personal spending changes.

Sources: Internal Revenue Code, 26 U.S.C. 32 (earned income tax credit); NASW Code of Ethics (2021), Ethical Principles: Social Justice

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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79. ETH-1579 (rev 2) · Best answer: A

A 16-year-old Mexican American client tells a social worker that darker-skinned relatives are 'less smart' and that lighter skin makes people 'more respectable.' The client avoids the sun to stay light and calls classmates with Indigenous features 'low class.' These views echo messages the client has heard from family and media. Which concept **MOST** likely explains the client's beliefs?

A. Internalized racial oppression
B. Acculturative stress
C. Assimilation
D. Stereotype threat

Why each option is right or wrong

A **Correct.** Internalized racial oppression is the acceptance, by members of a targeted group, of the dominant society's devaluing messages and racial hierarchy (Pyke, 2010). The client has taken in the idea that lighter skin and European features mean worth and applies it to family, peers and self. It beats C because assimilation describes trading heritage culture for the dominant culture, while this client has absorbed a racial hierarchy that devalues darker skin and Indigenous features. Pyke frames this as a product of structural racism, not a personal weakness.

B **Incorrect** (trap: Wrong model or concept). Acculturative stress is strain from adapting to a new or dominant culture, such as conflict between home and school norms. The client's statements show acceptance of a skin-color hierarchy rather than distress about adjusting.

C **Incorrect** (trap: Wrong model or concept). In Berry's acculturation model, assimilation means adopting the dominant culture while letting go of one's heritage culture (Berry, 1997). The client's statements rank people's worth by skin color, including relatives and self, rather than describing a shift in cultural practices or identity.

D **Incorrect** (trap: Wrong model or concept). Stereotype threat is anxiety about confirming a negative stereotype that can lower performance in evaluative settings. The client is endorsing the stereotypes, not fearing being judged by them.

Takeaway: Members of targeted groups may absorb the dominant society's devaluing messages about their own group; this reflects structural racism, not personal failing.

Sources: Pyke, K. D. (2010). What is internalized racial oppression and why don't we study it? Acknowledging racism's hidden injuries. Sociological Perspectives, 53(4), 551-572, whole article; Berry, J. W. (1997). Immigration, acculturation, and adaptation. Applied Psychology: An International Review, 46(1), 5-34, acculturation strategies: assimilation

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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80. INT-1087 (rev 2) · Best answer: B

A family service agency's director says the agency is now trauma-informed because two clinicians completed TF-CBT training. Meanwhile, front-desk staff call out clients' reasons for visits in the waiting room, and a no-show policy closes cases after two missed visits without outreach. Which recommendation is **MOST** consistent with a trauma-informed approach?

A. Train more clinicians in TF-CBT so every child is served

B. Train staff in each role and revise intake and attendance rules

C. Have intake staff screen every new client for trauma

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). TF-CBT is an evidence-based, trauma-specific treatment, but expanding it does not make the agency trauma-informed. The approach applies to every staff member and policy, including the waiting room and the no-show rule.

B **Correct.** A trauma-informed approach is organization-wide: everyone, including front-desk staff, realizes trauma's impact, recognizes its signs, responds through policy and practice, and resists re-traumatization (SAMHSA, 2014). Trauma-specific treatments such as TF-CBT are interventions, not the approach itself. Announcing visit reasons aloud and closing cases without outreach undermine safety and trust. It beats C because universal screening fits within trauma-informed care, but it adds a step while leaving the practices that already harm clients in place (SAMHSA TIP 57, 2014).

C **Incorrect** (trap: Good but not best). Screening is part of many trauma-informed programs, but it does not fix the practices already harming clients. Asking about trauma while privacy is ignored at the front desk may also feel unsafe to clients.

Takeaway: Trauma-informed care is an organization-wide approach covering all staff and policies; trauma-specific therapy alone does not make an agency trauma-informed.

Sources: SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), the four Rs; trauma-informed approach versus trauma-specific interventions; SAMHSA TIP 57 (2014), Part 1, Ch. 4: Screening and Assessment

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.1 Trauma-informed care across trauma types (complex, vicarious) · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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81. INT-1015 (rev 2) · Best answer: B

After being stuck in an elevator, a client now avoids medical appointments on upper floors. Using systematic desensitization, the social worker and client have built a ranked list of elevator situations, and the client has learned progressive muscle relaxation. What should the social worker do **NEXT**?

A. Have the client ride to the top floor right away
B. Pair relaxation with imagining the least distressing item
C. Pair relaxation with imagining the most distressing item

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Going straight to the most feared situation is flooding, a different exposure method. It can work, but it abandons the graded hierarchy and is not systematic desensitization.

B **Correct.** Systematic desensitization counters anxiety with an incompatible relaxed state, which Wolpe called reciprocal inhibition (Wolpe, 1958). With the hierarchy built and relaxation learned, the next step is to pair relaxation with the lowest-ranked item, moving up only when that item no longer provokes anxiety. It beats A because riding straight to the top is flooding, an intense, immediate exposure that skips the gradual hierarchy the client agreed to.

C **Incorrect** (trap: Misuses the technique). Pairing imagery with relaxation is the right technique, but systematic desensitization starts at the bottom of the ranked list. The most feared scene would bring on anxiety strong enough to overwhelm relaxation, so the client moves up only after each lower item no longer provokes anxiety.

Takeaway: Systematic desensitization: build a hierarchy, teach relaxation, then pair relaxation with items from least to most feared.

Sources: Wolpe, J. (1958). Psychotherapy by Reciprocal Inhibition. Stanford University Press, systematic desensitization
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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82. INT-1629 (rev 2) · Best answer: B

Two months after a parent's children entered foster care because of the parent's methamphetamine use, the parent has stayed sober, attended every visit, and has a relative willing to help. At a family team meeting, the parent says, 'Everyone only talks about what I did wrong.' The team then turns to drafting the case plan. Which approach to the plan is **BEST**?

A. List the parent's strengths in a separate section of the plan
B. Build each safety goal on a strength the parent already shows
C. Ask the parent how the meeting has felt so far

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Recording strengths is better than omitting them, but a separate list leaves them unused. The safety goals stay framed only around past harm, and the parent's sobriety, visits and relative do not shape the path to reunification.

B **Correct.** The strengths perspective does not ignore danger; it uses what is already working to address it (Saleebey, 2013). Signs of Safety practice builds safety goals from existing strengths, such as a support network (Turnell & Edwards, 1999). Here the relative can anchor a safety plan, and steady sobriety and visits can become measured steps toward reunification. It beats A because listing strengths in a separate section honors them on paper while leaving the safety goals deficit-based, which is the pattern the parent is protesting.

C **Incorrect** (trap: Good but not best). Checking in about how the meeting feels can matter, and the parent's frustration deserves a response. It does not answer the question of how to write the plan, though, and the parent's complaint is answered most directly by a plan that is built on what the parent has achieved.

Takeaway: Strengths-based child welfare plans use existing strengths as the means of meeting safety goals; they neither ignore risk nor list strengths separately.

Sources: Saleebey, D. (2013). The Strengths Perspective in Social Work Practice (6th ed.). Pearson, Principles of the strengths perspective; Turnell, A., & Edwards, S. (1999). Signs of Safety: A Solution and Safety Oriented Approach to Child Protection Casework. W. W. Norton, Building safety from existing strengths
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.1 Strengths-based and empowerment interventions · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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83. ASM-1059 (rev 1) · Best answer: A

On a home visit, a 77-year-old client who was widowed last year has had 2 falls in 3 months and forgets recent conversations. An adult child asks the social worker to arrange memory testing. The social worker notices many empty wine bottles in the recycling bin, and the client says a few glasses help with sleep. What should the social worker do **FIRST**?

A. Ask about the amount and timing of the client's drinking
B. Refer the client for the memory testing the family requested
C. Arrange a home safety evaluation to prevent further falls

Why each option is right or wrong

A **Correct.** In older adults, alcohol misuse often shows up as falls, memory problems, poor sleep or low mood and is easily mistaken for normal aging or dementia (SAMHSA TIP 26). The empty bottles and the client's use of wine for sleep after a major loss make drinking a likely contributor to both the falls and the forgetfulness. A respectful, specific question about quantity and pattern comes first because it shapes every next step. It beats B because memory testing done during heavy drinking may credit alcohol effects to a neurocognitive disorder.

B **Incorrect** (trap: Right action, wrong time). Memory evaluation may well be needed, but ordering it first ignores a visible, treatable contributor. Testing is more accurate once drinking has been assessed and, if needed, addressed.

C **Incorrect** (trap: Good but not best). Fall-proofing the home is worthwhile and may follow. It treats the falls as an environmental problem, though, and leaves a possible alcohol cause of falls and forgetfulness unassessed.

Takeaway: In older adults, falls and forgetfulness can signal alcohol misuse; assess drinking before attributing problems to aging or dementia.

Sources: SAMHSA TIP 26 (2020), Ch. 4: Treating Alcohol Misuse in Older Adults
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.8 Addiction and substance use: signs and impact on systems · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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84. ETH-1101 (rev 1) · Best answer: A

A wildfire destroys hundreds of homes in a neighboring state, and thousands of residents move into evacuation shelters. A licensed clinical social worker with crisis training wants to help and plans to drive there this weekend to offer counseling at the shelters. What is the **BEST** way for the social worker to act on this goal?

A. Volunteer through an organized disaster mental health response
B. Go to the shelters and offer crisis counseling on arrival
C. Wait to help until licensed in the affected state
D. Offer free video counseling to evacuees from home

Why each option is right or wrong

A **Correct.** The Code asks social workers to provide appropriate professional services in public emergencies to the fullest extent possible (NASW Code 6.03). Disaster mental health guidance tells providers to operate only within an authorized response system, which assigns roles, screens credentials and coordinates with local authorities (Brymer et al., 2006). Joining an organized response lets the worker help soon without duplicating services or practicing outside the host state's rules. A beats B because arriving alone bypasses incident command and the host state's rules on who may provide counseling.

B **Incorrect** (trap: Misreads law or ethics). Wanting to help quickly is admirable, but self-deployed providers can strain shelter operations, duplicate services and practice without authorization in another state. Psychological first aid guidance calls for working inside an authorized response system.

C **Incorrect** (trap: Good but not best). Full licensure in another state can take months, while the Code encourages service in emergencies. Organized responses screen volunteers' credentials and coordinate with host-state authorities, so waiting is unnecessary for someone willing to join one.

D **Incorrect** (trap: Misreads law or ethics). Video sessions with evacuees are still practice where the clients are located, which means following that jurisdiction's laws (1.04(e)). Offering them independently also bypasses the coordination that disaster responses depend on.

Takeaway: In public emergencies, offer professional help through an authorized, organized response rather than self-deploying.

Sources: NASW Code of Ethics (2021), 6.03; NASW Code of Ethics (2021), 1.04(e); Brymer, M., et al. (2006). Psychological First Aid: Field Operations Guide (2nd ed.). National Child Traumatic Stress Network and National Center for PTSD, Introduction and Overview: Professional Behavior
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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85. ASM-1689 (rev 1) · Best answer: C

A family that immigrated 4 years ago seeks help because a 15-year-old and the parents argue daily. The teen, now fluent in English, wants to spend evenings with friends and attend parties. The parents see this as rejecting family values and say the teen is 'becoming a stranger.' The teen's grades remain steady. Which intervention plan is **BEST**?

A. Individual sessions to help the teen manage anger with family
B. Parent training in setting consistent limits and consequences
C. Family sessions helping members value skills from both cultures
D. Guidance urging the parents to allow freedoms typical for local teens

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Individual anger work places the problem in the teen alone. The conflict is relational and cultural, and steady grades point away from an individual disorder, so working with the whole family fits better.

B **Incorrect** (trap: Wrong model or concept). Consistent limits help many families, but this approach treats the conflict as a parenting-skills deficit. It does not address the clash in cultural expectations that drives the daily arguments.

C **Correct.** Culture shapes the plan when the conflict itself grows from different rates of acculturation. Here the teen has taken on new norms faster than the parents, and both sides frame the fight in cultural terms. Bicultural Effectiveness Training was developed for this pattern; it helps family members see the skills of each culture as complementary rather than opposed, easing intergenerational conflict (Szapocznik et al., 1984). It beats B, the strongest competitor, because limit-setting training treats the problem as a discipline gap and can sharpen a values clash the family has not yet talked through.

D **Incorrect** (trap: Too directive). Advising the parents to adopt local norms imposes the worker's values and dismisses the family's culture. It would likely deepen mistrust rather than help parents and teen understand each other.

Takeaway: When family conflict stems from an acculturation gap, plan family work that bridges both cultures rather than treating one member or culture as the problem.

Sources: Szapocznik, J., Santisteban, D., Kurtines, W., Perez-Vidal, A., & Hervis, O. (1984). Bicultural Effectiveness Training: A treatment intervention for enhancing intercultural adjustment in Cuban American families. Hispanic Journal of Behavioral Sciences, 6(4), 317-344, whole article

Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.4 Culture in intervention planning · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

86. ASM-1620 (rev 2) · Best answer: A

PRETEST — unscored on this form; do not count it

An adult client in outpatient treatment for PTSD after a car crash agreed in the fifth session to begin processing the crash memory. The next week, the client says, 'I think I'm better now. Let's make this our last session.' Nightmares still occur several times a week, and the client still avoids driving. What should the social worker do **FIRST**?

A. Ask what has prompted the wish to stop now
B. Agree to end and explain how to return to services
C. Pause trauma processing and shift to coping skills
D. Point out that ongoing nightmares mean the goals are unmet

Why each option is right or wrong

A **Correct.** Readiness for termination rests on goals met and gains that hold, not on a request alone (Hepworth et al.). A request to stop right after trauma processing begins may reflect avoidance, a core PTSD symptom, or another reason such as cost or a hard reaction to the last session. Exploring what prompted it, without judgment, comes first because the answer decides what follows: support for the work, a slower pace or a planned ending the client chooses with full information (NASW Code 1.02). It beats D because citing symptoms before hearing the client's reasons can feel like pressure and push an avoidant client out of care.

B **Incorrect** (trap: Right action, wrong time). If the client still chooses to stop once the reasons and progress are discussed, the social worker respects that choice and leaves the door open. Agreeing at once skips learning whether avoidance is driving an early exit from effective treatment.

C **Incorrect** (trap: Good but not best). Slowing the pace of trauma work can help when processing feels overwhelming. Changing course before learning why the client wants to stop guesses at the problem and may not match what the client actually wants.

D **Incorrect** (trap: Too directive). Ongoing nightmares and avoidance are relevant, and reviewing progress against goals will matter. Leading with them before asking why the client wants to stop can feel like an argument and may harden the decision.

Takeaway: When a client asks to end abruptly, especially just after hard work begins, explore the reasons before agreeing, arguing or changing the plan.

Sources: Hepworth, D. H., et al. Direct Social Work Practice: Theory and Skills (11th ed.). Cengage, termination: indicators of readiness and premature termination; NASW Code of Ethics (2021), 1.02
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.6 Signs a client is ready for termination · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

87. INT-1167 (rev 2) · Best answer: C

Hours after a shooting at a warehouse, a social worker is providing psychological first aid at a family assistance center. An employee who hid during the shooting is trembling but answers questions clearly. The employee says their phone died and their family does not yet know they are alive. What is the **BEST** response?

A. Give the employee information on common stress reactions
B. Guide the employee through a brief breathing exercise
C. Help the employee contact loved ones right away

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Information about common stress reactions is a core part of psychological first aid and will help. Offered now, it comes ahead of the employee's own stated priority; PFA meets survivors' immediate needs first, and reaching family is this employee's most pressing one.

B **Incorrect** (trap: Good but not best). Breathing exercises help survivors who are overwhelmed or disoriented. This employee is coherent and has named a pressing practical need, so meeting that need comes before calming techniques.

C **Correct.** Psychological first aid responds to survivors' current needs and priorities, and one of its core actions is connecting survivors with primary support persons as soon as possible (Brymer et al., 2006). The employee is oriented, names a clear need, and the family is waiting for news. Helping make that contact meets the stated need and brings support. It beats B because stabilization techniques are for survivors who are disoriented or overwhelmed; this employee is trembling but thinking clearly.

Takeaway: Psychological first aid follows the survivor's immediate priorities, such as reaching loved ones, and does not require recounting the traumatic event.

Sources: Brymer, M., Jacobs, A., Layne, C., et al. (2006). Psychological First Aid: Field Operations Guide (2nd ed.). National Child Traumatic Stress Network and National Center for PTSD, Core Action 6: Connection with Social Supports

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.5 Crisis intervention and crisis treatment approaches · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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88. INT-1029 (rev 1) · Best answer: B

A probation officer sends a client to anger management after an arrest for shoving a partner. The client says the partner 'pushes my buttons.' The client also checks the partner's phone each night and controls all the household money. Coworkers describe the client as calm and easygoing. Which plan is **MOST** appropriate?

A. Enroll the client in a general anger management group
B. Refer the client to a partner-abuse intervention program
C. Offer joint sessions with the partner to improve communication

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). This matches the referral's wording, which makes it tempting. But the stem describes control aimed at the partner, not anger across settings, and anger-control framing can reinforce the client's view that the partner provokes the violence (Gondolf & Russell, 1986).

B **Correct.** Intimate partner violence is a pattern of coercive behavior used to gain power and control, not mainly a failure to regulate anger (Healey, Smith & O'Sullivan, 1998). Nightly phone checks, control of all the money, and calm behavior with coworkers point to control aimed at the partner. A partner-abuse intervention program targets those tactics and the client's accountability. It beats A because general anger management treats the violence as an anger problem that the partner 'provokes,' which fits the client's blaming account and leaves the controlling behavior untouched.

C **Incorrect** (trap: Ignores risk). Joint sessions can endanger a person who is being abused, because what they say may be punished later, and they imply shared responsibility for the violence (SAMHSA TIP 25). Conjoint work is not a first response to partner violence.

Takeaway: Coercive control and partner violence call for a partner-abuse intervention program, not general anger management or couples sessions.

Sources: Healey, K., Smith, C., & O'Sullivan, C. (1998). Batterer Intervention: Program Approaches and Criminal Justice Strategies. National Institute of Justice (NCJ 168638), Ch. 1, defining domestic violence; Ch. 2, couples counseling; Gondolf, E. W., & Russell, D. (1986). The case against anger control treatment programs for batterers. Response, 9(3), 2-5, whole article; SAMHSA TIP 25 (1997), Ch. 2: Survivors of Domestic Violence: An Overview (safety; cautions on couples therapy)

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.20 Helping clients manage anger · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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89. ASM-1664 (rev 1) · Best answer: A

At intake to a residential substance use program, a social worker needs to ask a 29-year-old client about any history of sexual violence. The client has heard the limits of confidentiality and agreed to continue with the intake questions. Which question is **MOST** effective for this purpose?

A. 'Has anyone ever forced or pressured you into sexual contact?'

B. 'Have you ever been sexually abused, molested or assaulted?'

C. 'Is there anything about your past that you would like to share?'

Why each option is right or wrong

A **Correct.** Behaviorally specific questions describe the act rather than asking a person to apply a label such as 'abuse' or 'rape.' Many people who experienced unwanted sexual contact do not use those labels, so label-based questions miss them. Research comparing question wording found that behaviorally specific items prompt more people to recall and report such experiences (Fisher, 2009). Asking in plain terms also keeps the tone matter-of-fact. It beats B because the label-based version is direct but depends on the client defining the experience the same way.

B **Incorrect** (trap: Misuses the technique). This question is direct, which helps, but it asks the client to label the experience. People who would not call what happened abuse or assault may truthfully answer no, so real histories go unrecorded.

C **Incorrect** (trap: Good but not best). An open invitation suits many topics, but it is too vague for a sensitive history. It leaves the client to guess what is being asked and makes it even less likely that something hard to name will come up.

Takeaway: Ask about sensitive experiences with behaviorally specific wording that describes acts, not labels the client may not use.

Sources: Fisher, B. S. (2009). Effects of survey question wording on rape estimates: Evidence from a quasi-experimental design. Violence Against Women, 15(2), 133-147, whole article
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.7 Eliciting sensitive or confidential information (substance use, sexual abuse) · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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90. ETH-1583 (rev 2) · Best answer: A

A client tells a social worker about being the sole Asian American employee on a team and being left out of meetings. A friend responded, 'I don't see race. People are just people.' Under Sue and colleagues' taxonomy, the friend's comment is **MOST** likely which type of microaggression?

A. Microinvalidation
B. Microinsult
C. Microassault

Why each option is right or wrong

A **Correct.** Sue et al. (2007) describe microinvalidations as messages that exclude, negate or nullify the thoughts, feelings or experiences of people of color; claiming not to see race is a central example. The friend's comment denied that race could have shaped the client's exclusion from meetings. It beats B because microinsults convey rudeness and demean a person's racial heritage, such as implying someone was hired because of race; the friend's comment erases the racial experience rather than demeaning it.

B **Incorrect** (trap: Wrong model or concept). A microinsult subtly demeans a person's racial identity, such as implying someone got a job through a quota. The friend did not demean the client's heritage but denied that race mattered at all.

C **Incorrect** (trap: Wrong model or concept). A microassault is an explicit, usually deliberate racial attack, such as a slur or purposeful exclusion. The friend's comment was not openly hostile and was likely meant to be supportive.

Takeaway: Color-blind statements that deny the role of race are microinvalidations: they negate the racial experiences of people of color.

Sources: Sue, D. W., Capodilupo, C. M., Torino, G. C., Bucceri, J. M., Holder, A. M. B., Nadal, K. L., & Esquilin, M. (2007). Racial microaggressions in everyday life: Implications for clinical practice. American Psychologist, 62(4), 271-286, taxonomy of racial microaggressions
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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91. ASM-1086 (rev 1) · Best answer: D

A psychiatric inpatient team is planning discharge for a patient with schizophrenia. The record shows the patient attacked neighbors twice in the past 3 years, a current belief that a neighbor is poisoning the water supply, and heavy drinking on weekends. Which factor **MOST** strongly predicts future violence?

A. The persecutory belief about the neighbor
B. Heavy weekend alcohol use
C. The diagnosis of schizophrenia
D. Prior violent behavior

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Persecutory beliefs can raise concern, especially when a specific person is the focus. Their link with violence is weaker and less consistent than a documented history of actual assaults.

B **Incorrect** (trap: Near-miss diagnosis). Substance use is an important, changeable risk factor and should be addressed in discharge planning. It adds to risk but predicts violence less strongly than a record of past violent acts.

C **Incorrect** (trap: Near-miss diagnosis). Most people with schizophrenia are never violent, and the diagnosis alone is a weak predictor. Treating the label as the main risk factor overlooks the behavioral history that matters most.

D **Correct.** Past violence is the most consistent predictor of future violence in people with and without mental disorders; in the MacArthur study, prior arrests and violent acts were among the strongest risk factors (Monahan et al., 2001). Two attacks on neighbors in 3 years outweigh the other factors listed. It beats A because persecutory beliefs may add risk, but their association with violence is weaker and less consistent than a record of actual assaults (Kaplan & Sadock).

Takeaway: Past violent behavior is the strongest single predictor of future violence; diagnosis alone predicts poorly.

Sources: Monahan, J., Steadman, H. J., Silver, E., Appelbaum, P. S., Robbins, P. C., Mulvey, E. P., Roth, L. H., Grisso, T., & Banks, S. (2001). Rethinking Risk Assessment: The MacArthur Study of Mental Disorder and Violence. Oxford University Press, Prior violence and arrest history as risk factors; Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Psychiatric emergencies: violence and assaultive behavior
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.1 Assessing risk of harm to self or others · **Qualifier** MOST · **Cognitive** recall · **Difficulty** 3 of 3

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92. ASM-1138 (rev 1) · Best answer: C

A 34-year-old began sertraline for depression 5 weeks ago. Two weeks later, the client stopped sleeping, spent recklessly and announced a plan to run for governor. The prescriber stopped the sertraline at once. Three weeks after it was stopped, the client remains on an inpatient unit with grandiosity, pressured speech and little need for sleep. Which diagnosis is **MOST** likely?

A. Substance/medication-induced bipolar and related disorder
B. Bipolar II disorder
C. Bipolar I disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). This diagnosis applies when mood elevation is attributable to a medication and fades as its effects wear off. Mania lasting weeks after sertraline was stopped goes beyond the drug's effect.

B **Incorrect** (trap: Near-miss diagnosis). Bipolar II disorder requires hypomania and depression with no manic episode ever. A syndrome severe enough for hospitalization and lasting weeks is manic, which makes the diagnosis bipolar I.

C **Correct.** DSM-5-TR counts a full manic episode that emerges during antidepressant treatment as evidence of bipolar I when the syndrome persists beyond the drug's physiological effect. Here mania continues 3 weeks after sertraline was stopped, with grandiosity, pressured speech and decreased need for sleep severe enough for admission. It beats A because a medication-induced diagnosis fits only when symptoms are attributable to the drug, and persistence well after stopping it points to an independent bipolar disorder (DSM-5-TR).

Takeaway: Full mania that begins on an antidepressant and persists beyond the drug's effect supports bipolar I, not a medication-induced disorder.

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar I Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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93. ASM-1535 (rev 1) · Best answer: C

Four months after a parent lost a factory job, a family of five meets with a social worker at a community agency to assess its strengths. Which observation **BEST** indicates a key family resilience process?

A. The parents keep the job loss from the children to protect them
B. Each member handles worries privately to avoid burdening others
C. Members describe the job loss as a challenge they will face together
D. The family keeps every routine exactly as it was before the loss

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Protecting children is a caring motive, but Walsh identifies clear, age-appropriate sharing of information as a resilience process. Secrecy often leaves children confused and anxious and keeps the family from facing the problem as a unit.

B **Incorrect** (trap: Good but not best). Shielding others from worry is well meant, yet open emotional sharing is one of the communication processes that support family resilience. Coping alone isolates members during a crisis.

C **Correct.** Walsh's family resilience framework identifies key processes in belief systems, organization and communication. Making meaning of adversity, seeing a crisis as a shared challenge the family can meet together, is a core belief-system process (Walsh, 2016). It beats D because stability can look like strength, but Walsh describes flexibility, the ability to reorganize roles and routines under new demands, as the resilient pattern; rigidly keeping everything the same after a major income loss can block needed adaptation.

D **Incorrect** (trap: Wrong model or concept). Some routines provide stability, but resilience depends on flexibility: changing roles, budgets and schedules to fit new circumstances. Keeping every routine exactly the same after a major income loss suggests rigidity rather than adaptation.

Takeaway: Family resilience shows in shared meaning-making, flexible organization and open communication, not in secrecy, private coping or rigid routines.

Sources: Walsh, F. (2016). Strengthening Family Resilience (3rd ed.). Guilford Press, key processes in family resilience

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.2 Family functioning and dynamics and their effects · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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94. ASM-1102 (rev 1) · Best answer: C

According to U.S. law enforcement data, which statement describes most people who sexually assault children?

A. They are strangers to the child
B. They first met the child online
C. They know the child or family
D. They are younger than 18

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Stranger abuse draws the most public attention, but strangers made up only about 7% of offenders against juveniles in law enforcement data. Focusing on strangers can lead adults to miss warning signs in people the child trusts.

B **Incorrect** (trap: Wrong model or concept). Online contact is a real route to exploitation, but it describes a minority of cases. Most offenders already have offline access to the child through family, school, sports or neighborhood ties.

C **Correct.** Most people who sexually abuse children are relatives or acquaintances, which is one reason grooming of the child and family often goes unnoticed. In Snyder's (2000) analysis of law enforcement data, about a third of offenders against juveniles were family members, nearly 6 in 10 were acquaintances, and 7% were strangers. It beats D because juvenile offenders are a sizable minority, about a third of offenders against juvenile victims, not most. Assessment should attend to trusted adults, not only strangers.

D **Incorrect** (trap: Wrong model or concept). Juveniles do commit a meaningful share of sexual offenses against children, about a third of offenders against juvenile victims in Snyder's data. That is a substantial minority, not most.

Takeaway: Most child sexual abuse is committed by someone the child or family knows and trusts, not by a stranger.

Sources: Snyder, H. N. (2000). Sexual Assault of Young Children as Reported to Law Enforcement: Victim, Incident, and Offender Characteristics (NCJ 182990). Bureau of Justice Statistics, Victim-offender relationship

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.12 Traits of perpetrators of abuse, neglect, exploitation · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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95. ETH-1687 (rev 1) · Best answer: A

In an employee assistance session, a white client says a workplace diversity training called them 'privileged.' The client says angrily, 'I grew up poor and worked for everything I have. Nobody handed me anything.' This is the first time the client has raised the topic. What is the social worker's **MOST** appropriate response?

A. Explore what the training's label stirred up for the client
B. Explain that racial privilege can exist alongside economic hardship
C. Agree that the training should have focused on class instead

Why each option is right or wrong

A **Correct.** When a client reacts defensively to the idea of racial privilege, exploring the reaction comes before teaching the concept. The anger likely carries hurt about the client's own hardship, and feeling heard makes later reflection on privilege possible (NASW Code 1.05(c)). It beats B because B's point is accurate, since advantage tied to race can coexist with economic struggle (McIntosh, 1989), but offered as the first response it reads as a correction and is likely to deepen the defensiveness.

B **Incorrect** (trap: Too directive). The point is accurate: race-based advantage can coexist with real economic hardship (McIntosh, 1989). As the first response to an angry client, though, it reads as a correction and is likely to harden the defensiveness. It fits once the client feels heard.

C **Incorrect** (trap: Boundary slip or collusion). Agreeing that class matters more sides with the defensiveness and implies racial privilege is not real. It may feel supportive, but it closes off reflection the client could benefit from.

Takeaway: When a client reacts defensively to discussions of racial privilege, explore the reaction before teaching the concept.

Sources: NASW Code of Ethics (2021), 1.05(c); McIntosh, P. (1989). White privilege: Unpacking the invisible knapsack. Peace and Freedom, July/August, 10-12, whole article
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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96. ETH-1660 (rev 1) · Best answer: C

Under U.S. law, how is a capable patient's request to have a ventilator removed generally classified?

A. Medical aid in dying
B. Voluntary active euthanasia
C. Refusal of medical treatment

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Medical aid in dying means a capable, terminally ill adult takes a prescribed medication to end life, and it is lawful only in some jurisdictions. Removing a ventilator at the patient's request is a different act, recognized as a treatment refusal.

B **Incorrect** (trap: Wrong model or concept). Active euthanasia means a clinician directly causes death, for example by injection, and it is illegal throughout the United States. Withdrawing a treatment the patient refuses lets the underlying illness take its course.

C **Correct.** U.S. courts treat a capable patient's refusal of life-sustaining treatment, including a ventilator, as a protected choice (Cruzan, 1990). The Supreme Court distinguished refusing treatment from being helped to end one's life, because when treatment is refused the underlying disease causes death (Vacco v. Quill, 1997). It beats A because medical aid in dying involves a prescribed medication the patient takes to end life and is lawful only in some states, whereas the right to refuse treatment is recognized nationally.

Takeaway: Courts classify removing life support at a capable person's request as treatment refusal, a category separate from aid in dying and euthanasia.

Sources: Cruzan v. Director, Missouri Department of Health, 497 U.S. 261 (1990); Vacco v. Quill, 521 U.S. 793 (1997)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.6 Law and ethics around death and dying · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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97. ETH-1068 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

A client logs on for a scheduled video session during a lunch break. The social worker sees that the client is driving, with the phone mounted on the dashboard. The client says it is fine to talk because the drive is 20 minutes. What should the social worker do **FIRST**?

A. Switch to audio so the client can watch the road
B. Ask the client to park safely or reschedule the session
C. Keep the session to a short check-in during the drive

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Audio-only can make a session workable when video is the barrier, but here the risk is driving while engaged in therapy. Emotional conversation distracts a driver whether or not the camera is on.

B **Correct.** Social workers assess whether remote services are suitable at the time of service (NASW Code 1.03(g)) and act to promote the client's well-being (1.01). A session while driving divides attention, endangers the client and others, and leaves no privacy for difficult topics. Pausing until the client is parked, or rescheduling, protects safety without ending care. B beats A because audio alone still asks the client to hold an emotional conversation while driving, which remains a dangerous distraction.

C **Incorrect** (trap: Ignores risk). Shortening the session reduces but does not remove the danger of talking through emotional material while driving, and it signals that the risk is acceptable.

Takeaway: Check each remote session's suitability; do not hold sessions while a client is driving or otherwise unsafe.

Sources: NASW Code of Ethics (2021), 1.03(g); NASW Code of Ethics (2021), 1.01
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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98. ASM-1502 (rev 1) · Best answer: D

A child welfare social worker observes a supervised visit between a 15-month-old in foster care and the birth parent. After a brief separation, the parent returns and holds out their arms. The toddler starts toward the parent, then stops and freezes with a blank stare. Next, the toddler backs toward the parent with face turned away. Which attachment pattern does this behavior **MOST** likely reflect?

A. Secure
B. Insecure-avoidant
C. Insecure-ambivalent
D. Disorganized

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Securely attached toddlers may protest separation but seek the caregiver on reunion and are comforted fairly quickly. Freezing and backing in with face turned away is not a secure pattern.

B **Incorrect** (trap: Wrong model or concept). Avoidant toddlers show little distress and calmly ignore or turn from the caregiver on reunion. The averted face fits partly, but avoidance is a consistent strategy, while this toddler freezes and approaches in a contradictory way.

C **Incorrect** (trap: Wrong model or concept). Ambivalent (resistant) toddlers are very distressed, cling yet push away angrily, and are hard to soothe (Ainsworth et al., 1978). This toddler shows a blank stare and freezing, not angry, clinging protest.

D **Correct.** Disorganized attachment shows contradictory or fearful behavior toward the caregiver on reunion, such as freezing, dazed expressions or approaching while looking away (Main & Solomon, 1990). This toddler starts toward the parent, freezes with a blank stare, then backs in with face averted, a mix that no organized strategy explains. It beats B because avoidant infants consistently ignore or turn from the caregiver and do not freeze or approach in contradictory ways. Disorganized attachment is most associated with maltreatment or frightening caregiving, which matters for visit planning.

Takeaway: Disorganized attachment appears as contradictory or fearful reunion behavior, such as freezing or approaching while looking away; avoidance is consistent ignoring.

Sources: Main, M., & Solomon, J. (1990). Procedures for identifying infants as disorganized/disoriented during the Ainsworth Strange Situation. In M. T. Greenberg, D. Cicchetti, & E. M. Cummings (Eds.), Attachment in the Preschool Years (pp. 121-160). University of Chicago Press, indices of disorganization; Ainsworth, M. D. S., Blehar, M. C., Waters, E., & Wall, S. (1978). Patterns of Attachment: A Psychological Study of the Strange Situation. Erlbaum, classification groups A, B, C

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.16 Typical vs. atypical development across the lifespan · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

99. ETH-1650 (rev 2) · Best answer: C

A social worker in long-term recovery attends a weekly community 12-step meeting, which is the worker's main recovery support. One evening, a current outpatient client walks in, sees the social worker and sits down. The social worker is unsure whether to keep attending. The client's next session is in 4 days. What should the social worker do **FIRST**?

A. Discuss the encounter with the client at the next session
B. Find a different meeting to attend from now on
C. Talk through the overlap with a supervisor or consultant

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Raising the encounter with the client is necessary, and it should happen soon. Doing it before the worker has sorted out how personal recovery needs shape the plan risks steering the client toward whatever is easiest for the worker.

B **Incorrect** (trap: Good but not best). Leaving the meeting gives up the worker's main recovery support and settles the question alone, before talking with anyone. Changing meetings may end up being part of the plan, but only after consultation and a conversation with the client.

C **Correct.** When a boundary question involves the social worker's own life, self-interest can cloud judgment, so consultation comes before acting. The meeting supports the worker's recovery, and the client has an equal claim to it, so both people's needs are at stake. A consultant helps the worker plan how to raise the encounter, protect the client's privacy and weigh options such as attending different meetings (NASW Code 2.05(a), 1.06(c); Reamer, 2020). It beats A because the worker is still unsure whether to keep attending, and the 4 days before the session leave time to sort out personal interests first; the talk with the client comes next.

Takeaway: When an overlapping relationship touches the social worker's own personal life, seek consultation before deciding how to handle it with the client.

Sources: NASW Code of Ethics (2021), 2.05(a); NASW Code of Ethics (2021), 1.06(c); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, unanticipated encounters with clients in personal settings
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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100. ETH-1530 (rev 1) · Best answer: B

A 22-year-old client at a community counseling center has a job offer in another city. The client tells the social worker, 'In my family, big decisions are made together. I want my parents to decide this with me.' What is the **BEST** response?

A. Encourage the client to make this choice independently
B. Help the client plan how to include the parents
C. Explore whether the parents are pressuring the client

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Encouraging an independent choice imposes one view of adulthood on a client who has stated a different, considered preference. Respecting self-determination means supporting how the client wants to decide, not only what the client decides.

B **Correct.** Respect for autonomy includes a person's choice to make decisions together with family (Beauchamp & Childress, 2019), and social workers help clients pursue goals in their own way (NASW Code 1.02). The client has said plainly that including the parents is the client's own wish, so the social worker helps carry it out, for example by planning the family conversation or a joint session. It beats C because probing for coercion fits when there are signs of pressure, such as fear or contradiction; here the client describes a shared family value.

C **Incorrect** (trap: Skips assessment). Checking for pressure is wise when a client seems fearful, hesitant or contradicts themselves. This client presents family involvement as a personal value, so probing for coercion without any sign of it treats the client's choice as suspect.

Takeaway: Self-determination includes choosing to decide with family; support the client's chosen decision process unless signs of coercion appear.

Sources: NASW Code of Ethics (2021), 1.02; Beauchamp, T. L., & Childress, J. F. (2019). Principles of Biomedical Ethics (8th ed.). Oxford University Press, Ch. 4: respect for autonomy; choosing to delegate decisions
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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101. ASM-1000 (rev 1) · Best answer: B

A 54-year-old client with no psychiatric history comes to a community mental health agency after 2 weeks of hearing voices and accusing neighbors of stealing. The client's partner reports new morning headaches. The client also complains of a burning-rubber smell in the apartment that no one else notices. What should the social worker do **FIRST**?

A. Obtain a detailed family psychiatric history
B. Arrange a prompt medical and neurological workup
C. Refer the client to a first-episode psychosis program
D. Begin psychoeducation about psychotic disorders

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Family history belongs in a full assessment and can be gathered soon. It does not come first when late onset, new headaches and an unusual smell point to a possible physical cause that needs prompt evaluation.

B **Correct.** Psychotic symptoms are not attributed to a mental disorder until medical causes are ruled out (DSM-5-TR). Several red flags point to a possible medical or neurological cause: a first psychosis after age 50, no psychiatric history, new headaches and a smell no one else notices (an olfactory hallucination). A brain lesion, seizure disorder or other illness must be checked promptly (Kaplan & Sadock). It beats A because family history adds useful detail but does nothing about a possibly dangerous physical cause; it can be gathered while the workup proceeds.

C **Incorrect** (trap: Hands off too soon). Early psychosis programs serve people with a primary psychotic disorder, usually younger adults. Referring now assumes a psychiatric cause before a medical one is ruled out. It may fit later if the workup is negative.

D **Incorrect** (trap: Skips assessment). Teaching about psychotic disorders assumes the diagnosis. With red flags for a medical cause, education about psychosis is premature and could mislead the client and partner.

Takeaway: New psychosis with late onset, headaches or unusual hallucinations (smell, touch, vision) needs medical evaluation before any psychiatric diagnosis.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Psychotic Disorder Due to Another Medical Condition; Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Schizophrenia spectrum chapter: differential diagnosis
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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102. INT-1607 (rev 1) · Best answer: C

An associate social worker at a family counseling agency records sessions, with client consent, for review in supervision. The supervisor asks the associate to bring one recording from this week. Which recording would be **MOST** useful for the associate's learning?

A. A session in which a new technique worked well
B. The most recent session, whatever happened in it
C. A session that felt stuck or went poorly

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Reviewing a success can reinforce a skill, but supervisees who bring only their best work hide the areas where supervision would help most.

B **Incorrect** (trap: Misuses the technique). A random recent session may be informative, but it leaves learning to chance. The supervisee's role is to choose material that targets current learning needs.

C **Correct.** Supervisees get the most from supervision when they bring the work they find hardest, including errors and stuck points, rather than work that shows them at their best (Kadushin & Harkness, 2014). Choosing where help is needed is part of self-assessment. A session that felt stuck shows exactly what the supervisor can help with. It beats A because a session that went well mainly confirms a skill already in place; it has some value, but less for learning.

Takeaway: Supervisees learn most by bringing the work they find hardest, such as sessions where they felt stuck, not their most polished work.

Sources: Kadushin & Harkness (2014). Supervision in Social Work (5th ed.). Columbia University Press, Ch. 5: Principles and Problems in Implementing Educational Supervision
Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.1 Supervisee's role: learning needs, self-assessment, prioritizing · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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103. ASM-1618 (rev 2) · Best answer: B

PRETEST — unscored on this form; do not count it

A social worker at a rural community agency meets a client who wants medication treatment for opioid use disorder. The nearest opioid treatment program is 80 miles away and requires near-daily visits at first. The client works days and has no car. The local primary care clinic offers in-person and telehealth visits. Which resource is **MOST** appropriate to pursue?

A. Daily methadone dosing at the distant opioid treatment program
B. A nearby prescriber of buprenorphine
C. A weekly peer support group in the county
D. A residential treatment program in the nearest city

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Methadone is effective, but for opioid use disorder it is dispensed through certified opioid treatment programs, which usually require near-daily visits at first (SAMHSA TIP 63). Without a car and with a day job, an 80-mile trip each day is unlikely to last.

B **Correct.** Resource selection weighs availability and accessibility, not only the type of service. Since the federal waiver requirement ended in 2023, clinicians with standard DEA registration for Schedule III drugs can prescribe buprenorphine (SAMHSA, MAT Act), so local primary care, in person or by telehealth, can provide medication without daily travel. Methadone for this disorder requires an opioid treatment program (SAMHSA TIP 63). It beats A because an 80-mile daily trip without a car, while working days, is likely to fail despite the medication's effectiveness.

C **Incorrect** (trap: Overrides the client's choice). Peer support can strengthen recovery, but the client asked for medication, the most effective treatment for opioid use disorder. Offering a group instead substitutes the social worker's preference for the client's informed choice.

D **Incorrect** (trap: Good but not best). Residential care is for needs that outpatient treatment cannot safely meet, and nothing here suggests that. It would cost the client the job when medication can be started locally.

Takeaway: Choose resources by accessibility as well as effectiveness; buprenorphine can now be prescribed in ordinary medical and telehealth settings.

Sources: SAMHSA (2023). Waiver Elimination (MAT Act), Consolidated Appropriations Act, 2023, Section 1262; SAMHSA TIP 63 (2021), Part 1: Introduction to Medications for Opioid Use Disorder Treatment
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.1 Identifying available, appropriate community resources · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

104. INT-1582 (rev 1) · Best answer: B

At the end of each final session, outpatient social workers hand their clients a satisfaction survey with the worker's name printed at the top. Each worker waits while the client fills it out and then collects it. Nearly every rating is 'very satisfied.' Which change would **MOST** improve the accuracy of the results?

A. Add more rating items to the survey for detailed feedback
B. Have staff outside the client's care collect anonymous surveys
C. Remove the worker's name but keep workers collecting it

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). More items can add detail, but they do not remove the pressure clients feel when rating their own worker face to face. Biased answers to more questions are still biased.

B **Correct.** Client satisfaction surveys tend to run very positive, and the bias grows when the person being rated hands out and collects the form (Royse, Thyer & Padgett, 2016). Making responses anonymous and having someone outside the client's care collect them reduces the pull to please the worker, and it protects the confidentiality of evaluation data (NASW Code 5.02(m)). This beats C because removing the name still leaves clients answering under their worker's eye and handing the form back to that worker.

C **Incorrect** (trap: Good but not best). Removing the name helps a little, but the worker still waits and collects the form. Clients may still give favorable ratings to avoid seeming ungrateful to the person who helped them.

Takeaway: Reduce social desirability bias in client feedback by making responses anonymous and having people not involved in care collect them.

Sources: Royse, D., Thyer, B. A., & Padgett, D. K. (2016). Program Evaluation: An Introduction to an Evidence-Based Approach (6th ed.). Cengage, Ch. 7: Client Satisfaction; NASW Code of Ethics (2021), 5.02(m)
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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105. ETH-1570 (rev 1) · Best answer: D

A program manager at an outpatient clinic tells a clinical supervisor to drop suicide screening at intake to shorten wait times. The supervisor raised the safety risk with the manager in writing and proposed a shorter screening tool, but the manager repeated the directive. What should the supervisor do **NEXT**?

A. Have clinical staff keep screening without telling the manager
B. Follow the directive while keeping a record of the objection
C. Propose a trial of the shorter tool to the manager again
D. Take the concern to the clinical director or compliance office

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Quietly continuing screening protects clients for now but defies a directive in secret, damages trust and leaves the unsafe policy in place. Open escalation through proper channels addresses the policy itself.

B **Incorrect** (trap: Ignores risk). Recording the objection documents the concern but still removes a safety step for every new client. Employer directives do not override the duty to avoid practices that put clients at risk.

C **Incorrect** (trap: Good but not best). Offering the alternative again is reasonable advocacy, but the manager has already refused it in writing. Each week of delay means more new clients go unscreened while the supervisor waits.

D **Correct.** Social workers do not let employer policies interfere with ethical practice and work to bring agency practices in line with the Code (NASW Code 3.09(d)). The supervisor has already raised the risk directly with the manager and offered an alternative, and the manager refused. The next step is formal channels above the manager (2.10(d)), since administrators must keep a work environment that supports Code compliance (3.07(d)). It beats C because repeating a refused proposal leaves screening suspended while new clients keep arriving.

Takeaway: When a manager refuses to reverse a directive that endangers clients, take the concern through formal channels above that manager.

Sources: NASW Code of Ethics (2021), 3.09(d); NASW Code of Ethics (2021), 2.10(d); NASW Code of Ethics (2021), 3.07(d)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 2 of 3

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106. INT-1033 (rev 2) · Best answer: A

A client tells the social worker that a cousin keeps pressing them to co-sign a car loan. Each time, the client explains why they can't, the cousin argues with every reason, and the client ends up saying they will 'think about it.' Which assertiveness technique is **BEST** for the social worker to teach?

A. Calmly repeat the same short refusal each time
B. Give a stronger reason the cousin cannot dispute
C. Offer to help the cousin find a different co-signer

Why each option is right or wrong

A **Correct.** The broken-record technique has the client calmly restate the same brief refusal, without new explanations, until the other person accepts it (Smith, 1975). The cousin argues with every reason, so each explanation opens a new debate that ends with the client giving in. Repeating one short 'No, I won't co-sign' gives the cousin nothing new to argue with while staying respectful. It beats B because a better reason is still a reason to argue with; the problem is the pattern of justifying, not the quality of the reasons.

B **Incorrect** (trap: Misuses the technique). Searching for an airtight reason keeps the client defending the decision. The cousin has already shown they will dispute any reason given, so this repeats the pattern that leads to giving in.

C **Incorrect** (trap: Good but not best). A workable compromise is a real assertiveness skill when both sides can give a little. Here the client simply wants to say no, and offering an alternative reopens the negotiation in which the client keeps giving in. A firm, repeated refusal fits this pressure better.

Takeaway: When someone argues with every reason, teach the broken record: calmly repeat one brief refusal without adding justifications.

Sources: Smith, M. J. (1975). When I Say No, I Feel Guilty. Dial Press, broken record
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.25 Teaching assertiveness skills · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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107. ASM-1506 (rev 1) · Best answer: B

A school social worker meets with a 9-year-old who has stopped turning in math assignments. The child compares their test scores with classmates' scores, says 'Everyone is smarter than me,' and refuses to try new tasks in case they fail. Which Erikson stage is the child **MOST** likely working through?

A. Initiative versus guilt
B. Industry versus inferiority
C. Autonomy versus shame and doubt

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Initiative versus guilt is the preschool stage (about 3 to 6), when children start activities and plans and may feel guilt over overstepping. A 9-year-old's distress about academic competence belongs to the next stage.

B **Correct.** Erikson's school-age crisis, industry versus inferiority (about ages 6 to 12), centers on building competence through schoolwork and skills, judged against peers (Erikson, 1963). This 9-year-old measures themself against classmates, concludes they are less capable and stops trying, signs of a growing sense of inferiority. It beats A because initiative versus guilt is the preschool task of planning and starting activities, and its conflict is guilt over self-started play rather than competence at school.

C **Incorrect** (trap: Wrong model or concept). Autonomy versus shame and doubt is the toddler stage (about 1.5 to 3), centered on independence in basic self-care. Fear of failing fits partly, but a 9-year-old judging their competence against classmates is facing the school-age crisis.

Takeaway: Industry versus inferiority (school age) concerns competence measured against peers; repeated failure and comparison can produce a sense of inferiority.

Sources: Erikson, E. H. (1963). Childhood and Society (2nd ed.). W. W. Norton, eight ages of man: industry vs. inferiority
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.16 Typical vs. atypical development across the lifespan · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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108. ETH-1017 (rev 1) · Best answer: D

A client at a community counseling center signed a release last month allowing the social worker to send monthly progress updates to the client's employee assistance program. Today the client hands the social worker a written statement revoking that release. What should the social worker do **FIRST**?

A. Ask the client to explain the reasons for revoking
B. Send one final update to close out the arrangement
C. Ask the program to return the updates already sent
D. Stop further updates and note the revocation in the record

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Talking about the change may matter clinically, and the worker can invite that discussion afterward. Making the revocation wait on an explanation treats a right as something the client must justify.

B **Incorrect** (trap: Misreads law or ethics). Once revoked, the release no longer authorizes disclosures. Sending another update, even to close out the arrangement, would be an unauthorized disclosure.

C **Incorrect** (trap: Good but not best). A revocation applies to future disclosures. Updates sent while the release was valid were lawful and cannot be recalled; the task is to stop new disclosures.

D **Correct.** Clients may revoke an authorization at any time in writing. The revocation applies to future disclosures but cannot undo actions already taken in reliance on it (45 CFR 164.508(b)(5)). Disclosure requires valid, current consent (NASW Code 1.07(b)), so the worker stops sending updates and documents the revocation. D beats A because asking about the reasons may help later, but the client's right to revoke does not depend on giving reasons, and disclosures must stop now.

Takeaway: A written revocation of a release takes effect at once for future disclosures; stop sharing and document it.

Sources: HIPAA Privacy Rule, 45 CFR 164.508(b)(5); NASW Code of Ethics (2021), 1.07(b)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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109. INT-1517 (rev 2) · Best answer: B

A parent must complete a parenting class under a court-ordered child welfare case plan. A few minutes into the first meeting, the parent says, 'Nobody asked what I need. I'm not sitting in some class.' What should the social worker do **FIRST**?

A. Describe what the court may do if the class goes unfinished
B. Reflect the parent's frustration at having no say in the plan
C. Separate the required steps from those the parent can choose

Why each option is right or wrong

A **Incorrect** (trap: Too directive). The parent does need accurate information about consequences, but leading with them sounds like a threat and increases pushback. Consequences fit within a clear account of what is fixed and what is open.

B **Correct.** Work with an involuntary client starts by acknowledging the client's reaction to the mandate. Anger at lost freedom, or reactance, is an expected response to pressure, and reflecting it lowers defensiveness and shows the worker is listening (Rooney, 2009). The parent names a specific grievance, not being asked, so a reflection that captures it opens the conversation. It beats C because clarifying what is required and what is open is the right next step, but leading with it before the parent feels heard can sound like more of the same control.

C **Incorrect** (trap: Right action, wrong time). Separating non-negotiable requirements from areas of choice is central to work with mandated clients and answers the complaint about having no say (Rooney, 2009). It lands best right after the parent's anger has been heard; offered first, it can sound like a sales pitch for the class.

Takeaway: With a mandated client's anger, reflect the reaction first; then separate non-negotiable requirements from areas of choice.

Sources: Rooney, R. H. (Ed.). (2009). Strategies for Work with Involuntary Clients (2nd ed.). Columbia University Press, non-negotiable versus negotiable elements of mandated contact

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.11 Engaging and motivating voluntary and mandated clients · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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110. INT-1564 (rev 2) · Best answer: C

A child welfare social worker places a 6-year-old in emergency foster care after finding the child alone at home for 2 days. The parent says the child is an enrolled member of a federally recognized tribe but has no enrollment card on hand. The agency plans to file a petition for foster care placement. What does federal law require of the agency regarding the tribe?

A. Wait for proof of enrollment before contacting the tribe

B. Bring in the tribe once reunification efforts have failed

C. Notify the tribe and apply the law's protections now

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Verification matters, but federal rules treat the child as an Indian child while membership is confirmed with the tribe. Waiting for documents delays notice and the tribe's chance to take part.

B **Incorrect** (trap: Misreads law or ethics). The tribe has a right to notice and to intervene from the start, not only as a fallback. The Indian Child Welfare Act also requires active efforts to keep the family together, which the tribe can help shape from the beginning.

C **Correct.** The Indian Child Welfare Act requires the party seeking foster care placement to notify the child's tribe of the proceeding, and no hearing may occur until at least 10 days after the tribe receives notice (25 U.S.C. 1912(a)). When there is reason to know a child is an Indian child, the child is treated as one until membership is confirmed or ruled out, and the agency works with the tribe to verify it (25 CFR 23.107). It beats A because waiting for a card delays the tribe's right to participate.

Takeaway: When there is reason to know a child is an Indian child, notify the tribe promptly and apply ICWA protections while membership is verified.

Sources: Indian Child Welfare Act of 1978, 25 U.S.C. 1912(a); Indian Child Welfare Act regulations, 25 CFR Part 23, 25 CFR 23.107(b)
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.7 Effects of policy, regulation, legislation on practice and services · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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111. ETH-1061 (rev 1) · Best answer: D

A client receiving video sessions for depression from home says, in a low voice, that their partner works from home, listens at the door and checks the client's phone. The client wants to keep meeting and works in an office two blocks from the clinic. What is the **BEST** way for the social worker to proceed?

A. Continue video sessions using headphones and a code word
B. Assess the client's comfort with the clinic's video platform
C. Switch to secure text messaging between sessions
D. Offer in-person sessions at the clinic during the workday

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Headphones keep the worker's voice private, but the client must still speak in a home where the partner listens. A code word helps end a call quickly; it does not make disclosure safe.

B **Incorrect** (trap: Good but not best). Assessing a client's relationship with technology is useful at the start of remote services. Here the problem is already clear: privacy and possible control at home, not the client's comfort with the platform.

C **Incorrect** (trap: Misuses the technique). Text messages arrive on the same phone the partner checks, so this would expose the client's disclosures and could increase danger.

D **Correct.** Social workers assess whether remote services suit each client and help arrange alternatives when they do not (NASW Code 1.03(g)); electronic services carry confidentiality risks that must be weighed (Tech Standards 2.04). A partner who listens at the door and checks the phone means privacy at home cannot be assured, and the controlling pattern raises safety concerns. The client's office near the clinic makes in-person sessions practical. D beats A because headphones protect only what the client hears; the partner can still overhear what the client says.

Takeaway: When home lacks privacy or safety, remote sessions may be unsuitable; offer a workable in-person or safer alternative.

Sources: NASW Code of Ethics (2021), 1.03(g); NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.04
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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112. INT-1543 (rev 1) · Best answer: C

A 72-year-old client with COPD receives a notice that Medicaid home care hours will be cut in half in 10 days. The client relies on the aide for bathing and meals and wants to fight the cut. The notice explains how to request a fair hearing. What should the social worker do **FIRST**?

A. Call the Medicaid caseworker to request that the cut be reversed
B. Gather medical records that document the client's care needs
C. Help the client file an appeal before the cut takes effect
D. Arrange a private-pay aide to cover the lost hours

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Informal negotiation is often the first rung of advocacy and may still help. Relying on it first risks missing the window that keeps services in place during an appeal. It fits alongside the appeal, not before it.

B **Incorrect** (trap: Right action, wrong time). Documentation will strengthen the case at the hearing, but gathering it first can push the request past the effective date. Evidence can be collected after the appeal is filed.

C **Correct.** Advocacy usually starts with the least adversarial step, but deadlines change the order. Under federal Medicaid rules, when a beneficiary requests a hearing before the date of action, services generally continue until the hearing decision (42 CFR 431.230(a)). Filing now protects the bathing and meal help the client depends on, and helping the client file keeps the client in charge of the challenge. It beats A because a call to the caseworker can happen at the same time, but if it fails after the effective date, the hours are already lost.

D **Incorrect** (trap: Good but not best). Paying privately may be costly and accepts a cut the client wants to challenge. It is a fallback if the appeal fails or a stopgap while other options are explored.

Takeaway: When a benefit cut has an appeal deadline, file the appeal promptly to preserve services, then pursue negotiation and evidence.

Sources: Medicaid fair hearing regulations, 42 CFR Part 431 Subpart E, 42 CFR 431.230(a)
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.13 Advocating for needed services and resources · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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113. ASM-1617 (rev 1) · Best answer: B

A primary care social worker gave a client with panic disorder a list of three low-cost counseling clinics. At follow-up 2 weeks later, the client has not called any of them and explains that phone calls with strangers trigger panic and that the clinic websites were confusing. The client still wants counseling. What should the social worker do **NEXT**?

A. Ask the client what made calling the clinics difficult
B. Call one of the clinics together with the client now
C. Give the client a shorter list with clearer directions

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Asking about barriers is the right step when they are unknown. The client has already named them, panic with strangers on the phone and confusing websites, so asking again delays the connection the client still wants.

B **Correct.** Linking a client to a resource includes navigating access and following through on coordination, not just handing over names (NASW Standards for Social Work Case Management, Standard 6). The client has already explained the barriers: calls with strangers trigger panic and the websites are confusing. A joint call during the visit, a warm handoff, removes both barriers while the client's motivation is high. It beats C because a shorter list still leaves the client to make the frightening call alone.

C **Incorrect** (trap: Good but not best). Clearer directions help with the confusing websites but not with the panic triggered by calling strangers. The client would still face the step that stopped them the first time.

Takeaway: When a client cannot act on a referral list, close the gap directly, such as with a warm handoff, and confirm the connection is made.

Sources: NASW Standards for Social Work Case Management (2013), Standard 6: Service Planning, Implementation, and Monitoring
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.1 Identifying available, appropriate community resources · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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114. ETH-1591 (rev 3) · Best answer: A

On an inpatient psychiatric unit, a Black adult patient is admitted with psychotic symptoms after a week without sleep. The patient is guarded with staff and tells the social worker about being restrained during a past emergency visit while a white patient who was shouting was not. In rounds, the team cites the patient's distrust of staff as paranoia that supports a schizophrenia diagnosis. What should the social worker do **FIRST**?

A. Point out that the distrust may reflect experiences of racism
B. Record the restraint account in the psychosocial assessment
C. Question the patient further to see if the distrust is a fixed belief

Why each option is right or wrong

A **Correct.** Implicit bias can lead clinicians to read a Black patient's distrust, often rooted in real experiences of discrimination, as a symptom. Black patients receive psychotic disorder diagnoses at three to four times the rate of white patients, and reviews link the gap in part to clinician bias and to distrust that colors clinical judgment (Schwartz & Blankenship, 2014). The patient describes a specific, plausible experience of unequal treatment, so naming this context in rounds, before the diagnosis is set, lets the team weigh the distrust accurately (NASW Code 1.05(b)). It beats C because probing a credible account as a possible delusion repeats the bias at issue.

B **Incorrect** (trap: Right action, wrong time). Documenting the account matters and should happen, but it can follow. A note the team may not read before the diagnosis is set leaves a possibly biased reading of the distrust in place.

C **Incorrect** (trap: Misuses the technique). Checking whether a belief is fixed and implausible is part of assessing possible delusions. This account is specific, plausible

and tied to a real event, so testing it as a symptom treats a report of unequal treatment as pathology, the very error at issue.

Takeaway: When a client's distrust may stem from experiences of racism, raise that context with the team before it is counted as a psychotic symptom.

Sources: Schwartz, R. C., & Blankenship, D. M. (2014). Racial disparities in psychotic disorder diagnosis: A review of empirical literature. World Journal of Psychiatry, 4(4), 133-140, whole article; NASW Code of Ethics (2021), 1.05(b)

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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115. ETH-1600 (rev 1) · Best answer: C

A 34-year-old bisexual client in a small town meets with a social worker about low mood and poor sleep. The client keeps a same-sex relationship hidden from coworkers, plans each conversation to avoid slips, and expects to be shunned if found out. The client adds, 'Part of me still thinks something is wrong with me.' Which concept **MOST** likely explains this pattern?

A. Acculturative stress
B. Stereotype threat
C. Minority stress
D. Identity foreclosure

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Acculturative stress comes from adapting to a new culture after migration, such as language barriers and clashing values. Nothing here involves migration; the strain comes from stigma about sexual orientation in the client's own community.

B **Incorrect** (trap: Wrong model or concept). Stereotype threat is worry about confirming a negative group stereotype, which can lower performance on a task in the moment. It would fit a test or job review; it does not explain ongoing concealment, expected rejection and self-stigma.

C **Correct.** The minority stress model holds that stigma adds chronic strain for sexual minority people through outside events and through internal processes: expecting rejection, concealing one's identity and internalized stigma (Meyer, 2003). This client shows all three: constant vigilance to keep the relationship hidden, expected shunning, and the belief that 'something is wrong with me,' along with low mood. It beats B because stereotype threat is a short-lived performance effect when a stereotype becomes salient, not a lasting pattern of concealment and self-stigma.

D **Incorrect** (trap: Wrong model or concept). Identity foreclosure means committing to an identity, often one handed down by family, without exploring alternatives. This client has a clear sense of orientation; the struggle is the cost of others' stigma, not a lack of exploration.

Takeaway: Hiding identity, expecting rejection and internalized stigma are core minority stress processes that can harm sexual minority clients' mental health.

Sources: Meyer, I. H. (2003). Prejudice, social stress, and mental health in lesbian, gay, and bisexual populations: Conceptual issues and research evidence. Psychological Bulletin, 129(5), 674-697, whole article
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.3 Culture, race, gender, sexuality, intersectionality shaping identity and behavior · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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116. INT-1017 (rev 2) · Best answer: A

In a residential treatment program, a 15-year-old comes out of a phone call with a parent shaking, panting, and saying they want to punch the wall. The teen has been learning DBT skills. What should the social worker help the teen do **FIRST**?

A. Splash cold water on their face and slow their breathing
B. Check the facts about what the parent said on the call
C. Talk through what upset them during the call

Why each option is right or wrong

A **Correct.** When emotional arousal is extreme, DBT teaches skills that change body chemistry quickly, such as cold water on the face and paced breathing, before any skill that depends on clear thinking (Linehan, 2015). The teen is shaking and breathing rapidly, with an urge to hit something, so lowering physical arousal comes first. It beats B because checking the facts is a valuable emotion-regulation skill, but it relies on reasoning that is hard to reach at peak arousal; it fits once the teen has calmed.

B **Incorrect** (trap: Right action, wrong time). Checking the facts helps a client test whether an emotion fits the situation. It is the right skill once arousal drops; at this intensity, the teen is unlikely to think it through.

C **Incorrect** (trap: Right action, wrong time). Talking through the call can help the teen make sense of it and feel heard, and it fits later. At this level of arousal, though, discussing the upsetting conversation tends to keep the emotion high; the body needs to calm first.

Takeaway: At peak emotional arousal, lower the body's arousal first with fast physical skills; cognitive skills work once the client is calmer.

Sources: Linehan, M. M. (2015). DBT Skills Training Manual (2nd ed.). Guilford Press, distress tolerance: TIP skills
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.6 Teaching emotion-regulation skills · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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117. ASM-1534 (rev 1) · Best answer: B

A pediatrician refers an 8-year-old with recurring stomachaches whose medical workup was normal. The child has missed 9 school days this term. During the first meeting, the parents mention that the stomachaches seem worst on days when they argue. What should the social worker do **FIRST**?

A. Recommend couples counseling to reduce the parents' arguing
B. Assess how each family member reacts when the pain begins
C. Teach the child relaxation skills to use when pain starts

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Couples work may become part of the plan if assessment confirms that parental conflict drives the symptoms. Recommending it now jumps from one comment to a conclusion and skips assessing how the whole family responds to the child's pain.

B **Correct.** Structural family theory notes that a child's symptom can detour parental conflict: the parents unite around the sick child instead of facing their own disagreement (Minuchin, Rosman & Baker, 1978). The parents' comment suggests such a link, but it needs assessment: what each member does when the pain begins and what happens to the arguing. It beats A because recommending couples counseling acts on a hypothesis before the family pattern has been explored and may leave the child's role in it unaddressed.

C **Incorrect** (trap: Right action, wrong time). Relaxation training can help a child manage pain and may be added later. Starting with it treats the stomachaches as the child's individual problem before the family pattern around them is understood.

Takeaway: When a child's symptom seems tied to family conflict, assess the family's interaction around the symptom before choosing an intervention.

Sources: Minuchin, S., Rosman, B. L., & Baker, L. (1978). Psychosomatic Families: Anorexia Nervosa in Context. Harvard University Press, detouring of parental conflict through a child's symptom

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.2 Family functioning and dynamics and their effects · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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118. INT-1067 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

A closed 16-week grief group for older adults meets at a senior center. In week 12, the social worker reminded members that the group ends in 4 weeks. In week 14, two members miss without notice, and a member who had been sleeping well says the sleep problems are back. What should the social worker do **NEXT**?

A. Offer to extend the group by 4 more weeks

B. Refer the member with sleep problems for evaluation

C. Invite members to talk about the approaching last session

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Extending the group avoids the feelings the ending stirs up and may teach members that endings can be dodged. For a grief group, a planned, well-processed ending is itself a chance to practice coping with loss.

B **Incorrect** (trap: Hands off too soon). Sleep changes in an older adult can warrant evaluation if they persist or come with other warning signs. Here they appear as the group nears its end, a common regression the leader should first address in the group.

C **Correct.** Members often react to a group's approaching end with absences, denial, or a return of old symptoms; these are expected termination reactions (Yalom & Leszcz, 2020). Having announced the ending, the leader's next task is to bring those reactions into the room, help members name feelings about another loss, review gains, and plan support after the group. It beats B because returning sleep problems could need

evaluation if they persist, but in week 14 of a grief group they fit the pattern of reacting to the coming separation, which the group can address now.

Takeaway: Absences, denial and returning symptoms near a group's end are expected termination reactions; address them openly in the group.

Sources: Yalom, I. D., & Leszcz, M. (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books, termination of the group

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.23 Group work: managing process and building cohesion · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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119. ETH-0108 (rev 1) · Best answer: B

During a session, an adult client who is angry about a recent layoff says, 'My manager is going to get what's coming.' The client has no known history of violence and quickly changes the subject. What should the social worker do **FIRST**?

A. Notify the manager and local police of a possible threat

B. Ask the client directly what the statement meant

C. Document the statement and revisit it next session

D. Consult a supervisor about the duty to warn

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Warning the manager and police is a protective step if assessment shows a serious threat. Taking it on an ambiguous remark breaches confidentiality without a basis and may damage the relationship and the client's livelihood.

B **Correct.** Confidentiality yields to protective action when disclosure is needed to prevent serious, foreseeable and imminent harm (NASW Code 1.07(c)). This remark is vague, so the social worker first assesses it: what the client meant, and any intent, plan, means or past violence. That assessment decides whether a protective disclosure, which HIPAA permits for a serious and imminent threat, is needed (45 CFR 164.512(j)). It beats D because consultation is valuable once the facts are known, and only the client can supply them now, in session. If the client voices intent or a plan, protective steps follow.

C **Incorrect** (trap: Ignores risk). Waiting until the next session leaves a possible threat unexplored. Documentation matters, but the meaning of the remark needs to be assessed now, while the client is present.

D **Incorrect** (trap: Right action, wrong time). Consulting about a duty to warn is wise once the social worker knows whether a real threat exists. Without asking the client first, the supervisor has too little information to guide the decision.

Takeaway: An ambiguous threat calls for immediate assessment of intent, plan and means before deciding on any protective disclosure.

Sources: NASW Code of Ethics (2021), 1.07(c); HIPAA Privacy Rule, 45 CFR 164.512(j)(1)(i)

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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120. INT-1611 (rev 2) · Best answer: B

A hospice social work supervisor has been quietly rewriting a new staff member's incomplete bereavement risk screens instead of returning them, as is done for other staff. In consultation, the supervisor traces the rewriting to a protective pull: the staff member's recent loss of a parent closely resembles the supervisor's own. What should the supervisor do **NEXT**?

A. Keep examining the protective reaction in consultation
B. Give the staff member the same corrective feedback others receive
C. Share the similar loss with the staff member to build trust

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Ongoing consultation can help, but the supervisor has already traced the rewriting to the reaction. More reflection without action leaves the incomplete screens and the unequal treatment in place. Continued consultation fits alongside the corrective step, not instead of it.

B **Correct.** Supervisors should evaluate supervisees fairly against clearly stated criteria (NASW Code 3.03), and supervisors are expected to keep their own countertransference from distorting supervision and evaluation (Bernard & Goodyear, 2019). Having traced the rewriting to that reaction, the supervisor's next step is to hold this staff member to the same standard, which also builds the screening skill that bereaved families rely on. It beats A because the reaction and its effect are already identified; further reflection alone prolongs unequal treatment and withholds feedback the staff member needs to improve.

C **Incorrect** (trap: Boundary slip or collusion). Supervisor self-disclosure should serve the supervisee's learning. Sharing the parallel loss centers the supervisor's experience, may blur the relationship, and leaves the incomplete screens unaddressed.

Takeaway: After recognizing countertransference toward a supervisee, correct its effect by applying the same standards and feedback given to others.

Sources: NASW Code of Ethics (2021), 3.03; Bernard, J. M., & Goodyear, R. K. (2019). Fundamentals of Clinical Supervision (6th ed.). Pearson, Supervisory relationship: transference and countertransference
Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.3 Transference and countertransference in supervision · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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121. ETH-1145 (rev 2) · Best answer: C

A social worker in a family support program reported suspected neglect after finding a client's 4-year-old alone at home, and explained the report to the client beforehand. At the next home visit, the client says, 'I trusted you, and you called CPS on me. Why should I keep seeing you?' What is the **BEST** response?

A. Offer to transfer the client to another worker in the program
B. Explain that the law required the report and move to today's plan
C. Invite the client to talk about feeling betrayed by the report
D. Promise to talk with the client before making any future report

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). A transfer may become right if the client still wants a different worker after talking it through. Offering it first treats the rupture as unfixable and can feel like rejection.

B **Incorrect** (trap: Good but not best). Explaining the law is accurate, and the client was already told before the report. Repeating it and moving on skips the sense of betrayal, which is what threatens the relationship.

C **Correct.** A mandated report does not end the helping relationship; the worker keeps serving a client who still needs help and works to avoid abandonment (NASW Code 1.17(b)). The client's question is an opening, not a decision to leave, and talking through the sense of betrayal is how the alliance is repaired. The report was explained beforehand (1.07(d)), so repeating the legal reason does not meet the client's hurt. C beats B because restating the law answers why the report happened but not the client's feeling of betrayal, and moving on may end engagement.

D **Incorrect** (trap: Misreads law or ethics). A worker cannot promise advance discussion of every future report, because some situations require reporting without delay or warning. The promise could not be kept and would mislead the client.

Takeaway: After a mandated report, keep serving the client and work through the breach of trust rather than defending the report or handing off.

Sources: NASW Code of Ethics (2021), 1.17(b); NASW Code of Ethics (2021), 1.07(d)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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122. ETH-1108 (rev 2) · Best answer: C

A client in treatment for depression works at a warehouse. The client describes unpaid overtime, a paycheck $600 short, and a supervisor who threatens to cut hours if workers complain. The social worker has explored how the job affects the client's mood, and the client says recovering the wages would ease both the bills and the hopelessness. What should the social worker do **NEXT**?

A. Focus sessions on coping skills for workplace stress
B. Explore further what the lost wages mean to the client
C. Connect the client with a free workers' rights legal clinic
D. Call the employer to request the unpaid wages

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Coping skills can ease stress, but the client has named a concrete injustice whose resolution would lift the depression. Treating only the internal response misses the environmental cause the client identified.

B **Incorrect** (trap: Right action, wrong time). Exploration has already been done, and the client has stated a clear goal. Going back over meaning delays action on the problem the client asked for help with.

C **Correct.** The Social Justice principle and 6.04(a) direct social workers to challenge injustice, including barriers to fair employment, and 1.16(a) calls for referral when another professional's expertise is needed to serve a client fully (NASW Code). Unpaid

wages and retaliation threats are legal problems a workers' rights clinic can address, and the client has said recovering the money would ease both finances and mood. C beats A because coping skills alone treat the distress while leaving the injustice driving it untouched; skills work can continue alongside the referral (NASW Code 1.16(a)).

D **Incorrect** (trap: Misreads law or ethics). Contacting the employer would reveal that the client is in treatment without consent (1.07(b)) and could trigger the retaliation the client fears. Any contact with the employer needs the client's informed direction.

Takeaway: When a client's distress stems from an injustice such as wage theft, link the client to resources that can remedy it, not only coping help.

Sources: NASW Code of Ethics (2021), Ethical Principles: Social Justice; NASW Code of Ethics (2021), 1.16(a); NASW Code of Ethics (2021), 6.04(a); NASW Code of Ethics (2021), 1.07(b)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.5 Core values: competence, social justice, integrity, dignity and worth · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3